

A CARDINAL'S PROMISE COMPANION

CARDINAL'S TOOLKIT



You Are Not Alone. You Are the Difference!

You Are
Not Alone



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Certified Dementia Practitioner

CARDINAL'S PROMISE SERIES

The Cardinal's Toolkit

You Are Not Alone. You Are the Difference!

*A Practical Guide for Families Navigating Care, Decisions,
and the Journey That Changes Everything*

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with Hope Brizzi, PharmD · Project Consultant

“Bear ye one another’s burdens.”

— GALATIANS 6:2

How to Read the Colors

This toolkit is color-coded so families can find the right level of urgency quickly.

■ **RED — Act Today**

Crisis, urgent safety, hospital discharge, hospice, last-days decisions, or caregiver collapse.

■ **YELLOW — Watch Closely**

Something has changed. Get support soon, ask better questions, and do not ignore the pattern.

■ **BLUE — Plan Ahead**

Documents, money, benefits, medication lists, and conversations best handled before a crisis.

IN A CRISIS RIGHT NOW?

Skip ahead. "**What Are You Facing?**" — the Start Here pages just before Chapter 1 — routes you to the right chapter in under a minute, and "**Numbers That Matter**" on the last page puts every phone number in one place.

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EMERGENCY

If someone is in immediate danger — chest pain, trouble breathing, signs of stroke, a serious injury, uncontrolled bleeding, or a person who cannot be safely moved — **call 911**. This handbook helps you plan. It does not replace emergency care.

Dedication

In memory of Lou Brizzi — my father, who inspired this book.

Thank you for choosing me, for believing in me, and for showing me what love looks like.

Mission

To help families avoid unnecessary suffering by understanding the healthcare system before a crisis forces them to.

If this book helps one family make one better decision, it has done its job.

Why I Wrote This Toolkit

My father, Lou Brizzi, died on Christmas Day, 2025. Like most sons, I figured there'd be more time. I figured I'd be at his bedside the way I had been so many times before. I wasn't.

Two weeks later, on January 9, 2026, I was working — because that's what Lou taught me to do. Suit and tie, the same way he wore one for more than forty years. I was sitting in a facility parking lot when a cardinal landed on my driver's-side mirror. It was my seventeenth anniversary of being sober. Lou had pulled me out of a life that nearly ended in a West Palm Beach apartment, and there he was again. Still showing up.

I started journaling to get through the grief, and through the guilt of not being there at the very end. I wrote so much that my wife, Hope, finally told me to turn it into a book.

That book became *The Cardinal's Promise*. It's with an editor now. While I was writing it, I kept circling back to all the decisions that get dropped on families when someone they love starts to decline — and how almost nobody hands you a map. So I wrote one. *The Cardinal's Promise* is the raw version of my story, of being adopted by Lou and finding my way back to a life worth keeping. This is the toolkit that goes with it. For a lot of families, just knowing where to look is the whole difference between getting the help your parent needs and missing it completely.

This isn't a feel-good memoir. It's a working guide — built to knock down the myths and help you ask sharper questions when it counts.

The Cardinal's Promise is my story. This toolkit is yours.

Aging Whispers Before It Shouts

Nobody picks the day they become a caregiver.

It starts with a fall. Or a diagnosis. Or a hospital stay. Or a discharge planner on the phone asking where your parent is going next, like you're supposed to already know. One day you're a son or a daughter. The next you're making decisions nobody prepared you for.

I know how that sneaks up on you, because looking back, my parents had been aging for years before I let myself see it.

In 2019, my mother and I were walking through Central Park. She was sixty-eight. She stepped toward a curb and went down. It didn't seem like much at the time — people trip, people fall. We got her up and went on with our day. But it stuck with me. It wasn't that she fell. It was *how* she fell. I remember thinking, my mom is getting old. Then life moved on and I let it. I didn't ask questions. I didn't look closer. I didn't connect anything.

Three years later she was visiting us in Pensacola, standing in our kitchen, and I noticed her hand shaking. She looked down at it and said, almost offhand, "I need to get in with an acupuncturist."

That night I described it to Hope, my wife, a pharmacist. She didn't hesitate. "Rob, your mom has Parkinson's."

I started crying. Not because a doctor had confirmed anything — none had, yet — but because somewhere in me I already knew she was right. A neurologist confirmed it later.

Here's what I understand now: my silence those three years came from love and fear and denial, all tangled together. I wasn't afraid of being wrong. I was afraid of being right. Knowing something and being able to look straight at it are two different things, and when it's your own parent, looking straight at it is the hardest thing there is.

Aging almost never announces itself all at once. It whispers before it shouts. The curb in Central Park was a whisper. The shaking hand in my kitchen was another. I only heard them once I looked back.

So hear this before anything else: you're not behind. Nobody teaches this. You're walking into the middle of the storm, same as the rest of us did. You don't have to know everything today. You just have to know the next right step. Keep this book close. It's meant to make sense even on the worst day.

How to Use This Handbook

This book is built to be useful on your worst day, not read straight through.

Start wherever the pressure is. If you're in a crisis right now, go straight to Start Here — the triage pages just before Chapter 1 — and to whichever chapter matches what you're facing. If you've got a little more breathing room, read Part 1 first to get your bearings, then work into the care-options, financial, documents, and hospice sections as you need them.

Every chapter follows the same pattern, on purpose: a plain-truth explanation, what families usually think, the teaching, and then:

- **What to Do This Week** — concrete steps you can work through and check off.
- **The Tools** — the worksheets, checklists, and fill-in fields for that chapter.
- **Rob's Note and the Chapter Takeaway** — the heart of it, and the single next question to carry forward.

Write in it. The worksheets in each chapter belong to you. Copy them, fill them in, take them to appointments, hand them to a sibling. A handbook with notes scribbled in the margins is a handbook doing its job.

It gets specific where that matters. Medicaid, aging services, and a lot of benefits are run at the state and county level, which is exactly where generic national advice leaves families stranded. When a state program, office, or rule applies, this book points you to the right door to knock on — your state Medicaid office, your State Health Insurance Assistance Program (SHIP), your Area Agency on Aging, and your county Department of Social Services. **Your State Guide** at the back of this book shows you how to find your state's version of every one of them.

And it doesn't replace the professionals. What it does is help you ask better questions and show up prepared. The real decisions about money, law, health, and benefits still belong with qualified experts in your state. Use this to walk into those rooms already knowing what to ask.

What Are You Facing?

You do not have to read this book from the beginning before you use it. If your family is calm and planning ahead, start at Part 1 and move through the toolkit in order. If something is happening right now, this page is the map: find the situation closest to yours, go there, and come back for the rest later.

Red, Yellow, Blue — Where Are You Right Now?

Most families walking into this book are in one of three places. Find your color, and start there.

• RED — A CRISIS RIGHT NOW

Your parent is coming out of the hospital, is no longer safe at home, or is declining quickly, and needs a higher level of care — assisted living, skilled nursing, or hospice.

Start here: Chapter 8 (Assisted Living), Chapter 10 (Skilled Nursing and Rehab), and Chapters 11–12 (Palliative Care and Hospice). For who pays, see Part 3.

• YELLOW — SOMETHING HAPPENED

A fall, an infection, a hospital visit, a sudden change. Your parent may be able to stay home with the right support, and it may be time to ask whether home health is appropriate.

Start here: Chapter 2 (Falls, Weakness, and Decline) and Chapter 6 (Home Health); consider Chapter 7 (Private Duty Care).

• BLUE — PLANNING AHEAD

Nothing is on fire, but your parent is getting older and you want to get ahead of the money and coverage questions before a crisis forces the issue.

Start here: Chapter 13 (Medicare, and Original vs. Advantage), Chapter 17 (Prescription Assistance), and Chapter 14 (Medicaid, if it may be needed). Then Part 4 (The Documents That Matter).

If This Is Happening, Start Here

IF THIS IS HAPPENING	GO TO
Mom fell, seems weaker, or is suddenly less steady	Chapter 2: Falls, Weakness, and Decline
Dad is forgetting medications, bills, appointments, or directions	Chapter 3: Memory Loss and Dementia

Your parent is repeating questions or acting confused in new ways	Chapter 3: Memory Loss and Dementia
The caregiver is exhausted, angry, resentful, or overwhelmed	Chapter 4: Caregiver Burnout
One person is doing everything and the family is not helping	Chapter 4 and Chapter 5: The Family Meeting
The family is arguing, avoiding decisions, or blaming each other	Chapter 5: The Family Meeting
Your parent is coming home from the hospital and you feel unprepared	Chapter 10: Skilled Nursing and Rehab
Your parent needs help bathing, dressing, cooking, or staying safe	Chapter 7: Private Duty Care
Living alone does not seem safe anymore	Chapter 8: Assisted Living
Doctors are talking about comfort, symptoms, or quality of life	Chapter 11: Palliative Care
You are wondering if it may be time for hospice	Chapter 12: Hospice
Death may be getting close	Part 5: Hospice and the Last Days
You are confused about Medicare, Medicaid, insurance, or who pays	Part 3: Paying for Care
You do not know where the legal, medical, or financial papers are	Part 4: The Documents That Matter

EMERGENCY

If you are in immediate danger — chest pain, trouble breathing, signs of stroke, a serious injury, uncontrolled bleeding, or a person who cannot be safely moved — **call 911**. This toolkit can help you organize decisions. It does not replace emergency care.

If You Are Not Sure Where to Start

Start with these three questions.

1. Is anyone unsafe today? Unsafe can mean falling, wandering, leaving the stove on, missing critical medication, driving dangerously, or being alone when they cannot manage safely. If yes, focus on safety first — see Chapters 2, 3, and 4.

2. Has something changed recently? A change may be physical, mental, emotional, financial, or behavioral: more falls, new confusion, weight loss, missed medications, unpaid bills, less bathing, more sleeping, new agitation, caregiver exhaustion. If yes, write down what changed — see Chapter 1.

3. Who needs to be involved? Most caregiving crises get worse when one person carries everything alone. Ask who is the main caregiver, who has medical and financial information, who can attend appointments, and who needs to be part of decisions — see Chapter 5.

In the First 24 Hours — Do These

SEVEN ACTIONS, NOT SEVEN CHAPTERS

1. **Handle immediate safety** — the person, the stove, the car keys, the stairs.
2. **Photograph the insurance and Medicare cards**, front and back.
3. **Gather the medication list** — or the bottles, in one bag (Chapter 25).
4. **If a hospital is involved, ask:** "Is my parent admitted as an inpatient, or under observation?" Write down the answer and who gave it — Chapter 13 explains why this one question matters so much.
5. **Find out who is legally authorized to make decisions**, and where the power of attorney papers are (Chapters 19–20).
6. **Start a notes page** — what changed, when it started, who told you what.
7. **Then go to the chapter this page pointed you to.** That is your next right step.

In the next 24–72 hours: read the chapter that matches your situation, hold a short family meeting (Chapter 5), check the caregiver's own limits honestly (Chapter 4), and start the Important Documents Folder (Chapter 19) and the Emergency Contacts and Crisis Plan (Chapter 26).

Finding immediate help: your Area Agency on Aging, your county or state social services office, your state Medicaid office, and your State Health Insurance Assistance Program (SHIP) for Medicare counseling. Your State Guide at the back of this book shows how to find each one, and Numbers That Matter — the last page — puts every phone number in one place.

THE CARDINAL RULE

When everything feels urgent, do not try to solve the whole future. Write down: What changed? What is unsafe today? Who needs to know? What is the next call? The next appointment? The next document to find? That is enough for now.

One more thing. You don't have to solve all of this today. Find the most urgent issue, do one thing about it, and then take the next right step. That's it. That's the whole method.

For the Future Planners

If nothing is urgent yet, this is where you start.

In fact, a calm stretch may be the best time to use this book.

Most families wait until the fall, the hospital stay, the diagnosis, the discharge meeting, or the late-night phone call. By then, every decision feels heavier because it has to be made fast. Planning ahead does not mean you are expecting the worst. It means you are protecting the people you love from having to guess later.

Start with the blue chapters: Medicare Basics, Medicaid Basics, Private Pay, Prescription Assistance, The Important Documents Folder, Healthcare Power of Attorney, Living Will / Advance Directive, Financial Power of Attorney, Medication and Doctor Lists, and Emergency Contacts and Crisis Plan.

Your goal is simple: know where the documents are, understand who can make decisions, talk about money before panic enters the room, and write down the medical information no one wants to hunt for during an emergency.

A future planner does not need to solve everything. Start with three questions: Who can speak for my parent if they cannot speak for themselves? Where are the documents, passwords, insurance cards, and medication lists? What would my parent want if life changed quickly?

That is not fear. That is love with a clipboard.

■ WORKSHEET — FUTURE PLANNER SNAPSHOT

Who can speak for my parent if they cannot speak for themselves?

Where are the documents, insurance cards, passwords, and medication lists?

What would my parent want if life changed quickly?

What are the three calls we should make before a crisis?

PART 1 · SOMETHING HAS CHANGED

CHAPTER 1

Something Has Changed

Part 1 — Something Has Changed. This part is for the moment when your gut knows something is different, but you do not yet know what it means. A parent may seem weaker. More forgetful. Less steady. More withdrawn. Or maybe nothing dramatic has happened yet, but the house feels different. The rhythm has changed. Families often wait for a major event before they act — a fall, a hospitalization, a stove left on, a doctor saying, "I'm concerned." But many caregiving journeys begin before the crisis, with small signs that are easy to explain away. Sometimes those explanations are true. Sometimes they are the family's way of buying time. You do not need to panic. You do need to pay attention.

"The best time to plant a tree was twenty years ago. The second best time is now."

— PROVERB

THE PLAIN TRUTH

Aging does not always announce itself clearly. Sometimes decline arrives loudly, through a fall, an infection, or a frightening phone call. Other times it arrives quietly.

The refrigerator has less food in it. The laundry is piling up. The same question is asked three times. The pillbox is full when it should be empty.

One change may not mean much; a pattern means more. This chapter will not diagnose your parent — it will help you notice patterns and know what to do with them. A better first question than "Is this normal aging?" is: **what changed, when, and is it affecting safety?**

WHAT FAMILIES USUALLY THINK

Families often think they need to be certain before they act. So they wait — until they can prove something is wrong, until the parent agrees, until the doctor brings it up. They wait because naming the change makes it real.

But caregiving does not require perfect certainty. It requires honest attention. You are not accusing your parent of failing. You are noticing that life may require more support than it used to. That difference matters.

What May Be Changing

Physical changes. More falls or near falls, trouble standing from a chair, new weakness, weight loss, sleeping more, less interest in food, poor hygiene, trouble walking, new pain, shortness of breath, swelling, frequent infections, or new incontinence. A sudden change should always be taken seriously.

Memory and thinking changes. Repeating the same question, getting lost in familiar places, missing appointments, forgetting medications, confusing dates, trouble managing bills, poor judgment, falling for scams, or new suspicion. Memory changes that affect safety, finances, medication, or daily living should not be ignored.

Home safety changes. Spoiled food, burned pans, cluttered walkways, unpaid bills, mail piling up, scattered medication bottles, poor lighting, loose rugs, or trouble getting in and out of the shower. The home often tells the truth before the person does. Look gently. Do not shame. Just notice.

Emotional and behavioral changes. Withdrawal, irritability, anxiety, depression, new anger, suspicion, loss of interest in hobbies, refusing help, or increased confusion in the evening. Behavior is information. It may be grief, pain, depression, infection, dementia, medication side effects, or loneliness. The point is not to label it immediately — it is to write it down and ask better questions.

When to Call the Doctor

Call the doctor when a change is new, worsening, or affecting daily life. Call especially for sudden confusion, a fall, new weakness or trouble walking, new shortness of breath, chest pain, signs of infection, medication mistakes, weight loss, dehydration, new incontinence, or sudden mood changes.

A sudden change in an older adult can have a medical cause. Do not assume it is "just aging." Confusion is sometimes caused by infection, dehydration, medication changes, pain, poor sleep, or low oxygen — problems that may be treatable. If the change is sudden, call. If the person may be in immediate danger, call 911.

What Is Urgent and What Can Wait

CALL 911 FOR

Chest pain, trouble breathing, signs of stroke, serious injury from a fall, uncontrolled bleeding, loss of consciousness, sudden severe confusion, or a person who cannot be safely moved. **Signs of stroke are sudden:** face drooping, arm weakness, slurred or garbled speech, sudden confusion, sudden vision loss, or trouble walking. Note the time it started and call 911 immediately — stroke treatment is time-limited.

Call the doctor soon for: a fall without obvious serious injury, new confusion or weakness, medication errors, weight loss, less eating or drinking, increased pain, new swelling, or a caregiver who can no

longer manage safely. (One exception: a head strike in someone on a blood thinner is an emergency-department visit the same day — see the red box in Chapter 2.)

Schedule a planning conversation for: gradual decline, increasing need for help, difficulty managing the home, unclear documents, no emergency plan, or concerns about driving or living alone. Planning early is not overreacting. It is how families avoid being forced into decisions during a crisis.

How to Raise It

Raise it with your parent first, and gently. Lead with care, not a verdict. "I've noticed the stairs seem harder lately — can we figure out something together?" lands far better than "You can't manage anymore." People resist being managed and respond to being respected — preserve their dignity and their voice in the decision wherever you safely can. Then bring it to the physician. Before the visit, write down specific examples: "got lost driving to church on March 3." Vague worry is easy for a busy doctor to dismiss; concrete observations get taken seriously.

WHAT TO DO THIS WEEK

1. **Write down the change.** Use facts and dates. Do not rely on memory during stress.
2. **Check immediate safety.** Bathroom, medications, food, stove, driving, ability to call for help, and the caregiver's safety too.
3. **Call the doctor** if the change is new or concerning. Use your notes. Be specific.
4. **Tell the key family members.** Create awareness, not panic.
5. **Choose the next right section of this toolkit.** Physical change → Chapter 2. Memory → Chapter 3. Caregiver overwhelmed → Chapter 4. Decisions needed → Chapter 5.

■ MINI-TOOL — THE "SOMETHING HAS CHANGED" NOTE

Use this when you are worried but not sure what to do next.

What changed? _____

When did it start? _____

Is it getting better, worse, or the same? _____

Is anyone unsafe today? _____

What have we already tried? _____

Who needs to know? _____

What is the next call? _____

What is the next appointment? _____

What information do we need to find? _____

■ QUESTIONS TO ASK THE DOCTOR

1. Could this change be caused by infection, dehydration, medication, pain, or another treatable issue?
2. Are any medications possibly causing confusion, dizziness, weakness, or falls?
3. Should we check bloodwork, urine, oxygen, or blood pressure?
4. Is it safe for my parent to live alone right now?
5. Should my parent be driving?
6. Would home health, therapy, or nursing support help?
7. Should we consider a memory evaluation?
8. What changes should make us call you again — and what should send us to the hospital?

ROB'S NOTE

Families usually know more than they think.

They may not know the diagnosis. They may not know the benefit rules. But they know when something feels different.

I have watched families talk themselves out of that feeling because they did not want to upset anyone. I understand that. No one wants to be the person who says, "Mom is not safe anymore."

But noticing a change is not betrayal. It is care. You are not taking over. You are paying attention. And sometimes paying attention is the first act of love that turns panic into a plan.

CHAPTER 1 TAKEAWAY

You do not need to diagnose the problem today. The next right question is: **what changed, and is anyone unsafe today?**

Falls, Weakness, and Decline

THE PLAIN TRUTH

A fall is not just a fall. For an aging parent, a fall can be a warning sign that something else is changing.

Sometimes the cause is simple — a loose rug, poor lighting, a pet underfoot. But sometimes a fall points to something bigger: weakness, dizziness, medication side effects, dehydration, infection, poor vision, or decline after a hospital stay. Families often focus only on whether anything broke. That matters, but the better question is: **why did the fall happen, and what needs to change so it does not happen again?**

WHAT FAMILIES USUALLY THINK

Families often say, "She just tripped," "It was one time," or "We moved the rug, so it should be fine now." Sometimes that is true. But caution is not a care plan. Most older adults are not falling because they are careless. They are falling because something has changed — the body, the home, the medications, the disease, or the level of support needed. The family's job is not to blame. It is to investigate.

Why Falls Matter

A fall can change everything quickly — fractures, head injury, hospitalization, rehab, loss of confidence, less activity, more weakness, more dependence. Even without a major injury, many older adults become afraid after a fall. They move less, their legs weaken, and the risk of the next fall rises. That loop can become a trap: fall, fear, less movement, more weakness, another fall. This is why a "minor" fall should not be ignored. It may be the first sign of a major shift.

After a Fall: First Questions

Start with safety. Are they hurt? Did they hit their head? Are they bleeding? Can they move normally? Are they confused? Do they have new pain? Are they on blood thinners? Do they remember what happened? Has this happened before?

ACT NOW

If there is serious injury, a possible head injury, sudden confusion, chest pain, trouble breathing, one-sided weakness, severe pain, or they cannot be safely moved, **call 911**. Do not drag or force someone up if you are not sure they are safe to move — a second injury can happen when a frightened caregiver tries to lift someone alone.

While they are on the floor: check for injury before any move, keep them warm and calm, and if they are uninjured but cannot get up, call 911 or the non-emergency line and ask for a **lift assist** rather than lifting alone. And know the term "long lie": an older adult who has been down for an hour or more needs medical evaluation even without visible injury.

When to Call the Doctor After a Fall

Call after a fall if it is new or repeated, if they hit their head, are on blood thinners, have new pain or weakness, are dizzy or confused, do not remember falling, are walking differently, are afraid to walk, are eating or drinking less, recently changed medication, or you are not sure why the fall happened. Do not assume a fall is normal aging. A doctor may review medications, check blood pressure and hydration, screen for infection, or order therapy. The goal is not just to document the fall — it is to prevent the next one.

HEAD INJURY + BLOOD THINNERS

If your parent takes a blood thinner — anticoagulants like warfarin, Eliquis, or Xarelto, antiplatelet drugs like Plavix, or even daily aspirin — and hits their head, do not wait for symptoms. **Go to the emergency department the same day, even if they seem fine.** Bleeding inside the head can show up hours or days later. After any head strike, watch for worsening headache, vomiting, new confusion, or unusual sleepiness over the next 24–72 hours.

Weakness Is Information

Weakness may look like trouble standing from a chair, holding walls or furniture while walking, shuffling steps, needing more help with stairs, difficulty bathing, more fatigue after small tasks, or refusing a needed cane or walker. Weakness affects safety, toileting, bathing, nutrition, and the caregiver's workload. If your parent is getting weaker, write it down: when did it start, is it sudden or gradual, did it begin after a hospital stay or a medication change, is there pain, shortness of breath, or dizziness? Weakness has a story. The family needs to help tell it clearly.

Decline After the Hospital

Many families are shocked by how much an older adult can decline after a hospital stay. This does not always mean the decline is permanent, but it does mean the discharge plan matters. Before discharge, ask: Can they walk safely? Can they get to the bathroom? Do they need a walker, wheelchair, shower

chair, or bedside commode? Are they safe alone? Do they need home health or rehab? Who is managing medications? What changed during the stay, and who do we call if things get worse? Hospital discharge is not the finish line. It is a handoff — and bad handoffs create crises.

When Home Health May Help

After a fall, new weakness, illness, or a hospital stay, families should ask whether home health may be appropriate. Home health is medical care in the home — skilled nursing, physical, occupational, or speech therapy, medical social work, and limited aide support tied to the care plan. It is not the same as private duty care and does not usually provide long blocks of supervision, housekeeping, or ongoing sitting services.

IMPORTANT

Home health is not something a family can simply request over the phone and receive automatically. For Medicare-covered home health, a clinician must determine that the person meets the requirements — including a skilled need, homebound status, and the required face-to-face documentation. So the better request is not "Can you order home health?" It is: **"Can we schedule an evaluation to discuss this fall or decline and determine whether home health is appropriate?"**

Home Health vs. Private Duty Care

These sound similar but are not the same.

HOME HEALTH	PRIVATE DUTY CARE
Usually short-term and medical	Usually non-medical daily support
Ordered by a clinician; criteria apply	Arranged by the family; often paid privately
Nursing, PT, OT, speech, social work, limited aide	Bathing, dressing, meals, housekeeping, companionship
Not there all day	Can cover longer blocks, including overnight
Helps treat, teach, assess, and strengthen	Helps with daily support and supervision

A family may need both. The question is not which sounds better. It is: **what does my parent actually need at home?**

Home Safety After a Fall

Families often say, "She's lived in this house for forty years — it's perfectly safe." The familiarity is real, but so are the new risks. A home that was fine for a healthy adult can be full of hazards for someone

with reduced balance, vision, or cognition. And the changes that prevent falls usually extend independence rather than restrict it — they let a parent stay home longer and more safely.

A safer home does not have to be fancy. Start with the obvious risks: loose rugs, clutter, poor lighting, stairs without railings, slippery bathroom floors, no grab bars, a low toilet seat, furniture that is hard to rise from, and no phone or call device nearby. The bathroom is one of the highest-risk areas — consider grab bars, a shower chair, a handheld shower head, a non-slip mat, a raised toilet seat, a bedside commode, and a nightlight along the path from bed to bathroom. Small changes can prevent big injuries.

Near Falls Count Too

Families often remember the big fall but not the near falls — grabbing the counter suddenly, stumbling but catching themselves, losing balance while turning, or saying, "I just got dizzy for a second." Near falls are the smoke alarm before the fire. Write them down. Several near falls deserve to be taken as seriously as a fall to the ground.

When Falls May Mean More Help Is Needed

A fall does not automatically mean assisted living or memory care. But repeated falls may mean the current setup is no longer safe — especially if your parent lives alone, cannot reliably call for help, forgets to use the walker, falls at night, cannot bathe safely, is on blood thinners, has dementia, or the caregiver cannot supervise safely. The goal is not to keep someone home at all costs. It is to keep them as safe, supported, and dignified as possible. Sometimes home still works with the right help. Sometimes home is no longer the safest place. Avoiding the question does not make the risk disappear.

WHAT TO DO THIS WEEK

1. **Write down every fall and near fall** — date, time, place, what happened, injury, whether they hit their head, and whether they used a cane or walker.
2. **Call the doctor** — especially if the fall is new, repeated, unexplained, or tied to weakness, dizziness, or confusion.
3. **Ask about home health** — request an evaluation to see whether nursing, PT, or OT is appropriate at home.
4. **Review medications** — ask the doctor or pharmacist whether any could raise fall risk. Do not stop medications on your own.
5. **Make the home safer** — start with the path from bed to bathroom, then the shower, stairs, rugs, and lighting.
6. **Ask whether more support is needed** — physical or occupational therapy, home health, private duty care, rehab, or a care-setting change.

■ MINI-TOOL — THREE QUESTIONS AFTER A FALL

1. What happened? Where was the person? What were they doing? Did they trip, slip, faint, or feel dizzy? Did anyone see it?

2. What changed? New pain? New confusion or weakness? New fear of walking? New medication? Less eating or drinking?

3. What needs to change now? Call the doctor? Home safety changes? Therapy? Home health? More supervision? Safe alone tonight?

Fall Log — record every fall and near fall. Bring this to appointments.

DATE	TIME	LOCATION	WHAT HAPPENED	INJURY / NEXT STEP

■ QUESTIONS TO ASK THE DOCTOR

1. What could be causing the falls or weakness?
2. Could medications be contributing?
3. Should blood pressure be checked sitting and standing?
4. Should we check for infection, dehydration, anemia, or pain?
5. Is physical or occupational therapy appropriate?
6. Would an evaluation for home health be appropriate?
7. Does my parent need a cane, walker, wheelchair, shower chair, or bedside commode?
8. Is it safe for my parent to live alone, bathe alone, or drive?
9. What signs should make us call again — and what signs mean call 911?

ROB'S NOTE

Families often tell me about "the fall."

But after a few more questions, I usually hear about the three near falls before it. The stumble in the hallway. The hand on the kitchen counter. The bad turn in the bathroom. The moment Dad said, "I'm fine," but sat down for twenty minutes afterward.

A fall is not always the beginning. Sometimes it is the first thing the family can no longer ignore.

That is why I want families to write things down early — not to build a case against the person they love, but to build a plan around what is actually happening. Asking about home health is not dramatic. It is practical. A nurse or therapist in the home may see what the family cannot, because the family is too close, too tired, or too used to making it work.

CHAPTER 2 TAKEAWAY

Falls, weakness, and decline are information. The next right question is: **why did this happen, and what needs to change so it does not happen again?**

PART 1 · SOMETHING HAS CHANGED

CHAPTER 3

Memory Loss and Dementia

THE PLAIN TRUTH

Memory loss is not always dementia. But memory changes that affect safety, medications, money, driving, cooking, or judgment should not be brushed aside.

Families often notice the first signs long before a diagnosis — a repeated question, an unpaid bill, the stove left on, a familiar route that becomes confusing. At first, they explain it away. Sometimes those explanations are true. But when mistakes become patterns, the family needs to pay attention. Dementia is not only about memory. It can affect judgment, mood, language, movement, safety, and the ability to recognize risk. The goal is not to diagnose your parent at the kitchen table. It is to notice what is changing, protect safety, and ask for the right evaluation.

WHAT FAMILIES USUALLY THINK

Families often think dementia means one obvious thing: a person who does not know who or where they are. But many people with dementia can still hold good conversations, tell old stories, recognize family, and sound convincing on the phone. That is what makes it confusing. A parent may seem fine during a short visit but struggle when alone — well in the morning, confused by evening. The sibling who calls for five minutes may say, "Dad sounds fine." The daughter who manages the pillbox may say, "Something is wrong." The person handling the daily details often sees the truth first.

What Memory Loss May Look Like

Repeating questions, forgetting recent conversations, missing appointments, losing important papers, forgetting medications or taking them twice, leaving food out, getting lost while driving, trouble using the phone or stove, confusion with dates, difficulty managing bills, falling for scams, new suspicion or paranoia, or neglecting hygiene. One mistake may not mean dementia. A pattern deserves attention.

Safety Comes First

The first question is not "Is this dementia?" It is "**Is this safe?**" Memory loss becomes urgent when it affects medication, driving, cooking, finances, hygiene, or nutrition — or when it leads to wandering, falls, or an inability to respond to an emergency. A person can be loving, intelligent, and still unsafe. Safety does not erase dignity. Safety protects dignity from a preventable crisis.

Medication Safety

Medication problems are one of the clearest signs that more support may be needed — a full pillbox when it should be empty, pills on the floor, duplicate bottles, taking morning pills at night, or refusing medication out of confusion. Medication mistakes can cause falls, confusion, weakness, or hospitalization. If mistakes are happening, do not just remind the person to be careful. Change the system: a pill organizer, blister packs from the pharmacy, phone alarms, family checks, a medication review, or an evaluation for home health nursing to help with medication teaching. The question is not whether your parent "should" remember. It is whether the medication system is safe.

Driving Safety

Driving is one of the hardest conversations, because for many older adults it means independence and identity. But unsafe driving can put your parent and others at risk. Warning signs: getting lost on familiar roads, new dents, confusing the pedals, drifting from the lane, missing stop signs, becoming overwhelmed in traffic, or family feeling afraid as passengers. Ask the doctor, "Is it safe for my parent to continue driving?" and ask about a formal driving evaluation if available. If driving must stop, make a transportation plan first — for appointments, groceries, church, and social connection. Removing keys without a plan creates anger and isolation. When driving ends, support must replace what is lost.

Money, Cooking, and Wandering

Money and scams. Watch for unopened mail, late notices, duplicate donations, unusual withdrawals, new "friends" asking for money, or secrecy around finances. Handle it carefully — the goal is to prevent harm, not to seize control. Ask whether there is a financial power of attorney, who is trusted to help, whether bills can be on autopay, and whether professional guidance is needed. Financial decline may be a symptom, not a character flaw.

Cooking and the kitchen. Burned pans, a stove left on, spoiled food, little food in the house, or replacing meals with snacks. If the stove is unsafe, take it seriously — meal delivery, family-prepared meals, private duty help at meal times, stove shut-off devices, or removing the knobs. Food is independence and comfort, but safety comes first.

Wandering and getting lost. Leaving the house at night, trying to "go home" while already home, getting lost in stores, or restlessness in the evening. Wandering can become dangerous quickly. Ask whether the person can safely be alone, whether doors are secured, whether identification or a medical ID is available, and whether memory care is becoming necessary. Program the Alzheimer's Association 24/7 Helpline (1-800-272-3900) into your phone and ask about a wanderer locator program (MedicAlert, Project Lifesaver, or similar). Let trusted neighbors know proactively, and consider a heads-up call to the police non-emergency line. A person who wanders may be trying to meet a need their brain can no longer explain. The family still has to protect safety.

Mood and Behavior Changes

Dementia can bring anxiety, depression, irritability, suspicion, hallucinations, agitation, sundowning, or refusing care. These changes are painful — a daughter may say, "This is not my mother." That grief is real, but behavior is still information. Pain, infection, medication side effects, poor sleep, constipation, dehydration, and stress can all worsen confusion. When behavior changes suddenly, call the doctor. Do not assume every change is "just dementia."

Meeting Them in Their Reality

As dementia progresses, the rules of conversation change. The goal shifts from exchanging accurate information to preserving connection and dignity. When a person with dementia believes something that isn't true — that they need to go to a job they retired from decades ago, that a long-dead spouse is coming home — arguing or correcting rarely helps and often causes real pain. Respond to the feeling, not the fact. "You're worried about getting to work on time" acknowledges the emotion without a confrontation that neither of you can win.

Keep it simple and calm. Use short sentences and one idea at a time. Ask one question, not several. Give simple choices — "Would you like the blue sweater or the green one?" — rather than open-ended ones. Your tone and body language carry more than your words; a calm, warm presence reassures even when the words don't land.

A Predictable Routine Helps

A predictable routine is one of the most underrated tools in dementia care. It lowers anxiety, conserves everyone's energy, and keeps essentials from falling through the cracks. Build the day around anchors — consistent wake and sleep times, regular meals, and medication at the same times — and schedule the demanding tasks, like bathing and appointments, for the time of day your parent is at their best, which for many is the morning. Watch for "sundowning," the increased confusion and agitation that often arrives in the late afternoon and evening, and keep those hours calmer, quieter, and simpler. Leave room for what gives the day meaning, not just what keeps it functioning: a walk, music, a visit, time outside.

Rule Out the Reversible Causes

Some conditions that look exactly like dementia are treatable and even reversible. A urinary tract infection in an older adult can cause sudden, dramatic confusion. So can dehydration, a thyroid problem, a vitamin B12 deficiency, depression, or a bad combination of medications. When confusion arrives suddenly — from an infection, dehydration, or a medication — clinicians call it **delirium**: a state that comes on over hours to days, fluctuates through the day, and clouds attention. It is a medical urgency, and it is different from dementia's slow slope over months to years; using the word with the care team gets it taken seriously. Before accepting a dementia diagnosis as the whole story, insist that the doctor

rule these out. The phrasing that works: **"Is there anything simple and reversible we might be missing here?"** True dementia is progressive, but the real question is always: dementia caused by what?

When to Call the Doctor

Call when memory or thinking changes are new, worsening, sudden, or affecting safety — sudden confusion, new hallucinations or paranoia, medication mistakes, getting lost, falls, weight loss, unsafe driving, wandering, or new incontinence. Ask for an evaluation, and use specific examples. Instead of "Dad is getting forgetful," say, "Dad got lost driving home from church, missed two medication doses this week, and paid the same bill three times."

When Home Health or Memory Care May Help

Home health may help when memory loss is connected with a medical need, recent decline, falls, medication concerns, weakness, hospitalization, or caregiver teaching. It is not long-term supervision, a sitter service, or private duty care — and it is not ordered simply because a family asks over the phone. For Medicare-covered home health, a clinician must evaluate the person and confirm the requirements are met, including skilled need, homebound status, and face-to-face documentation. If you are concerned, ask for an appointment: "We are seeing memory and safety changes. Can we schedule an evaluation to determine whether home health is appropriate?"

Memory care may be needed when dementia creates safety risks that cannot be managed at home — ongoing wandering, continued medication mistakes, being unsafe alone, a caregiver who cannot sleep, unsafe agitation, or increasing falls. Memory care is not a punishment. It is a care setting. Families should not wait until the caregiver collapses or the person is harmed before having the conversation.

WHAT TO DO THIS WEEK

1. **Write down examples with dates** — medication mistakes, driving concerns, safety issues, repeated questions, wandering, or behavior changes.
2. **Check safety** — medications, driving, cooking, being alone, ability to call for help, and money.
3. **Call the doctor** and ask for a memory or cognitive evaluation, using specific examples.
4. **Review medical causes** — infection, dehydration, pain, medication side effects, depression, or poor sleep.
5. **Get documents in order** — healthcare and financial power of attorney, advance directive, medication and doctor lists, emergency contacts. Do this early.
6. **Decide what support is needed now** — family check-ins, home health, private duty care, adult day care, assisted living, memory care, or a family meeting.

■ MINI-TOOL — MEMORY AND SAFETY SNAPSHOT

Use this when you are worried about memory loss but unsure what to do next.

What changed? _____

When did it start? Is it getting worse? _____

Is medication safe? _____

Is driving safe? _____

Is cooking safe? _____

Is the person safe alone? _____

Are bills and money safe? _____

Is the caregiver safe and able to continue? _____

Who needs to know? What is the next call? _____

■ DEMENTIA SAFETY CHECKLIST

Check any concern that applies. If several are checked, do not wait — call the doctor and begin a family planning conversation.

- | | |
|---|---|
| ☐ Repeats questions often | ☐ Wanders or tries to leave |
| ☐ Forgets or misuses medications | ☐ Is confused at night |
| ☐ Gets lost driving or has new driving concerns | ☐ Has unsafe anger or agitation |
| ☐ Leaves stove or appliances on | ☐ Has poor hygiene |
| ☐ Forgets to eat or has spoiled food in the home | ☐ Cannot call for help reliably |
| ☐ Misses bills or falls for scams | ☐ Caregiver is exhausted or unsafe |
| ☐ Misplaces items or accuses others of stealing | |

■ QUESTIONS TO ASK THE DOCTOR

1. Could this be dementia, delirium, depression, medication side effects, infection, dehydration, or pain?
2. Should we do a memory screening and review medications?
3. Should we check labs or urine?
4. Is my parent safe to live alone, drive, cook, and manage medications?
5. Would an evaluation for home health, nursing, therapy, or social work help?
6. Should we see a neurologist, geriatrician, or memory clinic?
7. What changes should make us call again — and what should be treated as urgent?

ROB'S NOTE

Dementia can be especially hard because families lose the person in pieces.

One day they are still telling stories from forty years ago. The next day they cannot remember whether they ate lunch. They may recognize you, then accuse you. They may sound fine on the phone, then leave the stove on after dinner.

That back-and-forth confuses families. It can also divide them. The person who visits for an hour may think everything is fine. The person who handles the medications, bills, and nighttime calls knows it is not.

If that is you, write things down. Not to prove everyone else wrong — to help the family see clearly. Memory loss needs compassion. It also needs a plan. Both can be true.

CHAPTER 3 TAKEAWAY

Memory loss is about safety, judgment, and daily life — not only forgetting. The next right question is: **what is no longer safe to leave to memory alone?**

Caregiver Burnout

THE PLAIN TRUTH

Sometimes the emergency is not only the aging parent. Sometimes the emergency is the caregiver.

A caregiver can look responsible on the outside and be falling apart on the inside — still making appointments, filling pillboxes, paying bills, managing the discharge, and trying to keep their own life from collapsing, all while tired in a way sleep does not fix. Caregiver burnout happens when the needs of the person receiving care become greater than one person can safely carry. It is not weakness, selfishness, or a lack of love. It is a warning sign. When a caregiver burns out, everyone becomes less safe. A good care plan must include the caregiver. If the plan protects the parent but breaks the caregiver, it is not a real plan.

WHAT FAMILIES USUALLY THINK

Families often think caregiving should be manageable because it is done out of love. "She's my mother." "I should be able to do this." "My siblings are busy." "I don't want strangers in the house." Sometimes love gives people strength. But love does not remove exhaustion. Love does not lift a person safely from the bathroom floor, make dementia predictable, or turn one worn-out daughter into a full care team. Families often wait too long because asking for help feels like failure. It is not failure. It is planning.

What Burnout Looks Like

Burnout can look like sadness, anger, control, or silence — a daughter who snaps at everyone, a son who stops answering calls, a spouse who cries in the shower. Watch for constant exhaustion, irritability, resentment, anxiety, depression, trouble sleeping, ignoring personal health, pulling away from friends, feeling trapped, anger toward the parent or siblings, guilt for wanting relief, making more mistakes, and dreading the phone. The caregiver may not use the word burnout. They may say, "I'm fine," or "I don't have a choice." Listen closely. Sometimes "I'm fine" means "I have no room left to fall apart."

IF YOU ARE AT THE EDGE

If you are having thoughts of harming yourself, or you are afraid of what exhaustion might make you do, call or text **988** — the Suicide & Crisis Lifeline — any hour. And tell your own doctor what caregiving is costing you: caregiver depression is a treatable medical condition, not a character flaw.

Respite care exists for exactly this moment — ask the doctor, a social worker, or the Eldercare Locator (1-800-677-1116) about respite options today.

The Hidden Load

Caregiving is not only the visible tasks. It is the hidden load — remembering refills, watching for decline, scheduling appointments, handling insurance, tracking bills, anticipating falls, keeping peace between siblings, reading discharge papers, and worrying at night. This is why one caregiver may seem "too emotional" to relatives who are less involved. They are not reacting to one task. They are carrying the whole weather system. A family cannot fix burnout until it names the actual load.

The Promise That Traps Families

Many families carry a promise: "I promised Mom I would never put her in a nursing home." Those promises usually come from love — but they are often made before anyone understands what care may require: before dementia causes wandering, before falls happen at night, before bathing becomes unsafe, before the caregiver has not slept in months. A promise made in one season may need wisdom in another. The deeper promise is usually not "I will keep you in this exact place no matter what." It is "I will make sure you are cared for. I will protect your dignity. I will not abandon you." Sometimes keeping the deeper promise means changing the plan. That is not betrayal. That is love growing up under pressure.

Resentment Is Information

Caregivers often feel ashamed of resentment. But resentment is information. It may mean the workload is unfair, there is no relief, boundaries are unclear, others are not helping, or the current plan no longer works. Instead of "I'm a terrible person for feeling this way," try "What is this resentment telling me about the care plan?" The need it points to may be rest, backup, money, a schedule, private duty care, adult day care, or a family meeting. Resentment is not the final answer. It is the smoke under the door.

The Family Pattern Problem

Caregiving does not create family patterns. It reveals them. The responsible sibling becomes more responsible. The avoidant sibling becomes more avoidant. Old wounds walk into the family meeting and sit down. This is why caregiving decisions turn emotional so quickly — families are deciding about medication and money, but also about history. You may not be able to fix the whole family system. But you can make the next decision clearer: write down facts, name the care needs, choose one point person, assign specific tasks, and set deadlines. Do not try to heal thirty years of history before deciding who takes Dad to the doctor on Tuesday.

When to Ask for Help

Ask for help before the caregiver breaks. Help may come from family, friends, community or faith groups, the primary doctor, home health (if there is a skilled need and criteria are met), private duty care, adult day care, respite care, assisted living, memory care, palliative care, hospice if appropriate, a social worker, or a counselor. Do not wait until you are desperate to learn what exists.

How to Ask Family for Help

Many caregivers ask too generally — "I need help." That is honest, but hard to act on. Name the specific task instead. Not "I need help with Mom," but "Can you take Mom to her appointment Thursday at 10?" Not "I'm overwhelmed," but "I need someone to handle medication refills every month." Specific requests are harder to ignore, and they reveal who is truly willing to help. End the family meeting with names, tasks, and dates — not vague kindness.

When the Parent Refuses Help

Many caregivers are trapped by one sentence: "They won't let me." Refusal is hard, but it does not automatically mean the current plan is safe. Ask whether the parent understands the risk, whether memory loss is affecting judgment, and whether a doctor needs to evaluate capacity or safety. Sometimes help gets in more easily with different words — "Someone is coming to help with laundry and lunch" instead of "You need a caregiver," or "The therapist wants you to use this so you can keep moving safely" instead of "You need a walker." The goal is not to win an argument. It is to lower the risk.

WHAT TO DO THIS WEEK

1. **Write down the actual workload** — every task the caregiver handles. The family cannot divide work it has not named.
2. **Take the burnout self-check** on the next page. Do not argue with the score. Use it as information.
3. **Hold a family meeting** (Chapter 5) that produces specific assignments, not vague promises.
4. **Add one form of relief this week** — a covered appointment, meals, help with bills, paid care, or exploring adult day care or respite.
5. **Protect the caregiver's health** — keep their own appointments, sleep, eat, and take time away. That is maintenance, not selfishness.
6. **Recheck the care setting** — is home still safe, is one caregiver enough, is nighttime care breaking the caregiver?

■ MINI-TOOL — CAREGIVER BURNOUT SELF-CHECK

This is not a diagnosis. It is a warning-light checklist. Use it when the caregiver says, "I'm fine," but everything around them suggests otherwise. Check any statement that feels true most days, then total each section.

Physical exhaustion (___ / 8)

- ☐ Tired even after sleeping
- ☐ Not sleeping enough
- ☐ Waking at night over caregiving
- ☐ More headaches, tension, or pain
- ☐ Skipping my own appointments
- ☐ Eating worse than usual
- ☐ Getting sick more often
- ☐ Physically worn down

Emotional strain (___ / 8)

- ☐ Anxious much of the time
- ☐ Sad, numb, or hopeless
- ☐ Crying more easily
- ☐ Angry or irritated often
- ☐ Guilty when I want a break
- ☐ Feeling trapped
- ☐ Resentful that others don't help
- ☐ Dreading calls and new problems

Caregiving load (___ / 8)

- ☐ Managing most appointments
- ☐ Managing medications
- ☐ Managing bills and insurance
- ☐ Doing most transportation
- ☐ Doing bathing, meals, or daily care
- ☐ The main person everyone calls
- ☐ No reliable backup
- ☐ If I stop, everything falls apart

Safety concerns (___ / 8)

- ☐ Parent should not be left alone long
- ☐ Parent has fallen or nearly fallen
- ☐ Parent is confused about medication
- ☐ Parent is unsafe cooking, driving, or walking
- ☐ I've lifted my parent when it felt unsafe
- ☐ Afraid something bad happens if I leave
- ☐ Making more mistakes because I'm tired
- ☐ The current plan does not feel safe

Total score: _____ / 32

SCORE	WHAT IT MAY MEAN, AND THE NEXT STEP
0–7 · Watch & Plan	Managing for now. Keep a written care list, name one backup, review emergency contacts, organize documents.
8–15 · Stress Building	Hold a family meeting. Assign tasks. Ask what can come off the caregiver's plate. Consider private duty, adult day support, or a home health evaluation.
16–23 · High Risk	Schedule a family meeting soon. Ask for concrete help. Consider paid support. Review whether the parent is safe alone.
24–32 · Crisis	Do not wait. Call a family meeting. Contact the doctor, social worker, or care team. Consider urgent respite or a care-setting change. If anyone is in immediate danger, call 911.

The two most important questions: Is the caregiver safe? Is the person receiving care safe under the current plan? If the answer to either is no, the plan needs to change — not because anyone failed, but because the

need has outgrown the plan.

■ CAREGIVER LOAD LIST

Bring this to the family meeting. The goal is to make the invisible work visible, then assign a helper and a start date to each task.

TASK	CURRENTLY DONE BY	NEW HELPER / DATE
Appointments & doctor calls		
Medications & refills		
Bathing, dressing, daily care		
Meals, laundry, housekeeping		
Transportation		
Bills, mail, insurance		
Overnight supervision		
Emotional support & family updates		

What must come off the caregiver's plate this week? _____

ROB'S NOTE

I have met caregivers who could tell me every medication, every diagnosis, every appointment, and every change in their parent's condition.

Then I would ask, "How are you?" And the room would get quiet. Not because they did not know the answer — because no one had asked in a long time.

Caregivers often become the infrastructure of the family: the calendar, the pharmacy, the night watch, the emotional shock absorber. But even bridges have weight limits. A caregiver breaking down is not a private inconvenience. It is a care emergency.

If you are the caregiver, hear this clearly: you are allowed to need help before you collapse. You are allowed to say the plan is not working. You are allowed to love someone and still be exhausted.

CHAPTER 4 TAKEAWAY

Caregiver burnout is not weakness. It is information. The next right question is: **is the care plan protecting both the parent and the caregiver?**

PART 1 · SOMETHING HAS CHANGED

CHAPTER 5

The Family Meeting

THE PLAIN TRUTH

Most families do not need more opinions. They need a meeting.

Not a group text that turns into blame. Not a hallway argument at the hospital. Not five people saying, "Let me know what you need," and then disappearing into the wallpaper.

A real family meeting gives the family one place to name what has changed, what decisions need to be made, who is responsible for what, and what happens next. Caregiving gets harder when everything stays informal — one person knows the medications, another the finances, another has opinions but no assignments. A family meeting does not guarantee agreement. But it can move the family from reacting to deciding. The goal is not to make everyone happy. It is to protect the person receiving care, support the caregiver, and create the next clear plan.

WHAT FAMILIES USUALLY THINK

Families often think a meeting will make things worse. "We don't want drama." "My brother will never agree." "We can just text." Sometimes those concerns are real — family meetings can be emotional. But avoiding the conversation does not remove the problem. It only pushes the decision into a future crisis. A meeting is not about forcing everyone to feel the same way. It is about making the facts visible: what changed, what is unsafe, what the caregiver needs, what documents exist, what care options should be explored, and who is doing what this week.

When a Family Meeting Is Needed

Hold a family meeting when a parent has fallen, memory loss is affecting safety, medications are being missed, a caregiver is overwhelmed, living alone may not be safe, a hospital discharge or rehab discharge is coming, driving is becoming unsafe, the family disagrees, one person is doing almost everything, a care-setting change may be needed, hospice or palliative care has been mentioned, or the current plan no longer works. Do not wait until everyone agrees a meeting is necessary. The person carrying the most responsibility usually sees the need first. If that person says, "We need to talk," believe them.

Who Should Be There

Invite the people directly involved in care decisions: the parent (if appropriate and able), spouse or partner, adult children, the main caregiver, the healthcare and financial powers of attorney, a trusted family member, and, if involved, a care manager, social worker, clergy, hospice or palliative team member, or facility representative. Not everyone needs to be in every meeting. Some people create more heat than help. The guiding question is: who needs to know, who needs to decide, and who needs to help?

Should the Parent Be Included?

Whenever possible, the person receiving care should be included in decisions about their own life. They are not a project. They are a person. Consider whether they can follow the conversation, whether the meeting would overwhelm them, and whether a smaller conversation would be kinder. A parent may not manage every detail but can still answer what matters most: What matters most to you? Where do you feel safest? Who do you trust? What are you afraid of? What do you not want? Do not confuse limited capacity with no voice. Listen for what the person can still tell you.

Choose One Point Person

Every care plan needs one main point person. That does not mean they do all the work — it means they coordinate communication, so no one assumes someone else handled it. The point person keeps the medication list updated, communicates with doctors, shares updates, tracks appointments, and knows where documents are. They do not have to be the oldest child or the loudest voice. They need to be the person most able to help the plan hold together.

Separate Roles From Opinions

Everyone may have an opinion. Not everyone has a role. An opinion sounds like, "I think Mom should stay home," or "We should wait." A role sounds like, "I'll take Mom to the doctor Tuesday," or "I'll call three assisted living communities by Friday," or "I'll pay for two mornings of private duty care this week." Opinions without roles exhaust the caregiver. A family meeting should gently move people from commentary to contribution.

THE VELVET HAMMER

Use this sentence: **"Thank you for sharing your thoughts. What part of the plan can you take responsibility for this week?"** Soft handle, real impact.

The Family Meeting Agenda

Use a simple agenda. Do not let the conversation wander into every old family wound.

1. **What changed?** Name the reason for the meeting in one sentence.

2. **What is unsafe today?** Falls, wandering, missed medications, unsafe driving or cooking, caregiver exhaustion, no overnight coverage.
3. **What matters most to the person receiving care?** Name the priority — safety, comfort, or staying home as long as it is safe.
4. **What support is needed?** Family help, a home health evaluation, private duty, adult day care, assisted living, memory care, palliative care, hospice, or caregiver relief.
5. **Who is responsible for what?** Names. Dates. Deadlines. No fog.
6. **When will we meet again?** Set the next check-in. One meeting is not the whole plan.

Ground Rules, Siblings, Distance, and Money

Ground rules: speak respectfully, use facts not accusations, let the main caregiver speak without interruption, stay on the next decision, and if you have a strong opinion, take a role. If someone becomes cruel or disruptive, pause the meeting. A family meeting is not a fight to be won. The purpose is care.

When siblings disagree, return to facts: "Mom fell Monday and Friday." "Dad missed medication four times this week." "The caregiver has not slept through the night in three weeks." Facts do not remove emotion, but they give the conversation a floor.

The long-distance family member often underestimates the situation after a cheerful five-minute call. Ask better questions — Is she taking medication correctly? Has she fallen? Is the caregiver sleeping? — and offer specific help: bills, research, scheduling, paying for respite, or covering a weekend. Distance changes the kind of help you can give, not whether you help.

When money enters the conversation, start with clarity: what income, savings, and insurance exist; whether there is long-term care insurance or VA eligibility; who has authority to access financial information; and whether professional guidance is needed. Financial confusion damages both care and relationships. Do not let money conversations happen in the shadows.

When the Meeting Gets Emotional

It probably will. That does not mean the meeting failed. Tears do not mean the family is weak; silence does not mean nobody cares. These conversations are hard because they involve loss, fear, duty, money, old history, and love. When emotion rises, slow down and say, "Let's come back to the next decision," or "We do not have to solve the whole future today," or "What needs to happen in the next seven days?" That question pulls the conversation out of the storm and back onto the table.

WHAT TO DO THIS WEEK

1. **Name the reason for the meeting** in one sentence: "Mom has fallen twice, and we need a safer plan."

2. **Invite the right people** — decision-makers, caregivers, and those who need assignments.
3. **Bring written facts** — logs, medication lists, discharge papers, examples.
4. **Choose one point person** and a backup.
5. **Assign tasks** — each with a person, a deadline, and a next step.
6. **Schedule the next check-in** before the meeting ends.

■ WORKSHEET — THE FAMILY MEETING

Bring this to the meeting and fill it out as you talk.

Date of meeting _____

Person receiving care _____

Main caregiver _____

Family point person _____

Backup point person _____

Next meeting date _____

Why are we meeting?

What changed? What is unsafe? What decision cannot wait? _____

What matters most to the person receiving care? _____

People in the meeting

NAME	RELATIONSHIP	ROLE IN CARE

Decisions & task assignments

DECISION / TASK	PERSON RESPONSIBLE	DUE DATE
Call doctor / provider		
Update medication list		
Locate / update documents		
Research care options		
Give caregiver a break		

Documents to find or update

- | | |
|---|---|
| ☐ Healthcare Power of Attorney | ☐ Medication List & Doctor List |
| ☐ Financial Power of Attorney | ☐ Insurance / Medicare / Medicaid / VA information |
| ☐ Living Will / Advance Directive | ☐ Funeral home information |
| ☐ DNR / MOST / POLST (if applicable) | ☐ Emergency contacts |

This week's three actions

1. _____
2. _____
3. _____

Family agreement

We are writing this down so one person does not have to carry everything alone. The goal is not a perfect plan. It is to make the next week safer and clearer.

Main caregiver _____

Family point person _____

Family member / helper _____

Date _____

ROB'S NOTE

Family meetings are not easy because families are not spreadsheets.

They come with history. Old roles. Old injuries. The person who always fixes things tries to fix everything. The person who avoids conflict disappears. The person closest to the care may sound emotional because they are the one standing in the smoke.

That does not mean the meeting is useless. It means the meeting needs structure. Facts matter. Names matter. Dates matter.

The goal is not to win the family argument. The goal is to protect the person receiving care and stop one caregiver from carrying the whole house on their back. When in doubt, come back to one question: what needs to happen in the next seven days?

CHAPTER 5 TAKEAWAY

A family meeting makes the facts visible, assigns real tasks, and protects both the person receiving care and the caregiver. The next right question is: **who is doing what by when?**

PART 2 · CARE OPTIONS

CHAPTER 6

Home Health

Part 2 — Care Options. Once a family sees that something has changed, the next question is usually simple and overwhelming: what kind of help do we need? These chapters explain the major options — home health, private duty care, assisted living, memory care, skilled nursing and rehab, palliative care, and hospice — each in the same pattern, so families can compare without getting lost. You do not need to memorize every service. You need to know what each one is, what it is not, when it may help, what to ask, and what to do next.

“If you want to go fast, go alone. If you want to go far, go together.”

— AFRICAN PROVERB

THE PLAIN TRUTH

Home health is medical care provided in the home when a person meets the requirements and a clinician determines it is appropriate.

It may include skilled nursing, physical therapy, occupational therapy, speech therapy, medical social work, and limited home health aide support ordered as part of the care plan.

Home health can help after a fall, illness, surgery, hospitalization, new weakness, medication change, wound, disease flare-up, or decline in function. But it is not the same as private duty care. It is not long blocks of supervision, housekeeping, meal preparation, or someone sitting with your parent all day. That difference matters. Families often ask for home health when what they really need is daily help — and sometimes overlook home health when it could genuinely support therapy, nursing, teaching, or recovery at home. The goal of this chapter is to help you understand what home health may do, what it usually does not, and how to ask for an evaluation the right way.

Follow the Insurance

When my father was being referred to home health, a social worker asked an important question: "What insurance does he have?" At first, it seemed routine. But because my father was still working, Medicare was not his primary insurance. That detail mattered.

Years in healthcare taught me that not all insurance plans reimburse providers the same way. Some are easier to work with. Some pay differently. Some require additional approvals. During my career, I once

overheard a provider discussing a referral and saying something to the effect of: "I'll take this private-insurance patient if you send me your next two Medicare patients."

Whether said jokingly or seriously, it revealed something many families never see: behind every referral is a business reality. The lesson is not to become cynical. The lesson is to become informed. Ask questions. Why was this agency selected? What other agencies are available? Does this agency accept my insurance? Do I have a choice? Follow the insurance — not because money is more important than care, but because knowing how it works helps you make sure the care is right.

WHAT FAMILIES USUALLY THINK

Families often think home health means someone will stay with Mom, that Medicare will send an aide every day, that it will handle bathing, meals, and supervision, or that the doctor can just order it over the phone. Some of that comes from confusion, some from hope. Families are tired and scared. They hear "home health" and imagine a safety net large enough to catch everything. But home health is a specific service. It can be valuable and even powerful — but it is not every kind of help. A family needs to know the difference so they do not build a plan on the wrong promise.

What Home Health May Include

Skilled nursing — medication and disease teaching, wound care, symptom monitoring, safety teaching, caregiver instruction, and communication with the doctor.

Physical therapy — strength, balance, walking, transfers, fall risk, stairs, safe use of a walker or cane, and recovery after hospitalization or decline.

Occupational therapy — bathing, dressing, toileting, transfers, kitchen safety, energy conservation, equipment, and home setup for safer independence.

Speech therapy — swallowing concerns, communication, cognition, memory strategies, and changes after illness or stroke.

Medical social work — community resources, long-term planning, caregiver stress, advance care planning, and understanding care options.

Home health aide — limited personal care (such as bathing help) when it is part of the skilled plan of care. Aide visits are usually limited — not the same as daily private duty caregiving.

What Home Health Usually Does Not Provide

Home health usually does not provide all-day supervision, overnight or long shifts, housekeeping, meal preparation, transportation, grocery shopping, companionship, routine daily medication reminders without a skilled need, ongoing bathing help as the only need, sitting services so the caregiver can leave, or twenty-four-hour care. This is where families often get frustrated — they thought home health would

solve the whole home situation. It is not meant to replace family care, private duty care, assisted living, memory care, or long-term supervision. It may be one part of the care plan. It is rarely the whole plan.

Home Health Is Not Private Duty Care

The names sound similar; the services are different. A family may need both — home health to strengthen, teach, evaluate, and treat, and private duty to help with daily support and supervision. The question is not which one sounds better. It is: what does my parent actually need at home?

HOME HEALTH	PRIVATE DUTY CARE
Medical, intermittent, clinician-ordered	Non-medical daily help, often paid privately
Nursing, therapy, social work, limited aide	Bathing, meals, companionship, supervision
After illness, wounds, weakness, or decline	When the person cannot safely be alone
Not long blocks of supervision	Can cover longer blocks and overnight

When Home Health May Help

Ask about a home health evaluation after a fall, new weakness, trouble walking, surgery, hospitalization, rehab discharge, a new wound, medication changes, a new diagnosis, a disease flare-up, shortness of breath, increased confusion with a medical concern, or a decline in bathing, dressing, or transfers. Home health may also help when the family needs trained eyes inside the home — a therapist may notice unsafe transfers, a nurse may catch medication confusion, a social worker may see caregiver strain, an occupational therapist may spot a bathroom setup creating risk. Sometimes the home tells a different story than the doctor's office.

Home Health Requires Evaluation and Documentation

HOW TO ASK, THE RIGHT WAY

Home health is not usually something a family simply requests over the phone and automatically receives. For Medicare-covered home health, a clinician must determine whether the person meets the requirements — a skilled need, homebound status, and the required face-to-face encounter documentation. So the better request is not "Can you just order home health?" It is: **"Can we schedule an appointment or provider evaluation to discuss this recent change and determine whether home health is appropriate?"**

At the visit, bring clear examples — falls and near falls, new weakness, walking changes, trouble getting out of bed or chairs, medication concerns, bathing or dressing difficulty, recent hospitalization, wounds, shortness of breath, new confusion, and caregiver or home-safety concerns. Then ask: "Based

on what you are seeing, does my parent meet criteria for home health nursing, physical therapy, occupational therapy, speech therapy, social work, or other skilled services?" The goal is not to demand a service. It is to give the clinician enough information to evaluate whether home health is medically appropriate.

What "Homebound" Means in Plain English

Families often hear "homebound" and think it means the person can never leave the house. In plain language, homebound means leaving home is difficult, taxing, unsafe, or requires help, equipment, or considerable effort. A person may still leave for medical appointments or occasional reasons and still be considered homebound if they otherwise meet the requirements. Do not try to decide this alone — describe what is happening ("She needs help leaving the house," "He uses a walker and gets exhausted," "She cannot manage stairs safely," "Leaving home causes significant weakness or shortness of breath") and let the clinician evaluate it.

When Home Health Is Not Enough — or Not Ordered

Home health alone may not be enough when the person cannot safely be alone, dementia causes wandering, medications require daily supervision, the caregiver cannot sleep, falls continue despite therapy, or daily bathing, dressing, and toileting are unsafe. When it is not enough, the family may need private duty care, adult day care, assisted living, memory care, skilled nursing, palliative care, or hospice. That does not mean home health failed — it means the need is bigger than intermittent skilled visits.

Sometimes a clinician says home health is not appropriate — because the person does not meet the requirements (no skilled need, not homebound), or needs a different kind of care. If so, ask two questions: "What criteria are not being met?" and "What support do you recommend instead?" The answer "not home health" should not end the conversation. It should lead to the next question — outpatient therapy, private duty care, adult day care, a medication review, a specialist, a care manager, or a palliative or hospice evaluation if appropriate.

WHAT TO DO THIS WEEK

1. **Write down what changed** — use facts: "Mom fell twice in two weeks," "He gets short of breath walking to the bathroom."
2. **Decide whether there may be a skilled need** — fall risk, weakness, wounds, therapy, medication or disease teaching, caregiver teaching, or home safety.
3. **Ask for a provider evaluation** using the script in this chapter — not "order home health," but "determine whether home health is appropriate."
4. **Bring your notes** — fall log, medication list, discharge papers, symptoms, and safety concerns.

5. **Ask what services may be appropriate** — nursing, PT, OT, speech, social work, or aide as part of the plan.
6. **Make the home ready** — clear walkways, gather medications, and show the clinician the bathroom, bedroom, stairs, and walking paths.

■ WORKSHEET — HOME HEALTH EVALUATION

Use this before a provider visit or home health evaluation.

Person receiving care _____

Main caregiver / phone _____

Primary doctor / provider / phone _____

Preferred home health agency (if any) _____

Insurance _____

What changed? (check all that apply)

- | | |
|---|---|
| ☐ Fall or near fall | ☐ Medication concern |
| ☐ New weakness or trouble walking | ☐ Increased confusion |
| ☐ Trouble transferring | ☐ Shortness of breath or pain |
| ☐ Recent hospitalization or rehab stay | ☐ Difficulty bathing, dressing, or toileting |
| ☐ Recent surgery | ☐ Caregiver unable to manage safely |
| ☐ New wound | |

Current safety concerns

CONCERN	YES / NO / UNSURE	NOTES
Safe walking		
Safe transfers		
Safe bathing & toileting		
Medication safety		
Wound or skin concern		
Breathing / swallowing concern		
Confusion or memory concern		
Safe to be alone		

Current equipment in the home (check what applies)

- ☐ Cane
- ☐ Walker
- ☐ Wheelchair
- ☐ Shower chair
- ☐ Grab bars
- ☐ Raised toilet seat

- ☐ Bedside commode
- ☐ Hospital bed
- ☐ Oxygen
- ☐ Medical alert device
- ☐ Medication organizer

If home health starts

Agency name & phone _____

Start of care date _____

Primary nurse / therapist _____

Visit schedule _____

DISCIPLINE	NAME	PHONE
Nurse		
Physical therapist		
Occupational therapist		
Speech therapist		
Social worker		
Home health aide		

■ QUESTIONS FOR THE PROVIDER

1. Does my parent meet criteria for home health, and is there a skilled need?
2. Is my parent considered homebound, and was the required face-to-face encounter completed or scheduled?
3. Would nursing, physical therapy, occupational therapy, speech therapy, or social work help?
4. Would a home health aide be part of the care plan?
5. What documentation is needed?
6. What should we do if home health is not appropriate — what criteria are not being met?
7. What support is needed beyond home health?

■ HOME HEALTH SCRIPT

Calling the provider's office: "My parent has had a recent change in function and safety. We are seeing concerns with _____. Can we schedule an appointment or provider evaluation to determine whether home health is appropriate and whether the required documentation can be completed if they meet criteria?"

At the visit: "Based on what you are seeing today, does my parent meet criteria for home health services such as nursing, PT, OT, speech, social work, or aide support as part of the care plan?"

If the answer is no: "What criteria are not being met, and what support would you recommend instead?"

ROB'S NOTE

Home health can be one of the most helpful services a family receives — but only when the family understands what it is.

I have seen families feel disappointed because they expected someone to stay for hours, cook meals, supervise dementia, and take over the whole day. That is not home health.

I have also seen families miss the value of home health because they did not realize how much a nurse, therapist, or social worker could notice once inside the home. The doctor's office sees a patient sitting in a chair for fifteen minutes. Home health sees the narrow hallway, the bathroom without grab bars, the pill bottles on the counter, the caregiver lifting unsafely, the stairs that have become a mountain.

A home can look fine until someone tries to live safely inside it while weak, confused, short of breath, or recovering. Home health is not the whole answer. But sometimes it is the first trained set of eyes that helps a family understand what the next answer should be.

CHAPTER 6 TAKEAWAY

Home health is medical care in the home for people who meet the requirements — not private duty care, housekeeping, or all-day supervision. The next right question is: **can we schedule a provider evaluation to determine whether home health is appropriate, and what support is needed if it is not?**

PART 2 · CARE OPTIONS

CHAPTER 7

Private Duty Care

THE PLAIN TRUTH

Private duty care is non-medical help in the home. It may include help with bathing, dressing, meals, light housekeeping, companionship, transportation, errands, supervision, and safety checks.

Private duty care is often what families think home health will provide — but the two are different. Home health is medical, intermittent, and ordered when a person meets the requirements — Chapter 6 has the full rules. Private duty care is usually longer blocks of hands-on help and supervision, often paid privately (though long-term care insurance, VA benefits, or certain Medicaid programs may help depending on the situation). Private duty care can help a person stay home longer, and it can protect the caregiver from doing everything alone. The question is not, "Are we ready for caregivers?" It is: **what tasks are no longer safe or realistic for the family to handle alone?**

WHAT FAMILIES USUALLY THINK

Families often think private duty care means giving up. "We're not ready for strangers in the house." "Mom will never allow it." "Medicare should cover this." "We promised we'd keep her home." These concerns are common — care that enters the home and touches daily life can feel personal. But help at home is not failure. Sometimes it is the support that makes staying home possible. A caregiver coming in for a few hours may prevent a crisis, protect the main caregiver, and give the family enough breathing room to make better decisions.

The Yardstick: Activities of Daily Living

Professionals measure how much help a person needs using "activities of daily living" (ADLs) — the basic self-care tasks: bathing, dressing, eating, toileting, transferring in and out of a bed or chair, and continence. Alongside them are the "instrumental" activities of daily living (IADLs) — managing medications, handling money, cooking, shopping, using the phone, and getting around the community. Decline often shows up first in the instrumental tasks — a missed bill, a burnt pot — before it reaches the basic ones. Tracking ADLs and IADLs tells you where your parent actually stands. A steady rise in how much help they need signals that more in-home support, a different living arrangement, or a higher level of care may be needed soon.

Help only as much as needed. The instinct to take over completely is understandable, but doing too much can accelerate decline and wound dignity. If your parent can still button their own shirt slowly, let

them, even when it would be faster to do it for them. Reserve your help for what they truly cannot do safely — and protect dignity most carefully in bathing and toileting, where resistance is most common, especially with dementia. Go slowly, explain what you are doing, preserve privacy, and stay calm. A matter-of-fact, respectful manner does more than any technique.

What It May Include — and What It Usually Does Not

Private duty care may include bathing, dressing, grooming, toileting and incontinence care, meal preparation, light housekeeping, laundry, transportation, errands, companionship, dementia supervision, medication reminders (depending on state rules and agency policy), overnight care, and respite for family caregivers. Services vary by agency, caregiver training, and state rules. Always ask what the agency can and cannot do — do not assume.

It usually does not include skilled medical care unless the agency specifically provides licensed nursing. Many private duty caregivers cannot give injections, manage complex wounds, adjust medications, provide skilled nursing assessments, or replace a doctor, nurse, therapist, or hospice team. Some agencies offer licensed nursing separately; some do not. Ask directly.

Private Duty Care vs. Home Health

Families often mix these up (Chapter 6 draws the full distinction), and a family may need both — home health physical therapy helps Dad regain strength after a hospital stay, while private duty care helps him shower safely, eat lunch, and avoid being alone during the day. One supports medical recovery; the other supports daily life.

HOME HEALTH	PRIVATE DUTY CARE
Medical and intermittent	Non-medical daily help and supervision
Nursing, PT, OT, speech, social work, limited aide	Bathing, meals, errands, companionship, overnight care
Requires eligibility and clinical documentation	Often private pay; can be scheduled for longer blocks
Not long blocks of daily supervision	Can cover mornings, evenings, overnight, and respite

When Private Duty Care May Help

It may help when the person needs help with bathing, dressing, or meals; needs supervision or transportation; should not be alone for long; is at risk for falls; has dementia or wanders; forgets meals or medications; is lonely; needs overnight support; is coming home after a hospital or rehab stay; or has a caregiver who needs relief. It may also help when the family is deciding whether home is still realistic — a few weeks of help can reveal whether the person can manage with support, whether the caregiver is

sleeping, whether falls are reduced, and whether the cost is sustainable. Private duty care can be a bridge. Sometimes it becomes the long-term plan; sometimes it reveals that more care is needed.

How Much Care Is Needed?

Families often underestimate, starting with "Can someone come once a week?" when the real need is daily bathing help, meal setup, medication reminders, and supervision during unsafe hours. Match the schedule to the risk, not to the hope that everything will be fine. Ask when risk is highest — morning (getting up, bathing, dressing, medications), midday (missed meals, loneliness, wandering, falls), evening (worsening confusion, unsafe cooking), and overnight (getting up, wandering, toileting).

Cost and Who Pays

Private duty care can become expensive, so families need clear numbers. Ask about the hourly rate, minimum shift length, weekend and holiday rates, overnight billing, deposits and fees, cancellation policy, and what happens if a caregiver calls out. It is often paid privately — from savings, retirement income, family contributions, long-term care insurance, certain VA benefits, or some Medicaid waiver or state programs depending on eligibility. **Medicare generally does not pay for long-term custodial private duty care.** This is one of the most important points: if the need is daily supervision, bathing, meals, and companionship, do not assume Medicare will cover it. Ask specific questions before building the plan.

Choosing an Agency — or Hiring Privately

A good agency is more than a warm body in the house. Ask about licensing, background checks, training, dementia and fall-prevention experience, caregiver supervision, backup coverage, care plans, documentation, medication and transportation policies, minimum hours, rates, and how caregiver matching and replacement work. The cheapest option is not always the safest, and the most expensive is not automatically the best — look for reliability, communication, supervision, and fit.

Some families hire privately instead. That may lower cost and offer flexibility, but it can also mean no backup if the caregiver is sick, payroll and tax responsibilities, liability concerns, less supervision, and difficulty finding a replacement. A friendly arrangement can become complicated once money, bathing help, medications, keys, and family trust are involved. When in doubt, ask an elder law attorney, accountant, or care manager what risks to consider.

Introducing Help When a Parent Refuses

Many parents refuse at first — "I don't need a babysitter," "I don't want strangers here." Do not begin with the word "caregiver" if it creates resistance. Try softer, practical language: "Someone is coming Tuesday to help with lunch and laundry" instead of "We're hiring a caregiver"; "We're trying extra help for two mornings so everyone feels safer" instead of "You can't be alone"; "Let's try it for two weeks and

see what helps" instead of "This is permanent." The first goal may not be full acceptance. The first goal may be getting through the door.

When Private Duty Care Is Not Enough

It may not be enough when the person needs two-person transfers, is unsafe even with scheduled visits, has severe dementia behaviors or frequent wandering, needs twenty-four-hour supervision that is not financially sustainable at home, or when the caregiver cannot continue despite the help. At that point, families may need to consider assisted living, memory care, skilled nursing, palliative care, or hospice if appropriate. Changing the plan does not mean private duty care failed. It means the needs changed or became clearer.

WHAT TO DO THIS WEEK

1. **List the daily tasks that are no longer safe or realistic** — bathing, meals, transportation, supervision, overnight safety, caregiver respite.
2. **Identify the highest-risk times of day** — morning, midday, evening, overnight, weekends, shower days.
3. **Decide how many hours to try first** — often a few hours a few days per week to start.
4. **Call at least three agencies** and compare rates, minimums, training, and backup coverage.
5. **Decide who will manage the schedule** and name a backup.
6. **Review after two weeks** — is the parent safer, is the caregiver getting relief, are more or fewer hours needed?

■ WORKSHEET — PRIVATE DUTY CARE PLANNING

Use this before calling agencies or hiring help.

Person receiving care _____

Main caregiver / phone _____

Family point person / phone _____

Care schedule manager / phone _____

Current care needs

NEED	HOW OFTEN?	WHO DOES IT NOW?
Bathing / dressing / toileting		
Meals		
Laundry / cleaning		

Transportation / errands		
Medication reminders		
Companionship / supervision		
Overnight safety		
Dementia support		
Caregiver respite		

Highest-risk times of day (check all that apply)

- | | |
|------------------------------------|--|
| ☐ Morning | ☐ Weekends |
| ☐ Midday | ☐ Shower days |
| ☐ Afternoon | ☐ Appointment days |
| ☐ Evening | ☐ When caregiver is working |
| ☐ Overnight | ☐ When caregiver needs rest |

Best schedule to try first (check one)

- | | |
|---|---|
| ☐ 2–4 hours, 1–2 days per week | ☐ Overnight care |
| ☐ Morning help several days per week | ☐ Daily care |
| ☐ Evening help several days per week | ☐ 12-hour shifts |
| ☐ Weekend respite | ☐ 24-hour care |

Preferred start date _____

Preferred days & times _____

Minimum hours needed _____

Caregiver preferences

Preferred caregiver gender (if any) / language _____

Dementia experience needed? (yes / no / unsure) _____

Transfer or mobility help needed? (yes / no / unsure) _____

Transportation needed? (yes / no / unsure) _____

Pet & household notes _____

Agency comparison

QUESTION	AGENCY 1	AGENCY 2	AGENCY 3
Agency name & phone			

Hourly rate			
Minimum hours			
Weekend / overnight rate			
Background checks?			
Dementia training?			
Backup coverage?			
Care plan provided?			
Start date available			

■ QUESTIONS TO ASK A PRIVATE DUTY AGENCY

1. Are you licensed or registered as required in this state, and are caregivers employees or contractors?
2. Do you perform background checks, and what training do caregivers receive (including dementia)?
3. Can caregivers help with bathing, toileting, transportation, and medication reminders — and what are they not allowed to do?
4. What is the minimum shift length, the hourly rate, and are nights, weekends, or holidays billed differently?
5. What happens if the caregiver calls out, and can we request a different caregiver if needed?
6. Who supervises the caregiver, and how do you communicate with the family?
7. Do you help with long-term care insurance paperwork, and how quickly can care start?

■ HOME SETUP NOTES FOR THE CAREGIVER

Emergency contact _____

Primary doctor / preferred hospital _____

Pharmacy _____

Allergies _____

Mobility & bathroom safety notes _____

Meal preferences _____

Dementia / behavior notes _____

Things that calm the person _____

Things that upset the person _____

House rules _____

First-week care plan

TASK	PERSON RESPONSIBLE	DATE
List care needs & call agencies		
Compare costs & review budget		
Choose agency & schedule first shift		
Prepare home notes & update family		
Review after first week		

Two-week review

Is the current plan working? Yes ☐ No ☐ Somewhat ☐ Unsure ☐

What needs to change?

- | | |
|--|---|
| ☐ More hours | ☐ More family help or supervision |
| ☐ Fewer hours | ☐ Assisted living should be explored |
| ☐ Different caregiver or agency | ☐ Memory care should be explored |

Final decision — agency chosen & start date _____

Schedule _____

Main goal of private duty care _____

Next review date _____

■ INTRODUCTION SCRIPT — WHEN A PARENT RESISTS

"We are going to try having someone come in to help with _____. This is not about taking over. It is about making the day easier and safer. We'll try it for _____ and then see what is working."

If they say, "I don't need help," try: "I understand. This is also to help me — I need some support so I can keep doing this well."

If they say, "I don't want a stranger," try: "That makes sense. We'll meet them together, and if it's not a good fit, we'll speak up."

ROB'S NOTE

Private duty care can be hard for families because it feels like crossing a line.

Before this, care may have stayed inside the family. Then suddenly someone from the outside is helping with showers, meals, laundry, and the private rhythms of daily life. That can feel uncomfortable.

But I have seen private duty care give families their breath back. The daughter who has not slept gets a morning off. The spouse who is afraid to leave can go to the doctor. The parent who was skipping meals starts eating lunch again.

It is not always perfect. The first caregiver may not be the right match. The cost may be heavier than expected. But bringing in help is not a failure of love. It may be what keeps love from becoming exhaustion. The right question is not, "Should we be able to do this ourselves?" It is, "What support would make this safer for everyone?"

CHAPTER 7 TAKEAWAY

Private duty care is daily support, supervision, and non-medical help in the home. The next right question is: **what tasks are no longer safe or realistic for the family to carry alone?**

Assisted Living

THE PLAIN TRUTH

Assisted living is a residential care setting — not a nursing home, not memory care, not home health. It may help with meals, medications, bathing, dressing, laundry, housekeeping, safety checks, transportation, activities, and daily support.

Families usually consider assisted living when living alone no longer feels safe, when daily care has become too much, or when the caregiver can no longer carry it. The goal of this chapter is to help you compare communities clearly, ask better questions, and avoid choosing based only on the lobby, the brochure, or a first impression. **The lobby is not the care plan. The staffing, the medication process, and the fall response are the care plan.**

WHAT FAMILIES USUALLY THINK

Families often think a move to assisted living means giving up, or that Medicare will pay for it. Both are usually untrue. Assisted living is generally paid privately, though long-term care insurance, certain VA benefits, and some state programs may help. And moving is not surrender — for many families it means more safety, more connection, and staff available day and night. It can also give the family back its relationship, so the daughter can go back to being a daughter instead of a full-time nurse.

Is the Current Home Plan Still Working?

Consider assisted living when living alone no longer feels safe, falls or near falls are happening, meals are inconsistent, medication is missed, bathing has become irregular, the caregiver is exhausted, family support is unreliable, private duty care is becoming too expensive to manage, the person is isolated, or a spouse caregiver can no longer safely continue. If your family cannot answer, "Who is with them at the key times?" — that question alone is worth a conversation.

Match the Setting to the Need

Before touring, get honest about how much help the person needs — with bathing, dressing, toileting, transfers, walking, meals, medication, and nighttime support. Assisted living can meet many needs, but not all. Two-person transfers, complex medical care, severe dementia behaviors, or frequent wandering may require memory care or skilled nursing instead. Ask each community directly what care needs they

can and cannot manage, and what would trigger a required move-out or transfer. A community that answers that honestly is telling you something good about itself.

Costs, Honestly

Ask for a written cost sheet. Understand the base monthly rent, what is included, what costs extra, how care levels are priced, the one-time community fee, and how often rates can increase. Ask what happens — financially and practically — if care needs increase later, whether long-term care insurance and VA benefits are accepted or coordinated, and what payment is due before move-in. A plan that works emotionally but fails financially will not hold. Get the numbers in writing before you fall in love with a place.

Tour With Your Eyes Open

Brochures sell the lobby. You are buying the care. On a tour, watch how staff treat residents, whether the building feels clean and calm, whether residents seem engaged, and how quickly call buttons are answered. Ask to see care areas and resident spaces, not just the dining room and amenities. Notice the red flags — rushed or dismissive staff, vague answers about staffing or cost, pressure to sign quickly, no clear answer about falls or medication — and the green flags — staff who greet residents by name, costs explained in writing, honest talk about care limits, and a clear family-communication process. The worksheet in this chapter gives you the full question list to bring with you.

WHAT TO DO THIS WEEK

1. **List the care needs** — be honest about bathing, transfers, medication, nighttime, and memory support.
2. **Call and schedule tours** at three communities if possible.
3. **Get costs in writing** — base rate, care levels, extra fees, and how rates increase.
4. **Tour with the worksheet** and take notes at each community while it is fresh.
5. **Talk with your parent**, if appropriate, about what matters most to them in where they live.
6. **Hold a family meeting** (Chapter 5) to compare options and decide the next step.

■ WORKSHEET — ASSISTED LIVING TOUR & DECISION

Use this to compare communities and decide whether the current home plan is still safe. Tour at least three if you can.

Person receiving care _____

Main caregiver / phone _____

Family point person / phone _____

Current living situation _____

Preferred move-in timeframe / budget range _____

Is the current home plan still working? (check all that apply)

- | | |
|--|---|
| ☐ Living alone no longer feels safe | ☐ The caregiver is exhausted |
| ☐ Falls or near falls are happening | ☐ Private duty care is too expensive or hard to manage |
| ☐ Meals are inconsistent | ☐ The person is isolated or lonely |
| ☐ Medication is being missed or taken incorrectly | ☐ Driving is no longer safe |
| ☐ Bathing or hygiene is not happening regularly | ☐ A spouse caregiver cannot safely continue |

What does the person need help with?

NEED	NO HELP	SOME HELP	A LOT
Bathing / dressing / grooming			
Toileting / incontinence care			
Walking / transfers			
Meals			
Medication			
Housekeeping / laundry			
Transportation			
Nighttime support			
Memory support			

Community comparison

QUESTION	COMMUNITY 1	COMMUNITY 2	COMMUNITY 3
Community name & phone			
Base monthly cost			
Care level cost			
Medication cost			
One-time community fee			
Room type / move-in date			

Nurse on site / on call?			
Staff overnight?			
Medication management?			
Transportation included?			
Laundry / housekeeping included?			
Memory care available?			
Hospice / home health allowed?			
Pets allowed?			

■ QUESTIONS TO ASK ON THE TOUR

Care & staffing

1. What levels of care do you provide, and how do you decide what someone needs?
2. How often are care needs reassessed, and what happens if they increase?
3. What care needs would require a move-out or transfer?
4. Can you support bathing, toileting, incontinence, and transfers (including two-person)?
5. How many caregivers are on during the day, evening, and overnight?
6. Is a nurse on site or on call, and who manages medications?
7. How quickly are call buttons answered, and who should family call after hours?

Safety, cost & communication

1. How do residents call for help, and how are falls handled and reported to family?
2. How are medication changes, missed doses, or refusals handled?
3. What is the base rent, what is included, what costs extra, and how often can rates increase?
4. Is long-term care insurance or VA benefit accepted or coordinated?
5. Who is the main family contact, and how are concerns and care conferences handled?
6. When do you call the family, and when do you call 911?

Red flags (check any you notice)

- | | |
|---|--|
| ☐ Staff seem rushed, cold, or dismissive | ☐ Vague answers about staffing or cost |
| ☐ Residents appear ignored or distressed | ☐ Pressure to sign quickly |
| ☐ Strong odors or poor cleanliness | ☐ No clear answer about falls or medication |

☐ Tour skips care areas and resident spaces

☐ Unclear family-communication process

Green flags (check what reassures you)

☐ Staff greet residents warmly by name

☐ Medication and fall processes are clear

☐ Building feels clean, calm, and safe

☐ Family-communication process is clear

☐ Costs explained in writing

☐ Leadership is available and responsive

☐ Care limits discussed honestly

☐ You are not pressured to decide immediately

Decision summary

Community being considered _____

Main reasons this may be a good fit _____

Main concerns _____

Questions still unanswered _____

Family decision: Not a fit ☐ Possible fit ☐ Strong fit ☐ Need another tour ☐ Need financial review ☐ Compare with memory care ☐

The next right action is _____

Person responsible _____

Due date _____

ROB'S NOTE

Families often tour a community and fall in love with the chandelier in the lobby.

But your parent is not going to live in the lobby. They are going to live in the hallway, the dining room, the shower, and the moment they press a call button at 2 a.m.

So watch the hallway. Watch how staff talk to the people in wheelchairs. Watch how long the call light stays on. Ask the hard questions about falls and medication and what happens when care needs grow — and notice whether they answer you honestly or steer you back to the amenities.

The best communities will tell you what they cannot do. That honesty is worth more than any chandelier. You are not choosing a building. You are choosing the people who will show up for your parent when you cannot be in the room.

CHAPTER 8 TAKEAWAY

Assisted living is a care plan, not a lobby. The next right question is: **can this community safely meet the needs my parent has today — and be honest about the needs it cannot?**

PART 2 · CARE OPTIONS

CHAPTER 9

Memory Care

THE PLAIN TRUTH

Memory care is a residential setting designed for people with dementia or Alzheimer's who need more structure, supervision, and safety than regular assisted living can provide.

It is not a punishment. It is not "locking someone away." It is not giving up.

At its best, memory care provides a safer environment, predictable routines, dementia-trained staff, secured areas, medication support, meals, bathing help, activities, redirection, and supervision.

Families often wait too long because the decision feels painful — that is understandable. But dementia can create risks love alone cannot manage: a stove left on, walking out the door at night, refusing medication, falling because they cannot remember the walker. The question is not, "Are we taking away their freedom?" It is: **what setting gives them the safest, calmest, most supported life now?**

WHAT FAMILIES USUALLY THINK

Families often think memory care means the end. "Mom is not that bad." "Dad still knows who I am." "She can still have a normal conversation." But dementia is not measured only by whether someone recognizes family. A person can smile, tell old stories, and recognize loved ones while also being unsafe with medication, cooking, wandering, driving, bathing, or nighttime confusion. Memory care is not about whether the person still has moments of clarity. It is about whether the current setting can safely support them during the moments when clarity disappears.

Memory Care vs. Assisted Living

Assisted living and memory care may be in the same building, but they are not the same level of support. Assisted living may work when a person needs help with meals, bathing, dressing, medications, and general safety checks but can still live safely in a less secure setting. Memory care may be needed when dementia creates safety risks that require more structure — secured or monitored exits, dementia-trained staff, more frequent cueing, support for wandering, and calm redirection for confusion or agitation. Ask one question: **is this person unsafe because they cannot remember how to stay safe?** If yes, memory care belongs in the conversation.

When Memory Care May Be Needed

Consider memory care when the person wanders or tries to leave, gets lost, is unsafe with the stove or medication, falls or forgets the walker, cannot reliably call for help, is confused or frightened at night, becomes suspicious or agitated, refuses bathing or medication, or needs supervision throughout the day — or when assisted living or home care is no longer enough and the caregiver can no longer safely continue. Memory care is usually not about one bad day. It is about a pattern of risk. Write down the pattern. Families make better decisions when they can see what is actually happening.

The Wandering Risk

Wandering is one of the clearest reasons to consider memory care, and it does not always look dramatic at first — walking outside "just for a minute," trying to go home while already home, leaving at night, looking for a deceased spouse, exiting during appointments, or becoming restless in the evening. A person who wanders may not understand danger; they may be looking for something that feels real to them. Arguing usually does not fix it, and instructions may not be remembered ten minutes later. If wandering is happening, ask whether the person can safely be alone, whether the caregiver can sleep, and whether a secured memory care setting is needed. Wandering turns a home into a watchtower — families need to be honest about whether they can keep watch.

Nighttime, Behavior, and Caregiver Collapse

Nighttime often reveals how unsafe the plan has become — getting up repeatedly, wandering, falling on the way to the bathroom, not recognizing home, or the caregiver no longer sleeping. Dementia can also change mood and behavior: agitation, suspicion, hallucinations, refusal of care, or fear of being alone. Behavior is communication and can have treatable causes — pain, infection, constipation, dehydration, poor sleep — so report sudden changes to a clinician and do not assume everything is "just dementia." And memory care is sometimes needed not only because the person is declining but because the caregiver is. A care plan that depends on one exhausted caregiver staying awake forever is not a plan. It is a fuse.

How to Talk About It

Families often avoid the phrase "memory care" because it feels harsh. Lead with honesty and gentleness. Instead of "We have to put you in memory care," try "We are looking at a place with more support and a safer routine." Instead of "You cannot live alone anymore," try "We are worried the current setup is not keeping you safe." With dementia, one conversation may not settle it — you may need repetition, reassurance, and professional support. Do not turn every conversation into a courtroom. The goal is care, not winning the argument.

Tour Beyond the Locked Door

Memory care is not just a secured door — it is the quality of care inside the door. On a tour, ask how they manage wandering and exit-seeking, how they handle agitation and refusal of care, how

medications and falls are managed, how families are updated, and what care needs would require a transfer out. Watch how staff speak to residents: do they rush, correct harshly, or redirect calmly? Do they know names? Green flags include warm, name-by-name greetings, a calm and structured environment, a safe supervised outdoor space, dementia-adapted activities, and staff who can clearly explain how they handle the hard days. A beautiful building does not guarantee good dementia care. The care culture does.

WHAT TO DO THIS WEEK

1. **Write down the safety risks** — wandering, unsafe cooking or medication, falls, nighttime confusion, refusing care, caregiver exhaustion.
2. **Ask whether assisted living is enough** or whether dementia-related risks need a secured setting.
3. **Tour more than one memory care community** and ask about the hard days, not just the lobby.
4. **Talk with the doctor or neurologist** about sudden changes and treatable causes.
5. **Hold a family meeting** (Chapter 5) focused on safety, caregiver capacity, and cost.
6. **Decide the next step** — more supervision, private duty, adult day care, assisted living, memory care, or a provider evaluation.

■ WORKSHEET — MEMORY CARE DECISION

Use this when dementia is creating safety concerns and the family is deciding whether memory care should be explored.

Person receiving care _____

Main caregiver / phone _____

Family point person / phone _____

Diagnosis (if known) _____

Primary doctor / neurologist / phone _____

Why are we considering memory care? (check all that apply)

- | | |
|--|---|
| ☐ Wandering or leaving home | ☐ Refusing care |
| ☐ Getting lost | ☐ Unsafe anger or agitation |
| ☐ Unsafe cooking | ☐ Suspicion or paranoia |
| ☐ Medication mistakes | ☐ Cannot call for help reliably |
| ☐ Unsafe driving | ☐ Caregiver exhaustion |
| ☐ Falls or near falls | ☐ Assisted living or home care is not enough |
| ☐ Nighttime confusion | |

Dementia safety snapshot

SAFETY AREA	SAFE	UNSAFE	NOTES
Medication			
Cooking			
Wandering / exits			
Bathing / toileting			
Eating & drinking			
Walking / falls			
Nighttime safety			
Ability to call for help			
Caregiver safety			

Wandering & nighttime notes

Has the person left or tried to leave home? (yes / no / unsure) _____

When does it happen, and where are they trying to go? _____

Is the person confused or unsafe at night? (yes / no / unsure) _____

Is the caregiver sleeping? (yes / no / unsure) _____

Behavior & triggers

Behaviors causing concern _____

Things that calm the person _____

Things that upset the person _____

Best time of day / hardest time of day _____

Memory care community comparison

QUESTION	COMMUNITY 1	COMMUNITY 2	COMMUNITY 3
Community name & phone			
Base monthly / care level cost			
Secure exits & outdoor space?			
Staff dementia training?			
Overnight staffing			

Nurse on site / on call			
Behavior support process			
Fall & medication process			
Family-communication process			
Care needs they cannot manage			

■ QUESTIONS TO ASK MEMORY CARE

1. How do you manage wandering or exit-seeking, and are exits secured or monitored?
2. What dementia training do staff receive, and how many are on during the day, evening, and overnight?
3. How do you handle agitation, fear, aggression, or refusal of bathing and medication — what non-drug approaches do you try first, and what is your approach to antipsychotic medications?
4. How are medications and falls managed, and how quickly is family notified of a change?
5. How are activities adapted for different stages of dementia?
6. How do you learn the resident's routines, history, triggers, and preferences?
7. What happens if care needs increase, and what would require a transfer or discharge?

Decision summary

Main reasons memory care may be needed _____

Main concerns _____

Questions still unanswered _____

Next step / person responsible / due date _____

■ CONVERSATION SCRIPT

With family: "We need to talk about memory care not because we are giving up, but because dementia is creating safety concerns. The question is whether the current setting can still protect safety, dignity, and the caregiver."

With the person, if appropriate: "We are looking at a place with more support and a safer routine. The goal is not to take your life away. It is to make sure your days are safer, calmer, and better supported."

ROB'S NOTE

Memory care is one of the hardest conversations families face, because it feels like a doorway that only swings one way.

Families often wait because they are trying to honor the person they love. I understand that. But dementia changes the rules quietly. At first the family is helping with little things. Then managing medications. Then hiding the car keys. Then sleeping with one ear open. Then afraid to shower because the person may walk out the door.

No one planned for that. Memory care should never be treated casually — but it should not be treated as failure either.

Sometimes the most loving thing a family can say is, "This is bigger than what we can safely manage at home." That sentence hurts. It can also protect everyone. The goal is not to preserve the old plan. It is to care for the person in front of you now.

CHAPTER 9 TAKEAWAY

Memory care may be needed when dementia creates safety risks that cannot be reliably managed at home or in regular assisted living. The next right question is: **is the current setting safe during the moments when memory cannot protect them?**

Skilled Nursing and Rehab

THE PLAIN TRUTH

Skilled nursing and rehab are often part of the care journey after a hospital stay, illness, surgery, fall, infection, stroke, or sudden decline.

Families may hear different phrases: "skilled nursing facility," "SNF," "rehab," "short-term rehab," "nursing home," "subacute rehab." The words can be confusing.

In plain language, skilled nursing and rehab may provide short-term medical care, nursing, and therapy when a person is not ready to safely return home after the hospital. The goal may be to help the person regain strength, improve walking, recover from illness, and prepare for the next setting. Sometimes that setting is home. Sometimes it is assisted living, memory care, or long-term care. Sometimes the family realizes during rehab that the old plan is no longer safe. That is why this chapter matters. Rehab is not just a place to "get stronger." It is a decision point. The better question is not, "When are they going home?" It is: **what level of care will be safe after rehab ends?**

WHAT FAMILIES USUALLY THINK

Families often think, "Rehab will get Mom back to normal," "Dad will stay until he is ready," "Medicare will cover everything," or "They cannot discharge her if we are not ready." Sometimes rehab helps a person recover well. But not every person returns to their prior level. Older adults can lose strength quickly during illness, hospitalization, bedrest, or confusion, and a person may improve and still need more help than before. Families get into trouble when they assume rehab will solve the whole situation. Rehab may answer important questions — but it may not give the answer the family hoped for.

What Rehab May Provide — and What It Does Not Mean

A skilled nursing or rehab facility may provide nursing care, medication management, physical, occupational, and speech therapy, wound care, monitoring after illness or surgery, help with bathing, dressing, toileting, and transfers, meals, discharge planning, social work, equipment recommendations, caregiver training, and coordination with doctors. The exact services depend on the person's needs, the facility, insurance coverage, and the care plan. Ask what is being provided — do not assume.

Rehab does not always mean the person will return home independent. It does not guarantee daily progress, a long stay, or that insurance will keep paying, and it does not replace family planning or

automatically solve dementia, frailty, advanced illness, caregiver burnout, or unsafe housing. Begin discharge planning early. That conversation should not start the day before discharge.

Short-Term Rehab vs. Long-Term Care

This distinction matters. Short-term rehab is usually focused on recovery after a hospital stay, surgery, illness, or fall; the goal is improvement, and the stay may be covered by insurance if requirements are met (coverage depends on the payer, medical need, eligibility, and plan rules). One protection families should know: **Medicare coverage does not legally require the person to keep improving.** Under the *Jimmo* settlement, skilled care to maintain function or to prevent or slow decline can also qualify. If you are told coverage is ending because the person has "plateaued," ask for the written notice and ask how to appeal. Long-term care is custodial or ongoing care for someone who cannot safely live alone over time; it is often paid differently, and **Medicare generally does not pay for long-term custodial nursing home care.** Ask early: "Is this stay considered short-term rehab, long-term care, or something else?" That one question can prevent a lot of confusion.

Two more protections. Before the skilled nursing stay, ask whether the hospital stay was **inpatient or observation** — it can decide Medicare coverage (Chapter 13 explains the trapdoor). And when you are told coverage or a stay is ending and you believe it is unsafe, Medicare patients have a formal fast-appeal right through your Medicare Quality Improvement Organization (QIO) — the written notice the facility hands you includes the phone number, and the appeal clock is short, so act the same day.

Rehab Is a Handoff, Not a Holding Place

Families sometimes think rehab buys time to avoid the next decision, but time moves quickly and discharge planning begins almost immediately. During rehab, ask: What could the person do before the hospital, and what can they do now? Are they improving and participating in therapy? What help do they need with bathing, dressing, toileting, walking, and transfers? Can they safely return home, and what equipment will be needed? Will home health be appropriate after discharge? Will private duty care be needed? Is assisted living, memory care, or long-term care more realistic? Is palliative care or hospice appropriate? A rehab stay should give the family information. Use it.

Therapy and Nursing: What to Ask

Ask the therapy team how far the person can walk and whether it is independent, with supervision, or hands-on; whether they need a walker, cane, or wheelchair; whether they can get in and out of bed, transfer to a chair, get on and off the toilet, manage stairs, and bathe and dress safely; whether they are improving; and what would make home unsafe. Ask for plain language. Not just "She is min assist," but **"What does that mean at home at 2 a.m. when one person is helping?"** That is the real question.

Ask the nursing team what medical needs are being managed, whether medications are stable, whether there is wound care, pain, confusion, incontinence, poor eating, or shortness of breath, and what needs

to be monitored after discharge. Nursing sees things therapy may not — therapy knows how the person walks, but nursing may know whether they are confused at night, refusing care, or eating poorly. Both views matter.

Social Work, Discharge Planning, and the Care Conference

The discharge planner or social worker is your key contact. Ask about the expected discharge date, the recommended next level of care, what equipment is needed, whether home health will be arranged if appropriate, whether private duty care is needed, what safety concerns exist about returning home, what documents need signing, and what the options are if home is not safe — including what happens if the family disagrees with the discharge plan. Do not wait for the discharge planner to call you. Ask for a care conference.

A care conference is a meeting of the rehab team — nursing, therapy, social work, sometimes a dietitian — with the family and the patient if appropriate. Ask for one early, and use it to learn what has and has not improved, what help the person still needs, the discharge goal and expected date, what setting is safest next, and what training the family needs before discharge. The care conference should produce a plan, not just reassurance.

When Going Home May Not Be Safe

Going home may not be safe if the person cannot transfer safely or get to the bathroom, falls or is at high risk for falls, needs more help than one caregiver can provide, needs two-person assistance, or lives in a home with stairs that cannot be managed. It may also be unsafe if dementia creates supervision needs that cannot be met at home, if medications or wounds require skilled care the family cannot provide, if the person is not eating or drinking safely, or if the caregiver cannot physically or safely continue. If home is not safe, that is not a failure — it is information. It means the next setting needs to match the care the person actually needs now.

WHAT TO DO THIS WEEK

1. **Ask whether this is short-term rehab or long-term care** and how it is being paid for.
2. **Request a care conference early** — do not wait for the discharge call.
3. **Get plain-language answers** from therapy and nursing about walking, transfers, toileting, bathing, and what home would require.
4. **Ask what happens after discharge** — home health evaluation, private duty care, equipment, and monitoring.
5. **Compare the recommended setting honestly** — home, assisted living, memory care, or long-term care.
6. **Hold a family meeting** (Chapter 5) before discharge, not after.

■ WORKSHEET — REHAB & DISCHARGE PLANNING

Bring this to the care conference and update it as discharge approaches.

Person receiving care _____

Facility & phone _____

Discharge planner / social worker _____

Expected discharge date _____

Recommended level of care after discharge _____

Before vs. now

ABILITY	BEFORE HOSPITAL	NOW
Walking (distance / help needed)		
Transfers (bed, chair, toilet)		
Bathing & dressing		
Toileting / continence		
Stairs		
Memory / confusion		
Eating & drinking		

Care conference questions (check when answered)

- ☐ What has improved, and what has not?
- ☐ What help does the person still need?
- ☐ What is the discharge goal and date?
- ☐ What setting is safest next?
- ☐ What equipment will be needed?
- ☐ Will a home health evaluation be arranged?
- ☐ Is private duty care needed?
- ☐ What training does the family need before discharge?
- ☐ What are the options if home is not safe?

Discharge plan

TASK	PERSON RESPONSIBLE	DUE DATE
Confirm next care setting		

Arrange equipment		
Set up home health evaluation (if appropriate)		
Arrange private duty care (if needed)		
Gather medication & doctor lists		
Schedule follow-up appointments		

■ QUESTIONS TO ASK BEFORE DISCHARGE

1. Is this stay short-term rehab or long-term care, and how is it paid for?
2. Can my parent transfer, toilet, walk, and bathe safely enough for the planned setting?
3. What would make home unsafe, and what would we need to make it work?
4. Will home health be evaluated, and what skilled needs qualify?
5. What medications, wounds, or conditions must be monitored after discharge?
6. What should we watch for, and who do we call if things get worse?
7. What happens if we do not feel the discharge is safe?

ROB'S NOTE

Rehab is one of the most misread moments in the whole journey.

Families hear the word "rehab" and picture their mother walking back through the front door the way she left. Sometimes that happens. Often it does not.

I have sat with families on day three of a rehab stay who were already being asked about discharge, and they had not even caught their breath from the hospital. That is why I tell people: rehab is not a pause. It is a countdown. Ask for the care conference early. Ask therapy what "min assist" means at 2 a.m. Ask nursing what the nights look like.

The goal is not to rush the person home to prove the crisis is over. The goal is to send them to the place that can actually keep them safe. Sometimes that is home with help. Sometimes it is not. Either way, you want to be the family that asked the hard questions early — not the one handed a discharge date with nowhere safe to go.

CHAPTER 10 TAKEAWAY

Rehab is a decision point, not just a place to get stronger. The next right question is: **what level of care will actually be safe after rehab ends?**

Palliative Care

THE PLAIN TRUTH

Palliative care is specialized support for people living with serious illness. It focuses on comfort, symptoms, stress, goals of care, and quality of life.

Palliative care is not the same as hospice. This is one of the most important distinctions in the whole toolkit.

Hospice is generally for people who are nearing the end of life and are no longer pursuing curative treatment for the terminal condition. Palliative care can often begin earlier — a person may receive it while still seeing specialists, taking medications, going to the hospital, or trying to manage a serious illness. It may help with pain, shortness of breath, fatigue, nausea, anxiety, appetite changes, medication burden, repeated hospitalizations, difficult decisions, and family stress. The better question is not, "Is this the beginning of the end?" It is: **would extra support help us manage symptoms, stress, and decisions?**

WHAT FAMILIES USUALLY THINK

Families often hear "palliative care" and think it means hospice, that death is close, that the doctor is giving up, or that treatment stops. This confusion causes families to miss helpful support. Palliative care is not a punishment or a signal that the family did something wrong, and it is not the same as stopping treatment. It is an extra layer of support for serious illness. Many families do not need fewer doctors — they need someone to help connect the dots and ask: What is the goal now? What symptoms are making life harder? What does the patient understand? What decisions are coming? What matters most if the illness gets worse?

What Palliative Care May Provide

Services vary by setting and program, but may include symptom management (pain, shortness of breath, nausea, fatigue), emotional support, goals-of-care conversations, medication review, care planning, family meetings, advance care planning, help understanding treatment options, coordination with other doctors, social work, and spiritual support if available. Palliative care may be provided in hospitals, clinics, nursing facilities, assisted living communities, or sometimes at home, depending on local programs — ask what services exist in your area.

What Palliative Care Usually Does Not Mean

It does not usually mean the person must stop all treatment, that they must be dying soon, that hospice is automatically starting, that the family failed, that the doctor is abandoning the patient, that comfort is the only goal, or that no more hospital visits or specialists are allowed. Palliative care is not a single decision. It is a support service and a conversation partner that helps families choose well as illness changes.

Palliative Care vs. Hospice

Families need this difference in plain language. Both care about comfort, dignity, communication, and support — what differs is timing and eligibility.

PALLIATIVE CARE	HOSPICE
Can often begin earlier in a serious illness	For those in the last stage who meet eligibility
May be used alongside treatment	Comfort-focused when cure is no longer the goal
Focuses on symptoms, stress, quality of life, goals	A full team and a specific benefit structure

IF THE FAMILY IS UNSURE

Ask: **"Are we talking about palliative care for serious illness support, or hospice care because the illness may be in its final stage?"** That question can clear the room.

When Palliative Care May Help

Consider asking about palliative care with advanced heart, lung, kidney, or liver disease, cancer, neurologic illness, dementia with increasing complications, repeated hospitalizations or ER visits, uncontrolled pain or shortness of breath, weight loss, declining function, difficult treatment decisions, family disagreement about goals, confusion about prognosis, or concern that treatments are becoming harder than the illness. It may be especially helpful when everyone is working hard but no one is asking what the patient actually wants most.

The Goals-of-Care Conversation

A goals-of-care conversation is not only about medical treatment — it is about what kind of life the person wants as illness changes. Important questions: What does the person understand about the illness, and what does the family understand? What matters most right now? What symptoms are hardest? What is the person hoping for, and what are they afraid of? What treatments feel acceptable, and what feel too burdensome? If time is shorter than the family hoped, what would make the days better? These

conversations are hard, but they are how a family makes sure the care matches the person — not just the disease.

WHAT TO DO THIS WEEK

1. **Write down current symptoms** and what helps or worsens each.
2. **List hospital and ER visits** in the last six months and what keeps sending you back.
3. **Ask the provider about palliative care** using the script in this chapter.
4. **Clarify palliative vs. hospice** — ask which one is being recommended and why.
5. **Review advance care documents** — power of attorney, advance directive, DNR/POLST if applicable.
6. **Invite key family members** into the conversation and, if needed, hold a family meeting.

■ WORKSHEET — PALLIATIVE CARE CONVERSATION

Use this to prepare for a palliative care conversation, ask better questions, and write down what matters most.

Person receiving care _____

Main caregiver / phone _____

Primary doctor / provider / phone _____

Main diagnosis or illness _____

Specialists involved _____

Why are we considering palliative care? (check all that apply)

- | | |
|--|---|
| ☐ Serious illness is harder to manage | ☐ Family members disagree about the plan |
| ☐ Pain or shortness of breath is difficult to control | ☐ Quality of life is declining |
| ☐ Fatigue or weakness is increasing | ☐ Caregiver stress is increasing |
| ☐ Appetite or weight loss is a concern | ☐ Advance care planning is needed |
| ☐ Repeated hospitalizations or ER visits | ☐ Hospice has been mentioned, but family is unsure |
| ☐ Treatment decisions are becoming confusing | |

Current symptoms (check any present)

- | | |
|--|--|
| ☐ Pain | ☐ Anxiety or depression |
| ☐ Shortness of breath | ☐ Confusion |
| ☐ Fatigue / weakness | ☐ Trouble sleeping |
| ☐ Nausea or vomiting | ☐ Difficulty swallowing |
| ☐ Poor appetite / weight loss | ☐ Swelling or wounds |

Daily life snapshot

AREA	BETTER / SAME / WORSE	NOTES
Walking / bathing / dressing		
Eating & sleeping		
Breathing		
Pain control		
Mood & memory		
Caregiver stress		

What matters most? (in the person's own words, if they can answer)

What matters most to you right now? _____

What are you hoping for? _____

What are you most afraid of? _____

What do you want to avoid if possible? _____

Who do you want involved in decisions? _____

■ QUESTIONS TO ASK THE PALLIATIVE CARE TEAM

1. What symptoms can you help manage, and how can we improve comfort and quality of life?
2. What should we understand about this illness, and what changes should we expect next?
3. Are current treatments helping, or becoming too burdensome?
4. How can we reduce unnecessary crisis trips to the hospital, and who do we call after hours?
5. What support is available for the caregiver, and can you help with advance care planning or a family meeting?
6. Is hospice appropriate now, or should we continue palliative care — and what would tell us it is time to consider hospice?

Advance care planning — documents to find or update

- | | |
|---|--|
| ☐ Healthcare Power of Attorney | ☐ Medication list & doctor list |
| ☐ Living Will / Advance Directive | ☐ Emergency contacts |
| ☐ DNR / MOST / POLST (if applicable) | ☐ Funeral home information (if known) |

With a doctor or care team: "We are trying to better manage symptoms, stress, and decisions related to this serious illness. Would palliative care be appropriate to help with comfort, quality of life, goals of care, and family support?"

If they say it is not needed: "What support would you recommend for symptom management, caregiver stress, and planning?"

If hospice is mentioned: "Can you explain whether you are recommending hospice now, or palliative care while treatment continues?"

ROB'S NOTE

Palliative care is one of the most misunderstood words in medicine.

Families hear it and brace like they have just been told the worst. But palliative care is not the end of the road. It is someone walking beside you on the road, asking the questions no one else has slowed down to ask.

I have watched a palliative care team change a family's whole experience — not by adding another treatment, but by finally asking, "What matters most to you now?" and then building the plan around the answer.

You do not have to be dying to deserve comfort. You do not have to give up to want less suffering. Palliative care is what it looks like to take symptoms, stress, and dignity seriously while there is still a lot of living to do.

CHAPTER 11 TAKEAWAY

Palliative care is extra support for serious illness — comfort and clarity alongside treatment, not instead of it. The next right question is: **would extra support help us manage symptoms, stress, and decisions?**

Hospice

THE PLAIN TRUTH

Hospice is not giving up. Hospice is comfort-focused care for people who meet eligibility requirements and are nearing the end of life.

It is the most active form of love available. It says: I will not let you be in pain. I will not let you die alone. I will show up every day until the last one. Hospice brings a team — nurse, aide, social worker, chaplain, volunteers, and a medical director — around the patient and the family, with a focus on comfort, dignity, and quality of life when the goal is no longer curing the illness. Families often wait too long to ask about it because the word frightens them. But hospice is not the family choosing death. It is the family choosing support when the illness is already changing life. The better question is not, "Are we giving up?" It is: **would comfort-focused care help us care better now?**

WHAT FAMILIES USUALLY THINK

Families often think hospice means death is days away, that all treatment and medication stop, that they are abandoning their loved one, or that choosing hospice means they have failed. None of that is true. Hospice continues care — it simply changes the goal from cure to comfort. One thing does change, and families should know it going in: **while on hospice, Medicare generally stops paying for treatment intended to cure the terminal illness.** Care for other health conditions continues, comfort treatment continues, and the person can leave hospice at any time and return to regular Medicare coverage. Many families later say the same thing: "I wish we had called sooner." They discover that hospice did not shorten the time. It made the time better — the pain controlled, the fear lowered, the family finally able to be family again instead of a frightened medical crew.

What Hospice Provides

Hospice usually provides a team and a plan of care: regular nurse visits for symptom and comfort management, aide visits for personal care, a social worker for support and planning, chaplain or spiritual care if wanted, volunteers, and bereavement support for the family after death. It typically covers medications related to the terminal condition, equipment such as a hospital bed or oxygen, and supplies tied to the plan of care. Hospice can be provided wherever the person lives — at home, in assisted living, in memory care, or in a skilled nursing facility — and inpatient hospice may be available when symptoms require it. Services and visit frequency vary by agency and plan of care, so ask what each agency actually provides.

What Hospice Does Not Mean

Hospice does not usually mean a full-time bedside caregiver. This surprises families. Hospice supports the family and manages comfort, but it does not typically sit at the bedside twenty-four hours a day — the family, and sometimes private duty caregivers, still provide much of the hands-on daily care.

Hospice also does not mean the person can never change their mind. If someone improves or wants to pursue treatment again, they can leave hospice. And it does not mean stopping all care — comfort medications, personal care, and support continue. Knowing this early lets a family plan the hands-on coverage they will actually need.

Two Things Families Get Backwards

First, families fear hospice will be a financial burden. Under the Medicare Hospice Benefit it is often the opposite — it can cover the full hospice team, medications for pain and symptom relief, medical equipment and supplies, short-term inpatient and respite care when the team arranges it, and grief support for the family, with families usually paying almost nothing (often just a small copay of up to \$5 per comfort prescription and a small share of respite-stay costs — many hospices charge nothing at all). The one real gap is room and board if the person lives in a facility, which continues separately. Coverage always depends on eligibility and the plan, so confirm the specifics — but do not let the fear of cost keep you away, because for most families that fear is exactly backwards.

Second, families think hospice is a one-way door with a hard six-month clock. Hospice is generally for a prognosis of about six months or less, but it is reversible and renewable: a person can leave hospice and return, and the benefit can continue past six months with recertification if they remain eligible. There is also a level of care most families never hear about — continuous or "crisis" care, where a nurse can stay at the bedside for hours during a hard stretch of symptoms. If symptoms spike, ask for it by name. It is short-term and criteria-based — the hospice determines whether a crisis level of care is met — so ask what the criteria are when you call.

When to Ask About Hospice

Consider asking for a hospice evaluation when a serious illness is progressing and you are seeing a pattern of decline — more hospitalizations or ER visits, more weakness and time in bed, more sleeping, less eating and drinking, weight loss, more falls or infections, more confusion, more shortness of breath, wounds that will not heal, difficulty swallowing, or treatment that is no longer helping or has become too hard on the person. Also consider it when the doctors have mentioned comfort care, when a palliative team has raised it, or when the family finds itself quietly asking, "What are we doing this for?" Write down the pattern with specific examples. Patterns, not single bad days, tell the real story.

The Question Families Are Afraid to Ask

If someone is afraid that hospice means giving up, the words that help are simple: **"We are not trying to give up. We are trying to understand whether comfort-focused support would help now."**

Asking for a hospice evaluation does not commit anyone to anything. It is a conversation and an assessment. The family always chooses. And choosing hospice is not abandoning the person — it is deciding that their comfort, their dignity, and their time matter more than one more hard treatment that may not help.

WHAT TO DO THIS WEEK

1. **Write down the decline with specific examples** — eating, weight, walking, sleeping, breathing, confusion, infections, hospital visits.
2. **Ask the provider for a hospice evaluation** using the script in this chapter — an evaluation commits you to nothing.
3. **Call more than one hospice agency** and compare team, visit frequency, after-hours support, and services.
4. **Plan the hands-on caregiving coverage** — hospice supports the family; it is usually not a full-time bedside caregiver.
5. **Gather the documents** — power of attorney, advance directive, DNR/POLST, medication and doctor lists.
6. **Name a family point person** and set how the family will be kept updated.

■ WORKSHEET — HOSPICE QUESTIONS & DECISION

Use this when hospice has been mentioned or a serious illness is progressing, to understand whether hospice may be appropriate, what to ask, and who will provide care.

Person receiving care _____

Main caregiver / phone _____

Primary doctor / provider / phone _____

Main diagnosis or illness _____

Current location (home, AL, memory care, SNF, hospital) _____

Why are we considering hospice? (check all that apply)

- | | |
|---|--|
| ☐ Serious illness is progressing | ☐ More confusion or shortness of breath |
| ☐ More hospitalizations or ER visits | ☐ Wounds not healing; difficulty swallowing |
| ☐ More weakness / time in bed / sleeping | ☐ Treatment is no longer helping or is too hard on the person |
| ☐ Less eating or drinking; weight loss | |
| ☐ More falls or infections | |

☐ Doctors or palliative team have mentioned comfort care

☐ The person wants comfort-focused care

Decline snapshot (use specific examples — patterns matter)

AREA	WHAT HAS CHANGED?	WHEN DID IT START?
Eating & drinking		
Weight		
Walking & daily activities		
Sleeping & breathing		
Pain & confusion		
Falls & infections		
Hospital / ER visits		

What matters most now? (in the person's own words, if they can answer)

What matters most to you right now? _____

What are you hoping for? _____

What are you afraid of? _____

Where do you feel safest? _____

What do you want to avoid if possible? _____

Who do you want involved in decisions? _____

Who will provide hands-on care?

Hospice supports the family but does not usually provide a full-time bedside caregiver. Plan who covers each need.

CARE NEED	WHO WILL HELP?	PHONE
Daytime caregiver		
Overnight caregiver		
Medication support		
Bathing & toileting		
After-hours call person		
Backup caregiver		

Documents to have ready

- | | |
|--|--|
| ☐ Insurance cards | ☐ DNR / MOST / POLST (if applicable) |
| ☐ Medication list & doctor list | ☐ Recent hospital or rehab papers |
| ☐ Healthcare Power of Attorney | ☐ Emergency contacts |
| ☐ Living Will / Advance Directive | ☐ Funeral home information (if known) |

■ HOSPICE EVALUATION & AGENCY QUESTIONS

1. Does my loved one appear hospice appropriate, and for what diagnosis?
2. What decline and symptoms are you seeing, and what would hospice help with most?
3. What treatments would continue, and what medications, equipment, and supplies would hospice provide?
4. How often would the nurse and aide visit, and what support is available after hours?
5. Who do we call in a crisis at night, and what would make inpatient hospice appropriate?
6. What hands-on care is the family still responsible for, and would private duty care still be needed?
7. Can hospice support care at home, assisted living, memory care, or skilled nursing?
8. What happens if my loved one improves, or if we change our mind?

■ HOSPICE REQUEST SCRIPT

With a doctor, facility nurse, or hospice agency: "We are seeing decline and are wondering whether hospice would be appropriate. Can we talk honestly about whether the focus should shift toward comfort, dignity, and support?"

Or simply: "Given the changes we are seeing, would a hospice evaluation be appropriate?"

If family members fear it means giving up: "We are not trying to give up. We are trying to understand whether comfort-focused support would help now."

ROB'S NOTE

I have walked into a lot of rooms after the word "hospice" was finally said out loud.

Almost always, the fear in the room is bigger than the truth. Families think they are signing a surrender. What they are actually doing is finally letting someone help.

I have watched pain come down. I have watched a daughter stop being a nurse and go back to being a daughter, holding her father's hand instead of his medication chart. I have watched families say the things they were afraid they would never get to say.

Hospice did not take the time away. It gave the time back — softer, closer, more honest. So if you are standing at that door, afraid to open it, hear me: choosing hospice is not choosing to stop loving someone. It is choosing to love them all the way to the end, on their terms, with their comfort at the center. That is not giving up. That is showing up.

CHAPTER 12 TAKEAWAY

Hospice is not the family choosing death. It is the family choosing support when the illness is already changing life. The next right question is: **would comfort-focused care help us care better now?**

Medicare Basics

Part 3 — Paying for Care. Care decisions often become money decisions. Families may know what help is needed but not who pays for it, and that confusion creates panic, arguments, and false expectations. These chapters are not meant to turn you into a Medicare expert, Medicaid planner, or elder law attorney. They are meant to help you ask better questions. Rules can change, and coverage depends on the person's situation, plan, medical need, eligibility, provider participation, and documentation. When in doubt, ask the plan, provider, social worker, or a qualified professional — and do not build a care plan on assumptions.

“An investment in knowledge pays the best interest.”

— ATTRIBUTED TO BENJAMIN FRANKLIN

THE PLAIN TRUTH

Medicare is health insurance. It is not long-term care insurance. That one sentence can save families a lot of confusion.

Medicare may help pay for hospital care, doctor visits, skilled nursing facility care for a limited time when requirements are met, some home health care, hospice, preventive care, outpatient services, and prescription drugs depending on coverage. But Medicare generally does not pay for long-term custodial care if that is the only care needed — help with daily living such as bathing, dressing, toileting, meals, and supervision when there is no skilled medical need. This is where families get surprised: “Mom has Medicare — why won't it pay for assisted living?” The hard truth is that Medicare was not designed to pay for every kind of aging-related care. The better question is not, “Does Medicare pay for care?” It is: **what kind of care is needed, and does Medicare cover that specific care in this situation?**

WHAT FAMILIES USUALLY THINK

Families often think Medicare pays for nursing homes, assisted living, someone to stay with Dad, long-term dementia care, or private duty care — or that if the doctor says it is needed, Medicare automatically pays. These misunderstandings cause real problems: a family delays planning believing Medicare will cover care that is actually private pay, a caregiver burns out waiting for help Medicare

does not provide, or a family is shocked when a rehab stay ends sooner than expected. Medicare is important, but it has limits, and families need those limits explained plainly.

The Main Parts of Medicare

PART	WHAT IT GENERALLY HELPS COVER
Part A	Hospital insurance — inpatient hospital care, skilled nursing facility care under certain conditions, hospice, and some home health.
Part B	Medical insurance — doctor visits, outpatient care, preventive services, durable medical equipment, and some home health.
Part C	Medicare Advantage — private plans through which you receive your Medicare benefits. You are still in Medicare, but the plan's networks, prior authorizations, and copays apply. Call the plan directly.
Part D	Prescription drug coverage — depends on the plan, formulary, pharmacy, and cost-sharing.

Original Medicare vs. Medicare Advantage

This distinction matters. Original Medicare includes Part A and Part B, often with a Part D drug plan and a Medigap supplement, and families deal with Medicare rules and participating providers. Medicare Advantage plans are offered by private companies approved by Medicare; they may include drug coverage and extra benefits but also networks, referrals, prior authorization, and plan-specific rules. If your parent has Medicare Advantage, always ask: Is this provider in network? Is prior authorization required? What is the copay? What is covered and not covered? What happens if the plan denies coverage, and what appeal rights exist? The Medicare card alone does not tell the whole story — look at the actual plan.

One more caution: leaving an Advantage plan to return to Original Medicare later may mean medical underwriting to buy a Medigap supplement — a person with health problems can be turned down or charged more. Ask before switching, not after.

What Medicare Usually Does Not Cover

THE NEED CAN BE REAL AND STILL NOT COVERED

Medicare generally does not cover long-term custodial care when that is the only care needed — ongoing help with bathing, dressing, toileting, meals, housekeeping, supervision, long-term dementia care, assisted living or memory care room and board, custodial nursing home care, or private duty care

for daily support. There may be other paths (Medicaid waivers, VA benefits, long-term care insurance, state resources), but do not assume Medicare will pay for daily care simply because the need is real.

Home Health, Skilled Nursing, and the Inpatient Trapdoor

Home health. Medicare may cover home health — skilled nursing, therapy, medical social services, and part-time or intermittent aide care in the plan of care — when requirements are met. But it is not private duty care or all-day supervision, and it requires a clinician's evaluation, a skilled need, homebound status, and documentation. Ask, "Can we schedule a provider evaluation to determine whether home health is appropriate?" — not "Can Medicare send someone to stay with Mom?"

Skilled nursing facility care. Medicare Part A may cover a limited stay if requirements are met: Part A days available, a qualifying inpatient hospital stay, a Medicare-certified facility, and a need for skilled care or therapy. The qualifying stay generally means at least three consecutive days as an admitted inpatient — observation days and the discharge day do not count — though many Medicare Advantage plans and some programs waive this. Ask early whether the hospital stay was inpatient or observation, how many covered days are available, what you could owe, what happens when coverage ends, and what appeal rights exist.

Inpatient vs. observation status. This is a major trapdoor: a person may spend the night in the hospital and still be considered outpatient under observation status, which can affect whether Medicare covers a later skilled nursing stay. Ask early and more than once: "**Is my parent admitted as an inpatient, or under observation status?**" Write down the answer and who gave it.

Hospice, Assisted Living, and Equipment

Hospice. When eligibility requirements are met, Medicare covers hospice, including team support, medications related to the hospice plan, equipment, supplies, and bereavement support. But room and board in a facility is generally not covered unless the team determines short-term inpatient care is needed for symptom management, and hospice is a benefit — not a full-time caregiver.

Assisted living and memory care. Medicare usually does not pay the monthly residential room and board, though it may still cover medical services there — doctor visits, covered therapy, home health if requirements are met, hospice if eligible, equipment, or medications. The residential cost is usually paid another way: private funds, long-term care insurance, VA benefits, a Medicaid waiver in some states, or family contribution.

Durable medical equipment. Medicare may cover items like a walker, wheelchair, hospital bed, oxygen, or bedside commode when medically necessary, prescribed, and supplied by a Medicare-approved supplier. Ask whether it is covered, who orders it, which supplier to use, whether prior authorization is needed, what you will owe, and who teaches you to use it.

WHAT TO DO THIS WEEK

1. **Identify the type of coverage** — Original Medicare or Medicare Advantage — and find the plan name and number.
2. **Name the exact care needed** — hospital, rehab, home health, hospice, equipment, or residential care.
3. **Ask whether the care is skilled or custodial** — this is the question that decides most coverage.
4. **Ask what Medicare may cover** for this specific need, what requirements apply, and what you may owe.
5. **Ask what Medicare will not cover** so you can plan those costs another way.
6. **Ask who can help** — case manager, social worker, SHIP counselor, or benefits counselor.

■ WORKSHEET — MEDICARE COVERAGE QUESTIONS

Use this to understand what Medicare may cover, what it probably will not, and what to ask before a care decision.

Person receiving care _____

Main caregiver / phone _____

Coverage type (Original Medicare / Advantage / unsure) _____

Plan name & phone _____

Part D drug plan / Medigap (if any) _____

Current care need (check all that apply)

- | | |
|---|---|
| ☐ Hospital | ☐ Doctor visit or medication |
| ☐ Rehab / skilled nursing facility | ☐ Assisted living or memory care |
| ☐ Home health | ☐ Private duty care |
| ☐ Hospice | ☐ Long-term nursing home care |
| ☐ Durable medical equipment | |

Skilled or custodial?

Is this care skilled, custodial, or both? Skilled ☐ Custodial ☐ Both ☐ Unsure ☐ — record who answered and when.

Coverage questions (record answers)

QUESTION	ANSWER	DATE
What part of Medicare applies?		
Is prior authorization required?		

Is the provider in network?		
What requirements must be met?		
What will Medicare / the plan pay?		
What will we owe (deductible/copay)?		
How long might coverage last?		
What could end coverage; can we appeal?		

Hospital status question (ask early, more than once)

Is my parent admitted as an inpatient, or under observation? Inpatient ☐ Observation / outpatient ☐ Unsure ☐ — record who answered and when.

Care Medicare usually does not pay for long-term (plan these separately)

- ☐ Assisted living or memory care room and board
- ☐ Long-term custodial nursing home care
- ☐ Long blocks of private duty care
- ☐ Daily supervision, meals, or housekeeping
- ☐ Companionship
- ☐ Ongoing bathing help as the only need

How might this be paid instead?

- ☐ Private pay
- ☐ Long-term care insurance
- ☐ Medicaid (if eligible)
- ☐ VA benefits (if eligible)
- ☐ Family contribution
- ☐ State/local program
- ☐ Unsure

■ QUESTIONS TO ASK WHENEVER SOMEONE SAYS "MEDICARE SHOULD COVER IT"

1. Which part of Medicare — Original or Advantage — and is the provider in network?
2. Is prior authorization required, and what requirements must be met?
3. Is there a skilled need, and is this short-term or long-term care?
4. What does Medicare pay, and what do we owe?
5. What happens if coverage ends, and can we appeal a denial or discharge?
6. Who can explain this in writing?

ROB'S NOTE

Medicare confusion can make families feel foolish. They are not foolish.

The system is complicated, and the words sound like they should mean more than they do. Skilled care. Custodial care. Observation. Inpatient. Benefit period. Prior authorization. Advantage plan. Supplement. It is a whole alphabet soup, and families are usually trying to understand it while someone they love is falling, declining, or being discharged.

So do not try to memorize everything. Ask better questions. Write down names and dates. Ask what is covered, what is not, and what happens next.

Medicare can be a powerful support, but it is not a blank check for aging. The clearer the family gets about that, the fewer surprises they will face.

CHAPTER 13 TAKEAWAY

Medicare is health insurance, not long-term care insurance. It may cover certain medical or skilled services when requirements are met, but it usually does not pay for long-term daily custodial care. The next right question is: **what exact care is needed, and does Medicare cover that specific care in this situation?**

Medicaid Basics

THE PLAIN TRUTH

Medicaid is different from Medicare. Medicare is health insurance. Medicaid is a needs-based program that may help pay for medical care and, for people who qualify, certain long-term services and supports.

For many families, Medicaid becomes important when an aging parent needs long-term care — nursing facility care, home and community-based services, or other state programs — and private money is running out. Medicaid can be a lifeline. It can also be confusing. Rules vary by state, and eligibility can depend on income, assets, medical and functional need, marital status, transfers of assets, care setting, and program type. So be careful: do not assume, do not guess, do not move money without advice, and do not wait until discharge day to ask questions. The better question is not, "Does Medicaid pay for care?" It is: **what Medicaid program might apply, what are the eligibility rules in this state, and what steps should we take before money runs out?**

WHAT FAMILIES USUALLY THINK

Families often think Medicaid and Medicare are the same, that Medicaid only pays for the poor, that it will take the house right away, that they must spend every dollar first, that they can just give money to the kids, or that they should wait until the money is gone. Most of these are too simple. Medicaid is not a single thing that works the same everywhere — it is a federal and state program, and state rules matter. One family may be dealing with nursing facility Medicaid, another with home and community-based services, another with a waiver or help paying Medicare premiums (ask for "Medicare Savings Programs" by name). Families need the right doorway.

Medicaid vs. Medicare

Keep this distinction clear. A person can have both (often called "dual eligible"), but having Medicare does not automatically mean Medicaid is available, and having Medicaid does not mean every setting or service is covered.

MEDICARE	MEDICAID
Health insurance	Needs-based program

Hospital, doctor, short-term SNF, home health, hospice	May help pay for long-term services and supports
Generally does not pay for long-term custodial care alone	May cover nursing facility or home/community services if you qualify
Same federal rules nationwide	Administered by states — rules vary by state

Medicaid and Long-Term Care

Medicaid is the nation's primary payer for long-term services and supports, delivered in institutions such as nursing facilities or in community-based settings such as the home, depending on the program and eligibility rules. It may help people who need ongoing assistance because of chronic illness, disability, frailty, or functional limits. The important point: Medicaid may help with long-term care for people who qualify, but qualifying is not automatic. The person usually must meet both financial and functional or medical requirements, and those requirements — along with the program names, waiting lists, and application process — differ by state. Do not rely on what worked for a neighbor in another state. Medicaid is not gossip-friendly. It is paperwork-friendly.

Nursing Facility, Home & Community, Assisted Living, and Memory Care

Nursing facility Medicaid may help pay for that level of care for people who meet state rules, but qualifying usually involves a detailed financial review and clinical eligibility. Not every facility accepts Medicaid, and not every bed is a Medicaid bed — ask early.

Home and community-based services (HCBS) let some people receive care at home or in the community instead of an institution, often through state waiver programs. They can be valuable but are not always immediate — waiting lists can exist, and they may not cover everything. Plan early.

Assisted living and memory care coverage varies widely. In some states a waiver may help pay for certain services but not room and board; in others these settings stay mostly private pay. Ask the community and the state Medicaid office before move-in — not after the money is gone — whether Medicaid helps, whether there are limited slots or a waiting list, and what happens if private funds run out.

Spend-Down, Transfers, and the Look-Back Period

DO NOT MOVE MONEY WITHOUT ADVICE

"Spend-down" generally means reducing countable income or assets to meet Medicaid rules — but it does not mean giving money away or making random purchases. For long-term care Medicaid, transfers and gifts may be reviewed during a look-back period — currently five years (60 months) in most states — and can create penalties or eligibility problems. Before spending down, transferring

assets, changing titles, or making gifts, ask a qualified elder law attorney, Medicaid planner, or the appropriate state resource. Good planning can protect care options. Bad guessing can create expensive delays.

The Spouse at Home and Estate Recovery

The spouse at home. When one spouse needs long-term care and the other stays home, special rules (often called spousal impoverishment protections) may let the spouse at home keep certain income or assets within program rules. Do not assume both spouses must become completely broke before Medicaid helps — and do not assume everything is protected. Ask the right professional what the spouse can keep, how the home counts, and what documentation is needed.

Estate recovery. State Medicaid programs are required to seek recovery from certain enrollees' estates. For people age 55 or older, states must do so for nursing facility services, home and community-based services, and related hospital and prescription drug services. Rules, exemptions (for a spouse, disabled child, caregiver child, and others), deferrals, and hardship waivers vary. Do not panic — and do not ignore it. Estate recovery is a planning issue. Ask someone to explain, in writing, whether it applies in your situation.

When to Ask for Help

Ask for help early if long-term care may be needed, private funds are running low, nursing facility or memory care is being discussed, a spouse remains at home, the person owns a home, assets were transferred or gifted, there is a trust, family members disagree about money, or you are close to discharge with no payment plan. Possible helpers include the state Medicaid office, the local Department of Social Services, a facility business office, a hospital or rehab social worker, the Area Agency on Aging, a SHIP counselor, an elder law attorney, a Medicaid planner, a care manager, or a veterans service officer if VA benefits may apply.

WHAT TO DO THIS WEEK

1. **Identify the state and program question** — nursing facility, HCBS, waiver, assisted living, memory care, or help with Medicare costs.
2. **Find out what level of care is needed** and whether it matches a Medicaid program.
3. **Gather financial documents** — bank and income statements, insurance cards, property info, power of attorney.
4. **Ask about timing** — application length, waiting lists, retroactive coverage, and what to pay while pending.
5. **Do not transfer assets without advice** — talk to a qualified professional first.
6. **Ask about estate recovery** in calm, plain language and get the answer in writing.

■ WORKSHEET — MEDICAID PLANNING

This does not replace legal, financial, or Medicaid advice. It helps the family gather information and ask better questions.

Person receiving care _____

Main caregiver / phone _____

State of residence / county _____

Current location (home, hospital, SNF, AL, memory care) _____

What are we trying to figure out? (check all that apply)

- ☐ Will Medicaid help pay for nursing facility care?
- ☐ Will Medicaid help pay for care at home?
- ☐ Is there a Medicaid waiver?
- ☐ Will Medicaid help in assisted living or memory care?
- ☐ What happens when private funds run out?
- ☐ How does the spouse at home stay protected?
- ☐ Does estate recovery apply?
- ☐ How do we apply?

Financial snapshot (for gathering information, not deciding alone)

INCOME / ASSET	SOURCE / INSTITUTION	APPROX. VALUE
Social Security / pension		
Checking / savings		
Investment / retirement accounts		
Life insurance		
Home / real estate		
Vehicle		
Burial / funeral policy		

Documents to gather (check when found)

- ☐ Social Security & Medicare cards
- ☐ Health insurance cards
- ☐ Birth & marriage certificates
- ☐ Social Security award letter
- ☐ Pension & bank statements
- ☐ Investment & retirement statements
- ☐ Life insurance policies
- ☐ Property deed & mortgage
- ☐ Vehicle title
- ☐ Funeral / burial policy

- ☐ Trust documents
- ☐ Level-of-care assessment
- ☐ Healthcare & Financial Power of Attorney
- ☐ Records of any gifts or transfers

Transfers & gifts (do not move money or property without advice)

Have any gifts or transfers been made recently? Yes ☐ No ☐ Unsure ☐ — if yes or unsure, list date, what was transferred, to whom, and value, and ask a professional whether it creates a penalty.

■ KEY QUESTIONS TO ASK

1. Which Medicaid program may apply, and what are the income and asset limits?
2. What medical or functional level of care is required, and is there a waiting list?
3. What documents are needed, how long does the application take, and can coverage be retroactive?
4. What does the person pay from monthly income, and what happens to the spouse at home?
5. How is the home treated, and do transfers or gifts create problems?
6. Does estate recovery apply, and what protections or hardship waivers exist?
7. Does the facility accept Medicaid, are there limited slots, and what happens if private funds run out?

ROB'S NOTE

Medicaid conversations make families nervous. People worry about the house, the bank account, the spouse at home, the look-back, the application, the fear that one wrong move will ruin everything.

That fear is understandable. But fear is not a plan.

The best thing a family can do is slow down, gather documents, ask state-specific questions, and get qualified help before making financial moves. Medicaid may become the bridge that keeps care going when private funds are no longer enough — but it is a bridge with rules. Do not cross it blindfolded.

Ask early. Write everything down. And before moving money, changing titles, or making gifts, get advice.

CHAPTER 14 TAKEAWAY

Medicaid may help pay for long-term care for people who meet financial and functional requirements, but the rules vary by state and program. The next right question is: **which Medicaid program might apply, what are the rules in this state, and who can help us apply correctly?**

Private Pay

THE PLAIN TRUTH

Private pay means the family or the person receiving care pays directly for care using personal funds. This may include savings, retirement income, pensions, Social Security, investments, home sale proceeds, or family contributions.

Private pay often becomes part of the plan when care is needed but Medicare does not cover it, Medicaid is not available yet, or long-term care insurance does not apply. It may pay for private duty care, assisted living, memory care, long-term care, transportation, home modifications, respite, and services Medicare does not cover. It can feel uncomfortable because care costs are high and the subject is emotional — families worry about running out of money, about inheritance, about fairness between siblings, about a spouse at home. But avoiding the money conversation does not protect the family. It weakens the care plan. The better question is not, "Can we afford care forever?" It is: **what care is needed now, what does it cost, how long can we sustain it, and when do we need another plan?**

WHAT FAMILIES USUALLY THINK

Families often think Medicare will cover more than it does, that they should not have to use savings, that Mom worked hard for that money, or that Dad wanted to leave something to the kids. These thoughts are common — and dangerous if they delay planning. Money is not the most important thing, but it affects options. A family may want twenty-four-hour care at home that is not sustainable. A parent may want assisted living whose rate rises with care needs. A family may protect an inheritance so tightly that the person who earned the money does not receive the care they need. Care decisions and money decisions are connected. Money should not make every decision — but it should be named clearly enough that the family can plan honestly.

What Private Pay Covers — and What It Doesn't Solve

Private pay can cover what insurance does not: private duty caregivers, assisted living and memory care fees, adult day care, respite, care managers, transportation, meal delivery, home safety modifications, and supplies. It is often the glue between formal services — home health for skilled needs plus private duty for meals and bathing, hospice plus private duty help at night. But money can buy help; it cannot solve every problem. Private pay does not guarantee a perfect caregiver, a safe home, family agreement, unlimited time, or no future decline. Money creates options, and it can disappear quickly without a plan.

The family needs to understand the monthly burn rate: how much is care costing each month, and how long can this plan last?

The Monthly Burn Rate

The monthly burn rate is the amount spent on care each month. Families need this as **a number, not a vague guess**. Include housing or facility base rate, care level fees, private duty care, medications, supplies, transportation, insurance premiums, food, home maintenance, utilities, copays, equipment, and care manager fees. Then compare it to monthly income and available savings. A family may discover the plan is sustainable, works only for a limited period, or needs adjustment — or that Medicaid planning, long-term care insurance, VA benefits, a home sale, or a less expensive setting should be explored. Numbers do not remove emotion. But they turn fog into a map.

Home, Assisted Living, and Memory Care Costs

At home, a few hours a week may be manageable, but daily help costs more, overnight costs more, and twenty-four-hour care can become extremely expensive. Ask how many hours are needed, the hourly rate, minimum shifts, night and weekend rates, and how long it can be sustained. The question is not whether home is emotionally preferred — it is whether home is safe, supported, and financially sustainable.

Assisted living bills may include base rent, care level fees, medication management, community or move-in fees, supply and transportation charges, and rate increases over time. Ask for a written cost breakdown — not just the brochure number — and ask what is included, what costs extra, how care levels are priced, how often rates rise, and what happens if funds run out.

Memory care is often more expensive because it provides more supervision and dementia-specific support. Ask the total monthly cost today, what could make it increase, what needs would require added private duty care or a move, and whether Medicaid options exist in that state or community. Memory care can bring safety and structure — and drain funds quickly if not planned carefully.

Family Contributions and the Spouse at Home

Family contributions should be handled carefully, not built on vague promises. Write down who is contributing, how much, how often, for how long, whether it is a gift or a loan, who pays the bill, who tracks payments, and the backup plan if someone stops. Money between family members can become emotional; a written plan protects the relationships and keeps the care plan from collapsing because everyone "thought someone else was handling it."

WATCH CLOSELY — THE SPOUSE AT HOME

The spouse at home needs extra care in private-pay planning. Do not spend down money without understanding how the spouse at home will live — what income they need, what bills, housing, and

insurance costs remain, what assets are available, and whether Medicaid planning is likely later. Care planning should not protect one person by making the other financially unsafe. Both lives matter.

When to Ask for Professional Guidance

Ask for guidance when care costs are high, funds may run out within one to two years, a spouse remains at home, a home may need to be sold, Medicaid may be needed later, long-term care insurance exists, family members disagree about money, someone wants to transfer assets, or there are trusts or complex accounts. Possible professionals: an elder law attorney, financial advisor, accountant, Medicaid planner, long-term care insurance representative, veterans benefits counselor, facility business office, or care manager. A good question asked early can prevent a very expensive mistake later.

WHAT TO DO THIS WEEK

1. **Write down the current care costs** — every recurring cost. Do not guess.
2. **Identify monthly income** — Social Security, pension, retirement distributions, and other income.
3. **Estimate available savings** — list accounts and approximate values. Gather information; do not make changes yet.
4. **Calculate the monthly gap** — care cost minus income equals what must come from savings or another source.
5. **Ask how long the current plan can last** — divide available funds by the monthly gap.
6. **Decide when another plan is needed** — review insurance and VA benefits, ask about Medicaid planning, or compare settings before the money is gone.

■ WORKSHEET — PRIVATE PAY PLANNING

For planning only. It does not replace financial, legal, tax, Medicaid, or benefits advice.

Person receiving care _____

Main caregiver / phone _____

Financial point person / phone _____

Financial Power of Attorney (if known) _____

Current care setting _____

Monthly income

SOURCE	MONTHLY AMOUNT	NOTES
Social Security		
Pension		

Retirement distribution		
Rental / investment income		
Other income		
Total monthly income		

Monthly care costs

COST	MONTHLY AMOUNT	NOTES
Private duty care		
Assisted living / memory care base rate		
Care level & medication fees		
Supplies & incontinence		
Transportation & adult day care		
Housing, utilities, maintenance		
Insurance premiums & copays		
Equipment & care manager		
Total monthly care costs		

Monthly gap & how long the plan lasts

Total monthly care costs _____

Total monthly income _____

Monthly gap (costs minus income) _____

Funds available for care _____

Estimated months this plan can last _____

Date to review the plan again _____

Available funds for care

ASSET / ACCOUNT	APPROX. VALUE	ACCESSIBLE?
Checking / savings		
Investment account		

Retirement account		
Home equity		
Life insurance cash value		

Other payment sources to explore (check all that may apply)

- | | |
|--|--|
| ☐ Long-term care insurance | ☐ Home sale |
| ☐ VA benefits | ☐ Reverse mortgage (with professional guidance) |
| ☐ Medicaid planning / waiver / HCBS | ☐ Life insurance options |
| ☐ Family contributions | ☐ State, local, or community support |

Family contribution plan (write it down clearly)

NAME	AMOUNT	FREQUENCY	GIFT OR LOAN?

Who pays the care bill? _____

Who tracks contributions? _____

What happens if someone cannot continue? _____

■ COST QUESTIONS TO ASK A FACILITY OR AGENCY

1. What is the base monthly cost, what is included, and what costs extra?
2. How are care levels priced, and how often can rates increase?
3. Are there move-in fees or deposits, and are supplies, medication, and transportation included?
4. What happens if care needs increase, or if private funds run out?
5. Do you accept long-term care insurance, or Medicaid / Medicaid waiver if applicable?
6. What is the move-out policy, and can we get all costs in writing?

ROB'S NOTE

Private pay conversations can feel cold because they put numbers next to love. But the numbers are not the enemy. The numbers are there so the family can protect the care plan.

I have seen families avoid the cost conversation until the crisis arrives. Then everyone is shocked, rushed, and angry — and that is when bad decisions happen.

Talking about money does not mean you care less. It means you are trying to make care last. It means you are trying to protect the parent, the caregiver, the spouse at home, and the family from surprise.

Do not let guilt write the budget. Do not let fear hide the numbers. Put the costs on paper. Then decide what the next right step should be.

CHAPTER 15 TAKEAWAY

Private pay means using personal or family funds to pay directly for care that insurance or public benefits may not cover. The next right question is: **what does the current care plan cost each month, and how long can we safely sustain it?**

Long-Term Care Insurance

THE PLAIN TRUTH

Long-term care insurance is simple at the starting line. You either have it or you do not.

If your parent has a policy, it may help pay for certain types of long-term care, depending on the terms.

If your parent does not have a policy, this chapter will be short. Move on.

Do not spend energy trying to create coverage in the middle of a crisis. Long-term care insurance usually has to be purchased before care is needed. Once someone is already declining, in assisted living, needing memory care, or requiring daily help, it is usually too late to buy a new policy that solves the current problem. The better question is not, "Should we get long-term care insurance now?"

It is: **does a policy already exist, and if it does, how do we activate it?**

WHAT FAMILIES USUALLY THINK

Families often think, "I'm sure Dad has something," or "Mom was organized, so there must be a policy," or "We'll just buy a policy now." Maybe. Maybe not. The first job is not to guess — it is to find the policy.

What to Look For — and Where

Search for a long-term care insurance policy, premium notices, annual statements, insurance company letters, bank drafts for premiums, employer or retirement benefit documents, life insurance policies with long-term care riders, hybrid life/long-term care policies, insurer emails, financial advisor records, or an agent's contact information. Check the important-documents folder, filing cabinet, safe, desk drawers, bank statements, email, financial advisor portal, tax files, insurance binder, mail pile, and power of attorney files.

If There Is No Policy

If there is no policy, do not get stuck here. Move to the next payment questions: Can care be paid privately? Are VA benefits possible? Could Medicaid become part of the plan? Is there a state program or waiver? Does the care setting offer financial options? No policy means no long-term care insurance benefit to activate. That is not a moral failure. It is just information.

If There Is a Policy

If a policy exists, call the insurance company. Ask what is covered and how to start a claim — and do not assume every setting or service qualifies. Ask specifically about home care, assisted living, memory care, adult day care, and nursing facility care; caregiver and licensed-agency requirements; the daily or monthly benefit amount; the elimination period; the maximum benefit period; inflation protection; required assessments and documentation; whether it is reimbursement or a cash benefit; and who can submit claims.

Key Terms Families Need to Know

Benefit amount — how much the policy may pay, listed as a daily amount, monthly amount, or pool of money.

Elimination period — the waiting period before benefits begin, during which the family may need to pay privately. Ask whether it is counted in calendar days, service days, or another rule.

Covered setting — not every policy covers every type of care; ask whether it covers the setting you are considering.

Covered provider — some policies require a licensed agency or approved facility, and privately hired caregivers may not qualify.

Claim documentation — the insurer may require care notes, invoices, assessments, or provider forms. Without the right paperwork, payment may be delayed.

WHAT TO DO THIS WEEK

1. **Find out whether a policy exists** — check documents, bank statements, financial records, and advisor files.
2. **If no policy exists, move on** to the next payment option. Do not linger.
3. **If a policy exists, call the company** and ask how to start a claim and what documents are needed.
4. **Get the benefit details in writing** — ask for a benefits summary.
5. **Match the policy to the care plan** — make sure the provider or setting meets the policy requirements before choosing care.
6. **Assign one person to manage the claim** — claims involve ongoing invoices, forms, and follow-up.

■ WORKSHEET — LONG-TERM CARE INSURANCE POLICY FINDER

Use this to find out whether a policy exists and what to do next.

Person receiving care _____

Main caregiver / phone _____

Person checking for policy / phone _____

Step 1: Is there a policy?

Policy found? Yes ☐ No ☐ Unsure ☐ — if no, stop here and move to the next payment option. If yes or unsure, continue.

Step 2: Where did we look? (check all that apply)

- | | |
|---|---|
| ☐ Important documents folder | ☐ Financial advisor files |
| ☐ Filing cabinet / safe / desk drawers | ☐ Tax files |
| ☐ Bank statements | ☐ Mail pile |
| ☐ Email | ☐ Employer or retirement benefit documents |
| ☐ Insurance folder / binder | ☐ Life insurance documents |

Step 3: Policy information

Insurance company _____

Policy number _____

Company phone _____

Agent / advisor / phone _____

Policy type: Traditional long-term care ☐ Life insurance with LTC rider ☐ Hybrid ☐ Unsure ☐

Step 4: What does it cover?

QUESTION	YES / NO / UNSURE	NOTES
Home care covered?		
Assisted living covered?		
Memory care covered?		
Nursing facility covered?		
Adult day care covered?		
Family caregiver covered?		
Licensed agency required?		

Step 5: Benefit details

Daily / monthly benefit amount _____

Total benefit pool _____

Elimination period & how it is counted _____

Inflation protection? (yes / no / unsure) _____

Reimbursement or cash benefit? _____

■ STEP 6: CLAIM REQUIREMENTS TO ASK THE INSURER

1. How do we start a claim, and what forms are needed?
2. Is a provider assessment or doctor statement required?
3. Are invoices or care notes required, and must the provider be licensed?
4. How long does approval usually take?
5. Who can speak on behalf of the policyholder, and what power of attorney paperwork is needed?

Final decision

Where this lands: No policy — move to other payment options ☐ Policy exists, start a claim ☐
Policy exists but current care may not qualify ☐ More information needed ☐

Next step _____

Person responsible _____

Due date _____

ROB'S NOTE

Long-term care insurance is not something families should spend three weeks debating during a crisis.

Find the policy. If it exists, call and activate the process. If it does not exist, move on.

That may sound blunt, but it is kind to the family's energy. When care is needed now, the family does not need imaginary coverage. They need real options.

Paper first. Facts second. Decision third.

CHAPTER 16 TAKEAWAY

Long-term care insurance starts with one question: does a policy already exist? If yes, find out what it covers and how to start a claim. If no, move to the next payment option.

Prescription Assistance

THE PLAIN TRUTH

Medication costs can quietly drain a family's care budget. Sometimes the biggest financial problem is not the rent, the caregiver, or the facility. It is the monthly pile of prescriptions.

Families may assume the price at the pharmacy is the final answer. It may not be. There may be ways to lower prescription costs depending on the medication, insurance plan, income, pharmacy, diagnosis, manufacturer program, state program, or available alternatives. Not every person qualifies, not every medication has assistance, and some programs change or close — but families should still ask. The better question is not, "Why is this so expensive?" It is: **has anyone reviewed whether there is a safer, covered, or lower-cost way to get this medication?**

The Million-Dollar House and the Free Medication

Years ago, while working for a large pharmaceutical company, I was helping a physician's office find a way for a patient to afford a medication that cost more than \$850 per month.

The physician's office told me she would never qualify for assistance. "She lives in a multi-million-dollar home," they said.

I asked a different question: "What is her income?"

After reviewing the details, I explained that many pharmaceutical assistance programs focus primarily on income, not the value of a person's home. The office was skeptical. The patient applied anyway. She was approved. Her medication was provided at no cost.

That experience taught me a lesson I have seen repeated throughout healthcare: many families assume they do not qualify for help before they ever ask. **Never disqualify yourself. Ask the question.**

Complete the application. Let the program tell you no. One application can sometimes save more money than months of worrying.

WHAT FAMILIES USUALLY THINK

Families often think the pharmacy price is the only price, that Medicare covers all prescriptions, that they make too much to qualify for help, or that coupons always (or never) work. Some of these may be true in a specific case, but families should not guess. Prescription assistance is a question tree: you try one door; if it closes, you try the next.

Medicare Part D and Extra Help

Part D and drug coverage. Medicare drug coverage depends on the specific Part D or Medicare Advantage plan — formularies, tiers, preferred pharmacies, deductibles, copays, prior authorization, and step therapy. The same medication may cost different amounts under different plans. Ask whether it is covered, what tier it is on, whether prior authorization or step therapy applies, whether a preferred pharmacy, mail order, or 90-day supply costs less, and whether there is a covered alternative. Review the medication list at least once a year, especially during open enrollment.

The yearly cap on drug costs. As of this writing, Medicare drug coverage caps what a person pays out of pocket for covered Part D drugs each year — \$2,100 in 2026, with the amount adjusted annually. Once the cap is reached, covered drugs cost nothing for the rest of the year. Families can also ask about the **Medicare Prescription Payment Plan**, which spreads those out-of-pocket costs across the year in predictable monthly bills instead of large amounts at the pharmacy counter. Ask the plan or a SHIP counselor for the current cap amount and how the payment plan works — for a family staring at a large pharmacy bill, these two protections can change the whole picture.

Extra Help. Extra Help is a Medicare program that helps people with limited income and resources pay Part D costs — premiums, deductibles, and copays. Some people qualify automatically; others apply — before or after enrolling in Part D. Do not assume income is too high. Check whether your parent qualifies, and ask who can help with the application.

Manufacturer, State, and Foundation Programs

Manufacturer patient assistance programs (PAPs). Some drug companies offer programs that may provide financial assistance or free medication, sometimes for people with Part D and sometimes only for those without insurance. They vary and may require proof of income, prescription information, doctor forms, denial letters, or out-of-pocket records. Medicare has a tool to search assistance programs by drug. Ask the manufacturer or program directly.

State Pharmaceutical Assistance Programs (SPAPs). Some states offer programs to help with drug costs; availability varies. Ask whether this state has one, who qualifies, whether it works with Part D, and whether it helps with premiums, copays, or specific medications.

Disease-specific foundations. Some nonprofits help with medication costs for specific diagnoses — certain cancers, rare diseases, heart, lung, or kidney conditions. Funding opens and closes, so check current availability, covered medications, income limits, and whether the program works with Medicare.

Ask About Alternatives, Coupons, and a Medication Review

Alternatives. Sometimes the easiest savings come from one question: "Is there a lower-cost alternative that is safe and appropriate?" Ask about a generic, a therapeutic alternative, a different dose or formulation, a covered formulary option, a 90-day supply, or whether the medication is still needed.

Doctors do not always know the price at the counter; pharmacists often do. Do not stop or change medications without clinical guidance — but do ask.

Coupons and discount cards. Some discount programs may lower costs, but they may not apply with Medicare or Medicaid, and using one can mean the cost does not count toward the plan's deductible or out-of-pocket tracking. Sometimes the cash price is lower than the insurance price; sometimes it is not. Ask the pharmacist the insurance price, the cash price, whether a discount affects insurance tracking, and whether another pharmacy costs less.

Medication review. Prescription assistance is not only about finding money — it is also about reducing burden. Ask for a medication review if there are many medications, duplicates, multiple prescribers, or if the person is falling, confused, dizzy, sleepy, or nearing end of life. A medication that made sense five years ago may not make sense now. Do not stop medications on your own, but do ask whether the list still matches the person's goals, condition, and safety.

WHAT TO DO THIS WEEK

1. **Make a complete medication list** — name, dose, frequency, prescriber, pharmacy, and monthly cost.
2. **Ask the pharmacist for a cost review** of the most expensive medications and lower-cost options.
3. **Ask the prescriber about alternatives** — a generic, formulary option, or safe substitute.
4. **Check Medicare Extra Help eligibility.**
5. **Search manufacturer and state assistance** using Medicare's tool and the drug company.
6. **Assign one person to track medication costs** — savings require forms, calls, and follow-up.

■ WORKSHEET — PRESCRIPTION ASSISTANCE

Use this when medication costs are high. It does not replace medical advice — do not stop, skip, or change medications without guidance from the prescriber or pharmacist.

Person receiving care _____

Medication point person / phone _____

Primary pharmacy / phone _____

Primary doctor / prescriber / phone _____

Current drug coverage

Coverage type: Medicare Part D ☐ Medicare Advantage w/ drug coverage ☐ Employer/retiree
☐ Medicaid ☐ VA ☐ None ☐ Unsure ☐

Plan name & phone _____

Preferred pharmacy (if known) _____

Mail-order option? (yes / no / unsure) _____

Medication cost list

MEDICATION	DOSE	PRESCRIBER	PHARMACY \$	INSURANCE \$

Three most expensive medications _____

■ QUESTIONS FOR THE PHARMACIST

1. Which medication is costing the most, and is it running through the correct insurance?
2. Is this a preferred pharmacy — and would another pharmacy, mail order, or a 90-day supply be cheaper?
3. Is there a generic version or a discount option?
4. If we use a discount card, does it affect insurance tracking?
5. Should we ask the doctor about an alternative?

■ QUESTIONS FOR THE PRESCRIBER

1. Is this medication still needed, and is there a generic or lower-cost/formulary alternative?
2. Can the dose or formulation be changed safely to lower cost, and are any medications duplicated?
3. Could any medications be increasing falls, confusion, dizziness, or sleepiness?
4. Would a medication review be appropriate?
5. Is there a manufacturer assistance program, and can your office help complete the forms?

Assistance checks

Medicare Extra Help — could it apply? Yes ☐ No ☐ Unsure ☐ — who will check/apply?

Also check: Manufacturer patient assistance ☐ State Pharmaceutical Assistance Program ☐
Disease-specific foundation ☐

Documents often needed for manufacturer programs: prescription information, proof of income, insurance/Part D card, a doctor form, patient signature, and denial or prior-authorization information.

Medication safety review (check any concern)

- ☐ Too many or duplicate medications
- ☐ Missed or double doses
- ☐ Dizziness, falls, or sleepiness
- ☐ New confusion or poor appetite
- ☐ Multiple prescribers
- ☐ Old medications still in the home
- ☐ Hospice or palliative care involved

HOPE'S NOTE — FROM THE PHARMACIST

Two things I wish every family knew. First, your pharmacist can see what your doctor often cannot: every prescription, from every prescriber, on one screen — and we are the easiest person in healthcare to reach. No appointment needed; just ask for a minute at the counter.

Second, never let the price at the register be the end of the conversation. Ask us to check the cash price, a different pharmacy, a 90-day fill, and whether the prescriber should hear about an alternative. The counter price is a starting point, not a verdict. And ask us to run a discount program — GoodRx is the one most people know. Patients are often unsure about these, but the discount price is sometimes lower than your insurance copay. The trade-off: a discount purchase usually does not count toward your deductible or out-of-pocket cap, so ask us which way wins for each drug.

ROB'S NOTE

Prescription costs can embarrass families. People do not always want to say, "We cannot afford this medication."

Say it anyway. Say it to the pharmacist. Say it to the doctor. Say it to the social worker. Say it before pills are skipped, doses are stretched, or refills are delayed.

A medication cannot help if it is sitting behind a price the family cannot reach. There may not always be a solution, but there is almost always a better question. Ask what is covered. Ask what is cheaper. Ask what is still needed. Ask what help exists.

A quiet medication bill can become a loud crisis if nobody looks at it.

CHAPTER 17 TAKEAWAY

Prescription assistance starts with one practical step: make the medication list, find the expensive drugs, and ask the pharmacist and prescriber what can be done safely.

VA Benefits

THE PLAIN TRUTH

If the person receiving care is a Veteran, surviving spouse of a Veteran, or dependent of a Veteran, VA benefits may matter. **Do not assume. Check.**

Some families miss VA benefits because no one asks the simple question: did they serve? Other families assume VA will pay for everything, which is not safe either. VA benefits can be helpful, but eligibility depends on service history, discharge status, income, assets, disability status, health care enrollment, care needs, and the specific program. Benefits may include VA health care, VA pension, Aid and Attendance, Housebound benefits, survivor benefits, long-term services and supports, caregiver support, and burial benefits. The better question is not, "Does VA pay for care?" It is: **is this person connected to VA, and which VA benefit or program might apply?**

WHAT FAMILIES USUALLY THINK

Families often think, "Dad was in the military, so VA will pay," or "We make too much," or "He never used VA before, so it is too late," or "Aid and Attendance pays for assisted living automatically." Some of these may be partly true in certain situations, but many are too simple. VA benefits are not one bucket — they are several different doors. The family's first job is to find the right door.

First Question: Did They Serve?

Start here: did the person receiving care serve in the U.S. military? If yes or unsure, gather what you can — branch and dates of service, discharge status, DD214 or separation papers, any VA claim number, VA health care enrollment status, any service-connected disability rating, prior VA benefits, and surviving spouse status if applicable. Do not stop if you cannot find everything immediately. A Veterans Service Officer or VA contact may help identify what is needed.

Aid and Attendance, Housebound, and Pension

Aid and Attendance / Housebound. These are monthly payments added to a VA pension for qualified Veterans and survivors who need help with daily activities or are housebound. This is often the benefit families hear about when someone needs care. But it is not automatic — the person generally must first qualify for VA Pension or Survivors Pension, then meet additional requirements. The doctor completes VA Form 21-2680 to document the need, and it accompanies the pension application (VA Form 21P-

527EZ, or 21P-534EZ for survivors). Ask whether your parent qualifies for pension, whether Aid and Attendance or Housebound could be added, what income and asset rules apply, what medical forms are needed, and who can help apply.

VA Pension and Survivors Pension. VA Pension may provide monthly payments to eligible wartime Veterans who meet age or disability requirements and have limited income and net worth; Survivors Pension may help eligible surviving spouses or children. Do not guess eligibility — ask a VA-accredited representative or Veterans Service Officer. Key questions: Was service during a qualifying wartime period? Is the discharge status qualifying? Do income, net worth, and unreimbursed medical expenses fit the rules? Does assisted living or home care cost affect the calculation? Good paperwork matters here.

DO NOT MOVE MONEY HERE EITHER

VA pension has its own net worth limit and its own look-back period — currently 36 months — on asset transfers, with penalty periods for gifts made to qualify. Do not move money to qualify for Aid and Attendance without advice from a VA-accredited representative, and be wary of anyone selling financial products to "help you qualify."

VA Health Care, Long-Term Services, and Caregiver Support

Health care and long-term services. VA health care may provide access to geriatric and extended care programs — home-based primary care, homemaker and home health aide services, adult day health care, respite, skilled home health, community living centers, contract nursing home care, state Veterans homes, geriatric evaluation, palliative care, and hospice-related support. Availability and eligibility vary by location, clinical need, and program. Ask the local VA medical center, VA social worker, or Veterans Service Officer what applies.

Caregiver support. VA has caregiver support programs for caregivers of eligible Veterans — education, training, resources, respite connection, and support groups, plus the Program of Comprehensive Assistance for Family Caregivers for those who qualify. Ask whether the Veteran is enrolled in VA health care, whether there is a caregiver support coordinator, and what programs are available. Do not assume caregiver support only means money — sometimes the value is guidance, training, respite, or knowing whom to call. You can contact the VA Caregiver Support Line or a local team.

VA Benefits and Care Settings

VA does not simply pay for assisted living or memory care in every situation, but certain benefits may help — Aid and Attendance can add monthly pension support that the family can put toward care, and some state Veterans homes or community programs may be relevant. Even a monthly benefit that does not cover everything may help the care plan last longer. For hospice or palliative care, if the person is connected to VA, ask whether VA palliative or hospice support is available, whether it can be coordinated with Medicare hospice, and who coordinates care. Many Veterans also have Medicare,

Medicaid, or private insurance — the family needs to understand how benefits work together. Do not rely on a sales brochure or hallway rumor. Verify.

Documents and Who Can Help

Start gathering: DD214 or separation papers, Social Security number, any VA claim number, VA health care enrollment info, discharge status, marriage and death certificates if a survivor benefit is involved, medical records, a doctor statement, VA Form 21-2680 if applicable, income and asset information, care expense records, and power of attorney documents. For help, consider a local Veterans Service Officer, county Veterans Services office, VA medical center social worker, VA caregiver support coordinator, an accredited VA claims agent or attorney, or the state Veterans affairs office. A Veterans Service Officer can often identify which benefits apply and help complete forms — and for claims, ask whether the helper is VA-accredited.

WHAT TO DO THIS WEEK

1. **Ask the service question** — did the person serve, in what branch, and roughly when?
2. **Look for discharge papers** — DD214 or separation documents.
3. **Identify the current VA connection** — health care enrollment, disability rating, or benefits already received.
4. **Call a Veterans Service Officer or VA contact** to review eligibility.
5. **Gather care cost records** — home care, assisted living, memory care, medications, and other unreimbursed medical expenses.
6. **Assign one person to manage VA follow-up** — paperwork takes time and needs one owner.

■ WORKSHEET — VA BENEFITS SCREENING

Use this when the person may be a Veteran, surviving spouse, or dependent. It does not determine eligibility — it helps the family gather information and ask the right questions.

Person receiving care _____

Main caregiver / phone _____

VA benefits point person / phone _____

Service information

Did the person serve in the U.S. military? Yes ☐ No ☐ Unsure ☐

Branch & dates of service _____

Wartime service? (yes / no / unsure) _____

Discharge status _____

DD214 found? (yes / searching) — location _____

Current VA connection (check all that apply)

- | | |
|---|--|
| ☐ Enrolled in VA health care | ☐ Receives Aid and Attendance or Housebound |
| ☐ Has VA primary care / uses VA pharmacy | ☐ Surviving spouse receives a VA benefit |
| ☐ Has a VA disability rating | ☐ Has a VA social worker |
| ☐ Receives VA compensation or pension | ☐ Unsure |

Surviving spouse (complete if applicable)

Veteran name & date of death _____

Marriage certificate found? (yes / searching) _____

Death certificate found? (yes / searching) _____

Survivor benefits already received? (yes / no / unsure) _____

Possible VA benefits to explore (check what to explore)

- | | |
|--|--|
| ☐ VA health care enrollment | ☐ Long-term / home & community-based services |
| ☐ VA Pension / Survivors Pension | ☐ State Veterans home |
| ☐ Aid and Attendance | ☐ Caregiver support program |
| ☐ Housebound benefits | ☐ Respite |
| ☐ Service-connected disability compensation | ☐ Burial benefits |

Documents to gather (check when found)

- | | |
|---|---|
| ☐ DD214 / separation papers | ☐ VA Form 21-2680 (if A&A/Housebound) |
| ☐ Social Security number & VA claim number | ☐ Income, asset & bank statements |
| ☐ VA health care & Medicare cards | ☐ Care invoices & medical expense receipts |
| ☐ Marriage / death certificate (if survivor) | ☐ Power of attorney documents |
| ☐ Medical records & doctor statement | |

■ QUESTIONS FOR THE VETERANS SERVICE OFFICER OR VA CONTACT

1. Which VA benefits might apply, and is the Veteran enrolled in VA health care?
2. Is VA Pension, Survivors Pension, Aid and Attendance, or Housebound possible?
3. What income, asset, service-record, and medical-form requirements apply — and is VA Form 21-2680 needed?
4. Are unreimbursed medical expenses important to the calculation?
5. Could caregiver support, home-based services, respite, or a state Veterans home apply?
6. Could VA help with palliative care or hospice, and how does it work with Medicare?
7. What forms should be filed first, who can help complete them, and how long may it take?

Final decision

Where this lands: VA benefits do not appear relevant ☐ May apply — explore ☐ Review Aid and Attendance / Housebound ☐ Review VA health care enrollment ☐ Review caregiver support ☐ More information needed ☐

Next step _____

Person responsible _____

Due date _____

ROB'S NOTE

VA benefits are easy to miss because families are usually focused on the crisis in front of them — the fall, the hospital stay, the assisted living bill, the exhausted spouse, the memory care tour.

*But one question can open another door: **did they serve?***

If the answer is yes, do not assume and do not dismiss. Check. The benefit may not apply. The process may take time. It may not cover everything.

But if a Veteran or surviving spouse earned access to support, the family should at least know what is possible. Find the papers. Call the right person. Ask the question.

CHAPTER 18 TAKEAWAY

VA benefits may help some Veterans, surviving spouses, or caregivers, but eligibility depends on the person, the program, and the situation. The next right question is: **did they serve, and who can help us check what VA benefits may apply?**

The Important Documents Folder

Part 4 — The Documents That Matter. When a crisis hits, the family that can find the right paper moves; the family that cannot gets stuck. These chapters help you gather the medical, legal, and financial documents that decisions actually depend on — one trusted folder, the words on the key legal documents, and the information a family needs the moment someone can no longer speak for themselves. You do not need to become a lawyer. You need to be able to find the right document quickly.

“The palest ink is better than the best memory.”

— CHINESE PROVERB

THE PLAIN TRUTH

Sooner or later, someone will ask the family for a document. The hospital will want the medication list. The facility will need to know who has authority. The doctor will ask about advance directives.

These moments almost always come at the worst possible time — during a fall, a hospital stay, a sudden decline — when the family is tired, scared, and least able to go hunting through drawers. The important documents folder exists so that the right person can put a hand on the right paper in minutes, not hours. It is not about being organized for its own sake. It is about protecting the person's care and the family's calm. The better question is not, "Do we have all the paperwork?" It is: **can the right person find the right document quickly?**

WHAT FAMILIES USUALLY THINK

Families often think the paperwork can wait until things settle down, that one person "has it handled," or that everything is somewhere in the house if they ever need it. But things rarely settle down, the one person who has it handled is not always reachable, and "somewhere in the house" is not a plan at 2 a.m. in an emergency room. Building the folder is not a sign that something bad is coming. It is a gift to the family for the day something does.

What Belongs in the Folder

One folder — paper, digital, or both — should hold the essentials in a few clear categories: emergency and medical contacts; the medication list and doctor list; insurance and benefit cards; healthcare decision documents (power of attorney, advance directive, DNR or POLST if applicable); financial and legal documents (financial power of attorney, will, trust, deeds); care and facility paperwork; benefits and military or VA documents; and end-of-life and funeral information. You do not need every item on day one. You need a folder that exists and grows.

One small document worth adding while you are at it: a **HIPAA release**, which lets doctors and hospitals share medical information with the family members named on it. Without one, even a devoted daughter can be told "we can't discuss that."

Start With the First Five

Do not wait for the whole folder to be perfect. **A useful folder beats a perfect folder that never gets built.** Start with the five documents a family reaches for most: the medication list, the doctor list, the insurance cards, the healthcare decision documents, and the emergency contacts. Once those five are in one place, the folder is already doing its job — and you can keep adding.

Keep It Alive, and Tell People Where It Is

A folder only helps if the right people know where it lives. Tell the main caregiver, the backup caregiver, and the healthcare decision maker, and store it somewhere secure. Make copies of important cards and lists, and ask whether copies of legal documents are acceptable or whether originals are needed. Then review it every three months, and after any hospitalization, medication change, doctor change, insurance change, or move. Documents change; a folder that never gets updated becomes a museum. This folder should be alive.

WHAT TO DO THIS WEEK

1. **Choose one folder location** — paper, digital, or both — and one document point person.
2. **Gather the first five documents** — medication list, doctor list, insurance cards, healthcare decision documents, and emergency contacts.
3. **Make copies of important cards and lists**, and confirm whether legal documents need originals.
4. **Tell the right people where the folder is** — main caregiver, backup caregiver, and healthcare decision maker.
5. **List what is missing** and assign who will find each item and where to look.
6. **Set a review date** — every three months and after any major change.

■ WORKSHEET — IMPORTANT DOCUMENTS FOLDER CHECKLIST

The goal is simple: the right person can find the right document quickly.

Person receiving care _____

Date started / last updated _____

Document point person / phone _____

Backup document person / phone _____

Folder location & who has access

Paper folder location _____

Digital folder location _____

Password manager / access instructions _____

Who knows where the folder is _____

Emergency & medical contacts

ROLE	NAME	PHONE
Emergency contact		
Primary doctor		
Specialist		
Pharmacy		
Home health / hospice		
Preferred hospital		

Insurance & benefit cards (check when copied into the folder)

- | | |
|--|---|
| ☐ Medicare / Advantage / supplement | ☐ VA card |
| ☐ Part D prescription plan | ☐ Private / dental / vision insurance |
| ☐ Medicaid | ☐ Long-term care insurance information |

Healthcare decision documents

DOCUMENT	EXISTS?	COPY IN FOLDER?
Healthcare Power of Attorney		
Living Will / Advance Directive		
DNR / MOST / POLST		
HIPAA release		

Healthcare decision maker / phone _____

Alternate decision maker / phone _____

Financial & legal documents

DOCUMENT	EXISTS?	COPY IN FOLDER?
Financial Power of Attorney		
Will		
Trust		
Property deed / vehicle title		
Bank contact list / safe deposit info		

Attorney / phone _____

Financial advisor / phone _____

Accountant / phone _____

Care, benefits & military documents (check what applies)

- | | |
|---|--|
| ☐ Hospital / rehab discharge papers | ☐ Long-term care & life insurance policies |
| ☐ Home health or hospice paperwork | ☐ Pension / Social Security / Medicaid info |
| ☐ Facility or private duty agreement | ☐ DD214 / VA claim number / VA paperwork |
| ☐ Care plan / fall log | |

End-of-life & funeral information (a kindness to the family)

Funeral home preference / prepaid plan _____

Burial or cremation wishes _____

Cemetery information _____

Clergy / church contact _____

Important people to notify _____

Passwords & accounts

Do not write passwords in an unsafe place — use a secure password manager or trusted access plan.

Password manager used? (yes / no / unsure) _____

Who has access instructions? _____

Missing documents

MISSING DOCUMENT	WHO WILL FIND IT?	DUE DATE

--	--	--

Review schedule

Review this folder: Every 3 months ☐ After a hospitalization ☐ After medication or doctor changes ☐ After insurance changes ☐ After a move ☐ After legal documents are updated ☐

Next review date _____

ROB'S NOTE

The documents folder is not glamorous. No one brags about a binder.

But when the hospital asks for the medication list, when the facility asks who has authority, when the doctor asks about advance directives, when the family is tired and scared — that binder becomes mercy with tabs.

You are not building paperwork. You are building calm. A family with the right documents can move. A family without them gets stuck.

Start with the first five. Then keep going.

CHAPTER 19 TAKEAWAY

The important documents folder gives the family one trusted place to find medical, legal, financial, and emergency information. The next right question is: **can the right person find the right document in less than ten minutes?**

Healthcare Power of Attorney

THE PLAIN TRUTH

A Healthcare Power of Attorney names the person who can make medical decisions if your parent cannot make or communicate those decisions. **This document matters. A lot.**

When someone is alert and able to make decisions, they speak for themselves. But if they become confused, unconscious, seriously ill, or unable to communicate, the medical team needs to know who has authority to speak. That person may be called a healthcare agent, proxy, or surrogate — the term varies by state, but the purpose is the same. The better question is not, "Who in the family cares the most?" It is: **who is legally named to make healthcare decisions if this person cannot?**

WHAT FAMILIES USUALLY THINK

Families often think, "I'm the oldest, so it's me," or "I'm the spouse, so I decide everything," or "Everyone agrees, so we don't need paperwork," or "The hospital will know what to do." Sometimes the law provides default rules, and sometimes families agree. But crisis has a way of turning soft assumptions into hard arguments. The Healthcare Power of Attorney tells the medical team who should speak when the patient cannot. It gives the family a clear lane.

What It Does — and Does Not Do

Depending on the document and state law, a Healthcare Power of Attorney may cover decisions about doctors, hospitals, surgery, medications, tests, treatments, rehab, care-related placement, palliative care, hospice, artificial nutrition or hydration, life-sustaining treatment, comfort-focused care, and access to medical records. The basic goal is simple: the right person can speak with the medical team when the patient cannot.

It does not usually give someone control over money — that is a Financial Power of Attorney. And it does not mean the named person can ignore the patient's wishes. The agent's job is not personal preference; it is to honor the person's known wishes, values, faith, goals, and best interests. A Healthcare Power of Attorney is not a crown. It is a responsibility.

Capacity Matters — Do Not Wait

The best time to complete or update this document is while the person still has decision-making capacity — the ability to understand the decision, understand the consequences, and communicate a choice. If

memory is changing, do not wait. If dementia has been diagnosed, do not assume it is automatically too late, but do not delay either — ask the doctor and an attorney what is appropriate. Once a person loses capacity, signing new legal documents may no longer be possible, and families may face harder options — such as asking a court to appoint a guardian — a slower, more expensive, and more public process. The door is easiest to open before it locks.

Choosing the Agent — and a Backup

The best healthcare agent is not always the loudest, closest, or oldest. It is someone who can stay calm under pressure, listen to doctors, ask good questions, honor the person's wishes, communicate with family, be reachable, make hard decisions without making themselves the center, and put the patient's values ahead of personal guilt. This is not a popularity contest. It is a trust decision.

Every Healthcare Power of Attorney should name a backup agent if possible. The first person may be sick, traveling, overwhelmed, or unreachable. Ask who is the first choice, who is the backup, whether each person knows they are named, whether they understand the person's wishes, and whether they have a copy. A backup is not an insult. It is a safety net.

The Conversation Matters More Than the Form

The document matters, but so does the conversation. A healthcare agent should know what matters most to the person, what quality of life means to them, what treatments they would and would not want, where they would prefer to receive care, who they want involved, what faith or spiritual beliefs matter, whether they would want hospice or comfort-focused care, what they fear most, and what they hope for. Do not make the agent guess in the hallway outside an ICU room. Talk while there is time.

When Family Disagrees

A Healthcare Power of Attorney can reduce conflict, but it may not erase emotion. Family members may still disagree, push for more or less care, or accuse the agent of giving up. The agent should stay grounded in the person's wishes. Helpful language: "I hear that this is hard. My job is to follow what Mom wanted, not what is easiest for any of us," or "We are not making this decision from fear — we are trying to honor what Dad told us mattered most," or "Let's ask the medical team to explain the options clearly, then come back to what Mom would choose if she could speak." The agent does not need to win every argument. The agent needs to stay faithful to the patient.

Give Copies to the Right People, and Review It

Once complete, share copies with the healthcare agent, backup agent, primary doctor, hospital system, any facility, hospice or palliative team, main caregiver, and attorney, and put one in the important documents folder. Keep the original in a safe but accessible place — a document no one can find is a paper ghost. Review it if the person moved to another state, the named agent died or is no longer

appropriate, relationships changed (marriage, divorce, widowhood), wishes changed, a serious diagnosis occurred, dementia is progressing, or the document is old or missing. Do not assume an old form still does the job — ask an attorney whether it is valid and current.

WHAT TO DO THIS WEEK

- Find out whether one exists** — check the documents folder, safe, attorney files, and hospital records.
- Identify who is named** — the healthcare agent and the backup agent.
- Make sure the agent knows** they are named and understands the responsibility.
- Share copies** with the agent, doctor, facility, and care team.
- Have the wishes conversation** — what care would be wanted, and what would not.
- Update if needed** — if the document is missing, outdated, or no longer reflects the person's wishes, ask an attorney.

WORKSHEET — HEALTHCARE POWER OF ATTORNEY

This does not replace legal advice. State rules vary — ask an attorney if the document is missing, outdated, unclear, or if capacity is a concern.

Person receiving care _____

Main caregiver / phone _____

Document point person / phone _____

Document status

Does a Healthcare Power of Attorney exist? Yes ☐ No ☐ Unsure ☐

Date signed & state _____

Attorney/office that prepared it / phone _____

Original document location _____

Copy in documents folder? (yes / needs to be added) _____

Named agents

ROLE	NAME	RELATIONSHIP & PHONE	HAS COPY?
Primary agent			
Backup agent			
Second backup (if any)			

Do the named people know they are named? Yes ☐ No ☐ Unsure ☐

Copies shared with (check all that apply)

- | | |
|--|---|
| ☐ Healthcare agent & backup | ☐ Home health / hospice |
| ☐ Primary doctor / specialist | ☐ Palliative care team |
| ☐ Hospital system | ☐ Attorney |
| ☐ Assisted living / memory care | ☐ Important documents folder |
| ☐ Skilled nursing facility | |

Capacity concern

Concern about the person's ability to understand or sign documents? Yes ☐ No ☐ Unsure ☐ — if yes or unsure, ask the doctor and attorney what happens next.

■ THE WISHES CONVERSATION (ASK WHILE THE PERSON CAN STILL ANSWER)

What matters most to you if you become seriously ill? _____

What kind of care would you want — and not want? _____

Where would you prefer to receive care if possible? _____

Who do you want involved in medical decisions? _____

What role should faith or spiritual beliefs play? _____

What would you want your family to remember if decisions get hard?

■ CONVERSATION SCRIPTS

With the person: "We want to make sure your wishes are honored if there is ever a time when you cannot speak for yourself. Who would you trust to make medical decisions, and what would you want that person to know?"

With family: "This is not about control. It is about making sure the doctors know who is legally named to speak if Mom or Dad cannot."

With the doctor or facility: "We have a Healthcare Power of Attorney. What is the best way to make sure it is added to the medical record?"

ROB'S NOTE

The Healthcare Power of Attorney is one of those documents families do not appreciate until the room gets serious. Then suddenly everyone wants clarity.

Who can talk to the doctor? Who can make the decision? Who knew what Mom wanted? Who has the paper? Who is the backup?

Do not leave that to memory. Do not leave it to family politics. Do not leave it to the loudest person in the room. Put it in writing. Talk about it while there is time.

The best healthcare agent is not the person who wins the argument. It is the person who can carry the wishes.

CHAPTER 20 TAKEAWAY

A Healthcare Power of Attorney names the person who can make medical decisions if your parent cannot. The next right question is: **who is legally named to speak for this person if they cannot speak for themselves?**

Living Will / Advance Directive

THE PLAIN TRUTH

A Living Will or Advance Directive tells the medical team what kind of care a person would or would not want if they became seriously ill and could not speak for themselves.

A Healthcare Power of Attorney names *who* can speak. A Living Will or Advance Directive helps explain *what* the person would want.

Both matter. The names and forms may vary by state, but the purpose is the same: do not make the family guess in the hardest room of their life. These documents may address life-sustaining treatment, artificial nutrition and hydration, ventilators, CPR, comfort care, and other serious choices. This is not only paperwork. It is a gift — something for the family to hold when emotions are loud and decisions are heavy. The better question is not, "What would we want?" It is: **what did Mom or Dad say they would want?**

WHAT FAMILIES USUALLY THINK

Families often think, "We know what Mom would want," "Dad told me once," "Talking about this will scare them," or "The Healthcare Power of Attorney is enough." The problem is not that families do not care — it is that love does not always remember clearly under pressure. One sibling remembers one conversation; another remembers something different. One says, "Mom would fight." Another says, "Mom said she never wanted machines." Both may be grieving, both sincere, and both wrong if no one wrote it down. A Living Will or Advance Directive helps turn memory into guidance.

Healthcare Power of Attorney vs. Living Will

These often work together, but they are not the same. The Healthcare Power of Attorney names the person who can make decisions — it answers **who speaks**. The Living Will or Advance Directive explains the person's wishes about care — it answers **what they would want**. A family should try to have both: the agent needs authority, and the agent also needs guidance.

What a Living Will May Cover

Depending on the state and document, it may address CPR, ventilator use, feeding tube, artificial hydration, dialysis, surgery, antibiotics, hospital transfer, intensive care, comfort-focused care, palliative care, hospice, organ donation, spiritual preferences, preferred place of care, who should be involved,

what quality of life means, and what treatments would feel too burdensome. The document may not answer every question — that is why the conversation matters too. The paper is the map. The conversation is the lantern.

One more thing to know: a Living Will usually applies only in specific medical situations defined by state law — often a terminal condition or permanent unconsciousness certified by physicians. Ask the attorney or doctor what triggers it, so the family does not over-read its reach.

CPR and Code Status

CPR is often misunderstood. On television it looks quick, clean, and successful. Real life is different, especially for someone who is frail, seriously ill, or near the end of life. Ask the doctor: What would CPR look like for this person? What is the chance it would help? What injuries or complications could happen? Would it lead to a ventilator or ICU stay? Would it match the person's goals? Is a DNR or medical order appropriate? A Living Will may express wishes about CPR, but a medical order such as a DNR, MOST, or POLST may also be needed depending on the state and setting. Do not assume one form does everything. Ask.

Ventilators, Nutrition, and Hydration

Ventilators and intensive care. A ventilator helps someone breathe. For some it is temporary support during a reversible illness; for others it may prolong dying or lead to care they would not want. Ask whether it is expected to be temporary, what you are hoping it will fix, what happens if the person cannot come off it, whether they would want ICU care, and what comfort-focused care would look like instead.

Artificial nutrition and hydration. Feeding tubes and IV fluids feel personal — food means love, water means care. But near the end of life or in advanced illness, the body may not process food and fluids the same way; they may help in some situations and create burdens in others. Ask what problem you are trying to solve, whether it is temporary or long-term, whether it will improve comfort or strength, whether it could cause complications, what comfort feeding means, and what hospice or palliative care would recommend. Do not let guilt make the decision alone. Ask what helps the person.

Comfort-Focused Care

A Living Will can help clarify when the goal should shift from cure to comfort. Comfort-focused care may include pain, breathing, anxiety, and nausea relief; mouth and skin care; gentle repositioning; a quiet room; family presence; spiritual support; hospice; palliative care; and avoiding burdensome treatments. Comfort care is not "doing nothing." It is doing the things that matter when cure is no longer the goal. It is active, focused care — care with the noise turned down.

The Conversation, Memory, and Disagreement

The document should not appear out of nowhere during a crisis — talk while the person can still answer. Ask what matters most if they become seriously ill, what would make life still feel meaningful, what would feel like too much, what treatments they would and would not want, whether they would want to be home, whether they would want hospice, who they trust to speak for them, and what to remember if decisions get hard. Write down the answers; they do not have to be perfect, only honest.

If memory is changing, advance directives become urgent — a person with early dementia may still be able to share wishes and sign documents, depending on capacity, but waiting can close the door. And when family disagrees, the document lets everyone return to the person's stated wishes: "Let's come back to what Dad wrote down." "Mom named comfort as her priority — we need to honor that." "This is not about what we want; it is about what she told us she wanted." The document does not remove grief, but it can reduce guessing.

Give Copies to the Right People, and Review It

Once completed, copies should go to the healthcare agent and backup, the primary doctor, the hospital system, any facility, the palliative or hospice team, the important documents folder, and the attorney. Keep the original in a safe but accessible place. Review it if the person moved to another state, a serious diagnosis occurred, dementia is progressing, wishes changed, the named agent changed, the care setting changed, hospice or palliative care has been discussed, or the document is old or missing. Ask an attorney or qualified legal resource whether it is valid, current, and appropriate for the state.

WHAT TO DO THIS WEEK

1. **Find out whether one exists** — check the documents folder, safe, attorney files, and hospital records.
2. **Read it before the crisis** — do not wait until the hospital asks.
3. **Identify what it says** — CPR, ventilators, feeding tubes, comfort care, hospice — and write down the main points.
4. **Talk with the healthcare agent** so they understand the person's wishes.
5. **Share copies** with the doctor, agent, facility, and care team.
6. **Update if needed** — if it is missing, outdated, or unclear, ask an attorney or legal resource.

■ WORKSHEET — LIVING WILL / ADVANCE DIRECTIVE

This does not replace legal or medical advice. State rules vary — ask an attorney, doctor, or legal resource if the document is missing, outdated, unclear, or if capacity is a concern.

Person receiving care _____

Main caregiver / phone _____

Healthcare agent / phone _____

Document status

Does a Living Will / Advance Directive exist? Yes ☐ No ☐ Unsure ☐

Date signed & state _____

Attorney/office that prepared it / phone _____

Original document location _____

Copy in documents folder? (yes / needs to be added) _____

What does the document address? (check all that apply)

- | | |
|---|---|
| ☐ CPR | ☐ Hospital transfer / intensive care |
| ☐ Ventilator / breathing machine | ☐ Comfort care |
| ☐ Feeding tube | ☐ Hospice / palliative care |
| ☐ Artificial hydration | ☐ Organ donation |
| ☐ Dialysis | ☐ Spiritual wishes |
| ☐ Surgery / antibiotics | ☐ Preferred place of care |

The Wishes Conversation (ask while the person can still answer)

What matters most if you become seriously ill, and what would make life still feel meaningful?

What treatments would you want doctors to try — and what would you not want?

Would you want ICU care or a ventilator if recovery seemed unlikely?

Would you want artificial nutrition or hydration? _____

Would you want hospice or comfort-focused care if the illness progressed?

Where would you prefer to receive care, and who should be involved?

What do you want your family to remember if decisions get hard?

■ QUESTIONS FOR THE DOCTOR

CPR / code status

1. What would CPR likely look like for this person, and what is the chance it would help?
2. Would CPR likely lead to ICU care or a ventilator, and would it match the person's goals?

3. Is a DNR or medical order (MOST / POLST) appropriate, and does the current document match the situation?

Ventilator / feeding tube

1. Would a ventilator be temporary or likely long-term, and what happens if the person cannot come off it?
2. What problem would artificial nutrition or hydration solve, and would it improve comfort or cause burden?
3. What would comfort-focused care look like instead?

Comfort-focused care — what matters most (check all that apply)

- | | |
|--|---|
| ☐ Pain relief | ☐ Avoiding hospital transfer |
| ☐ Breathing comfort | ☐ Staying at home if possible |
| ☐ Anxiety relief | ☐ Hospice / palliative support |
| ☐ Quiet room / family present | ☐ Music, prayer, or familiar items |
| ☐ Spiritual support | |

■ CONVERSATION SCRIPTS

With the person: "We want to understand what kind of care you would want if there ever came a time when you could not speak for yourself. What would matter most to you?"

With family: "This conversation is not about giving up. It is about making sure we do not have to guess later."

With the doctor: "Can you help us understand whether the advance directive matches the medical situation we are facing now?"

ROB'S NOTE

Families do not usually avoid these conversations because they do not care. They avoid them because they care so much it hurts.

But silence does not protect anyone. It leaves the people you love standing in a hospital hallway trying to make sacred decisions with tired minds and broken hearts.

A Living Will or Advance Directive is not about choosing death. It is about choosing clarity. It says, "Here is what matters to me."

That is a gift. Not just to the doctors. To the family.

CHAPTER 21 TAKEAWAY

A Living Will or Advance Directive helps explain what care a person would or would not want if they could not speak for themselves. The next right question is: **what did Mom or Dad say they would want if the decision became hard?**

Financial Power of Attorney

THE PLAIN TRUTH

A Financial Power of Attorney names the person who can help manage money, bills, accounts, property, benefits, and financial decisions if your parent cannot manage them alone. This document matters because care costs money.

Bills still arrive. Insurance still needs handling. Benefits still need applications. Facility paperwork may need signatures. Taxes may still need filing. If memory changes or a crisis happens, the family may suddenly need someone with legal authority to act. Love does not automatically give that authority. Being the adult child does not. Paying attention does not. The better question is not, "Who has been helping with the bills?" It is: **who is legally authorized to handle financial matters if this person cannot?**

WHAT FAMILIES USUALLY THINK

Families often think, "I'm her son, so the bank will talk to me," "I pay Dad's bills already," "My sister is on the account," or "The Healthcare Power of Attorney covers money too." Families can sometimes help informally for a while, but when things get serious, that may not be enough. The bank may not speak with you. The facility may need an authorized signer. A Medicaid application may require financial records. The house may need to be sold. The family may agree emotionally, but the paperwork may still say no. That is why the Financial Power of Attorney matters.

Healthcare POA vs. Financial POA

These are different documents. The Healthcare Power of Attorney answers who can speak to doctors and help make healthcare decisions. The Financial Power of Attorney answers who can manage money, bills, accounts, benefits, and property. A family may need both. The same person may be named for both roles, or different people may be better suited for each — someone may be calm with doctors but terrible with paperwork, or good with money but overwhelmed by medical decisions. Choose wisely.

What It May Allow — and What It Does Not Mean

Depending on the document and state law, it may allow the agent to pay bills, manage bank and retirement accounts, handle insurance and Social Security or pension paperwork, manage taxes, pay caregivers, sign care agreements, apply for benefits, manage or sell property, review mail, work with

accountants and attorneys, manage long-term care insurance claims, and help with Medicaid paperwork. Do not assume the document allows everything — some powers may need to be specifically written. Ask an attorney if the authority is unclear.

It does not mean the agent owns the parent's money, can use it for themselves, or can ignore the person's wishes — and it does not strip siblings of the right to ask reasonable questions. The agent has a fiduciary duty: keep records, avoid mixing money, pay for the person's care, follow the document, avoid self-dealing, and ask for professional guidance when needed. Put plainly: **the money still belongs to the parent. The agent is just trusted to manage it properly.**

Durable, Immediate, or Springing

Durable usually means the document stays effective even if the person becomes incapacitated — which is exactly when families need it. A non-durable power of attorney may stop working at incapacity, depending on state law and wording. Ask whether the document is durable, when it becomes effective, who decides incapacity, what proof is required, and whether banks will accept it.

Immediate vs. springing. Some documents work immediately after signing, letting the agent help with bills and paperwork even while the person still has capacity. Others "spring" into effect only after incapacity is documented — which protects independence but can cause delays if institutions require specific proof. Know which type you have, and make sure it matches the family's reality.

Banks Can Be Difficult

Even with a valid Financial Power of Attorney, banks and financial institutions may have their own review process — they may want an original or certified copy, internal forms, attorney review, an updated document, or identification for the agent. This frustrates families. Do not wait until a bill is overdue or a facility move is pending. If possible, have the agent contact major institutions early and ask what they require. A document in a drawer is good. A document the bank has accepted is better.

When Memory Is Changing, and the Warning Signs

If memory is changing, this document becomes urgent — someone may need to help with bills, accounts, insurance, taxes, care costs, facility contracts, scam protection, and Medicaid or long-term care insurance claims. Financial mistakes happen quietly, and families often notice only after damage is done. Warning signs that help is needed include unpaid bills and late notices, mail piling up, confusion about accounts, large unexplained withdrawals, new "friends" asking for money, scam calls, missed insurance premiums, unfiled taxes, and a parent who becomes defensive or embarrassed about money. These signs do not mean the parent is bad with money — they may mean the system is now too heavy for one person. If memory is changing, check documents now. Do not wait until capacity is gone.

Choosing the Agent, Keeping Records, and Family Trust

The best financial agent is trustworthy, organized, calm, good with paperwork, available, able to keep records and communicate, not financially dependent on the parent, and able to avoid conflicts of interest. Do not choose someone just because they are the oldest or the loudest, or because they "deserve it." Choose the person who can carry the responsibility cleanly. The agent should keep clear records — bills paid, care expenses, reimbursements, transfers, insurance claims, benefit applications, facility and caregiver payments, and major decisions — in whatever format they will actually maintain. Clear records are armor for trust.

Money can cause conflict: one sibling worries it is spent too fast, another that it buys too little care, a third about inheritance or misuse. The agent should avoid unnecessary secrecy while respecting privacy: "The money belongs to Mom; my job is to use it for her care and keep clear records." "We can talk about the care budget, but the priority is Dad's safety and needs." "If we disagree, let's involve the attorney or advisor instead of guessing." Transparency does not mean everyone controls the checkbook — but darkness breeds suspicion. Let some light in.

Do Not Move Money Casually

PAUSE BEFORE MOVING MONEY

If Medicaid may be needed later, do not move money, retitle assets, give gifts, or add names to accounts without advice. A well-meaning transfer can create Medicaid problems; a casual gift can become a penalty issue; adding someone to an account can create tax, ownership, creditor, or family problems. Before financial moves, ask an elder law attorney, Medicaid planner, financial advisor, accountant, or appropriate state resource. Caregiving panic is a bad financial advisor.

WHAT TO DO THIS WEEK

1. **Find out whether one exists** — check the documents folder, safe, attorney files, and bank records.
2. **Identify who is named** — the financial agent and a backup.
3. **Confirm whether it is durable** — does it remain valid if the person becomes incapacitated?
4. **Find out when authority begins** — immediately, or only after incapacity is documented?
5. **Check whether banks will accept it** — the agent should contact major institutions and ask what they require.
6. **Update if needed** — if it is missing, outdated, or unclear, ask an attorney or legal resource.

■ WORKSHEET — FINANCIAL POWER OF ATTORNEY

This does not replace legal, tax, Medicaid, or financial advice. State rules vary — ask a qualified professional if the document is missing, outdated, unclear, or if capacity is a concern.

Person receiving care _____

Main caregiver / phone _____

Document point person / phone _____

Document status

Does a Financial Power of Attorney exist? Yes ☐ No ☐ Unsure ☐

Date signed & state _____

Attorney/office that prepared it / phone _____

Original document location _____

Is it durable? Yes ☐ No ☐ Unsure ☐

When does authority begin? Immediately ☐ After incapacity ☐ Unsure ☐

Named agents

ROLE	NAME	RELATIONSHIP & PHONE	HAS COPY?
Primary agent			
Backup agent			

Institutions to contact (will they accept the document?)

INSTITUTION	CONTACT / PHONE	POA ACCEPTED? (Y/N/PENDING)
Bank		
Investment / retirement firm		
Insurance / long-term care insurance		
Mortgage / landlord / utilities		
Facility business office		

Financial responsibilities needing attention (check all that apply)

- | | |
|---|---|
| ☐ Bills need to be paid | ☐ Medicaid or VA application may be needed |
| ☐ Mail needs to be reviewed | ☐ Taxes need filing |
| ☐ Bank accounts need monitoring | ☐ Home maintenance or possible sale |
| ☐ Care bills or facility contract | ☐ Subscriptions to cancel |
| ☐ Long-term care insurance claim to file | ☐ Fraud or scam risk |

Monthly bills & care costs

BILL / EXPENSE	AMOUNT	DUE	PAID FROM
----------------	--------	-----	-----------

Mortgage / rent / utilities			
Insurance			
Assisted living / memory care			
Private duty care			
Medical bills & medications			

Record-keeping plan

Who will keep records? _____

Where will records be kept? _____

Who receives updates, and how often? _____

■ CONVERSATION SCRIPTS

With the person: "We want to make sure bills, care costs, insurance, and benefits can be handled if there is ever a time when you cannot manage them yourself. Who do you trust to help with financial matters?"

With family: "This is not about taking control. It is about making sure someone has legal authority to pay bills, manage care costs, and keep records if Mom or Dad cannot."

With the bank: "We have a Financial Power of Attorney. What is your process for reviewing and accepting this document?"

ROB'S NOTE

The Financial Power of Attorney is not about greed. It is about groceries, bills, care, insurance, and keeping the lights on.

It is about making sure someone can pay the facility, call the insurance company, manage the bank account, and protect the person from financial confusion or harm.

Money gets emotional in families. That is why the paperwork matters. The agent should be trustworthy. The records should be clean. The purpose should stay clear.

The money belongs to the person receiving care. The job is to use it for their care, safety, dignity, and needs.

CHAPTER 22 TAKEAWAY

A Financial Power of Attorney names the person who can handle financial matters if your parent cannot. The next right question is: **who is legally authorized to manage bills, accounts, benefits, and care costs if this person cannot?**

Medical Orders — DNR, MOST, and POLST

THE PLAIN TRUTH

A living will states wishes. A medical order gives instructions that care teams — including EMS — can act on. That difference is the whole reason this chapter exists.

In an emergency, paramedics generally cannot follow a living will sitting in a folder. They need a signed medical order — an out-of-hospital DNR, or a portable order form such as a MOST or POLST — and it must be where they can see it. Medical orders are written by healthcare providers, based on conversations with the patient or their legal decision-maker, and they travel with the person across settings: home, assisted living, hospital, nursing facility, hospice. The better question is not, "Do we have the paperwork?" It is: **if the heart stops tonight, is there a signed order that says what to do — and can the responders find it?**

WHAT FAMILIES USUALLY THINK

Families often think a DNR means "do not treat," that signing one is giving up, that these forms are for every older adult, or that the living will already covers it. None of that is right. A DNR addresses one thing — CPR — and comfort, pain relief, dignity, and medical care all continue. Portable order forms are intended for people who are seriously ill or frail — someone whose death within the coming year would not surprise their clinician — not for every healthy 70-year-old, who should usually stop at an advance directive. And the order can be changed or voided at any time by the patient (or their surrogate, within state rules). It is not a one-way door. It is a standing instruction that should be reviewed at every care transition.

What Is a DNR?

A Do Not Resuscitate (DNR) order tells healthcare providers not to perform CPR (cardiopulmonary resuscitation) if a person's heart stops or they stop breathing. CPR can include chest compressions, electric shocks, and breathing support. A DNR does not affect other treatments unless specified — a person with a DNR still receives medical care, comfort care, and treatment for everything else.

What Is a MOST? What Is a POLST?

A **MOST** (Medical Orders for Scope of Treatment) form turns a person's preferences into signed medical orders across a range of treatments — CPR, hospitalization, antibiotics, feeding tubes, and

comfort care. Some states use the MOST; many others use a **POLST** — historically "Physician Orders for Life-Sustaining Treatment," now moving to "Portable Orders for Life-Sustaining Treatment" nationally — which does the same job. Names, colors, and signature rules vary by state (in many states a nurse practitioner or physician assistant may sign). Ask the provider: "Which portable medical order form does our state use, and is it appropriate for my parent now?"

Where the Form Lives Matters as Much as What It Says

These forms only work if they can be found in the moment. At home, the original — often a brightly colored form — is usually kept where responders look: on the refrigerator, by the bed, or at the front of the care binder. In a facility, it belongs at the front of the chart. And it should **travel with the person** — to the hospital, to rehab, to assisted living — and be reviewed at every transition and after any major change in health or goals. A signed order locked in a drawer protects no one.

Why These Orders Matter

These medical orders help ensure that a person's wishes are followed during emergencies, that families and caregivers understand what care is wanted, and that healthcare providers have clear instructions when there is no time for a conversation. Have these conversations early — with the provider and with each other. They reduce confusion and panic in the hardest moment a family will face.

WHAT TO DO THIS WEEK

1. **Ask whether a medical order is appropriate now** — these forms are for serious illness and frailty; a healthy parent usually needs only the advance directive.
2. **Find out which form your state uses** — ask the provider, or check Your State Guide at the back of this book.
3. **Have the code-status conversation with the doctor** — ask what CPR would realistically look like for this person (see the questions below).
4. **Get it signed and get copies** — the original stays visible with the person; copies go to the documents folder, the facility, and the decision maker.
5. **Tell the people who would call 911** — family, caregivers, and facility staff should know the order exists and where it is.
6. **Review it at every care transition** — and remember it can be changed or voided at any time.

■ WORKSHEET — THINKING ABOUT YOUR PREFERENCES

This worksheet is not a DNR, MOST, or POLST, and it has no legal or medical effect. Only an order signed by a healthcare provider on your state's official form directs medical care. Use this page to think and talk — then bring it to that conversation. It is written in your parent's voice; hand it to them if they can complete it, or work through it together.

1. If my heart stops, I would want:

- ☐ CPR ☐ No CPR (DNR)

2. If I become seriously ill, I would prefer:

- ☐ Full treatment (all possible medical care)
☐ Limited treatment (some interventions, but not all)
☐ Comfort care only

3. I would want to be cared for:

- ☐ At home ☐ With hospice support, wherever I am
☐ In a hospital

4. I have discussed my wishes with:

- ☐ Family members ☐ Legal decision-maker
☐ Healthcare provider

5. Additional notes about my preferences:

For the family (check when done)

- ☐ Asked the provider whether a MOST/POLST/DNR is appropriate now
☐ Order signed on the state's official form
☐ Original visible with the person (refrigerator / front of chart / binder front)
☐ Copies in the documents folder and with the decision maker
☐ Family, caregivers, and facility know it exists and where it is
☐ Review date set (and at every care transition)

■ QUESTIONS TO ASK THE DOCTOR ABOUT CODE STATUS

1. Given my parent's health, what would CPR realistically look like, and what is the chance it would work?
2. Is my parent someone who should have a portable medical order (MOST/POLST) now, or is the advance directive enough?
3. Which form does our state use, who signs it, and how do we complete it?
4. Where should the original live, and who needs copies?
5. What happens if my parent changes their mind — how is the order changed or voided?
6. Will this order follow my parent to the hospital, rehab, or a facility — and who checks it at each transition?

■ CONVERSATION SCRIPT

With the doctor: "We want my parent's wishes to be followed in an emergency, not just written in a folder. Is it time for a portable medical order — in our state, I understand that may be a MOST or POLST — and can we complete it together?"

With family: "A DNR is not 'do not treat.' It is one instruction about CPR. Everything else — comfort, medicine, dignity, care — continues. We are making sure the paramedics know what Mom wants, so nobody has to guess."

ROB'S NOTE

The word DNR scares people. It can sound like a door slamming.

But a DNR is not "do not treat." It is one instruction about CPR. Comfort, pain relief, dignity, and care all continue.

These orders exist so that in the hardest moment, no one has to guess — the paramedic, the nurse, the family all know what the person wanted. Talk to the doctor early. Ask what CPR would really mean for this person. Then let the order match the wishes.

That is not giving up. That is making sure love has the last word instead of panic.

CHAPTER 23 TAKEAWAY

A DNR, MOST, or POLST turns a person's healthcare wishes into a medical order that care teams — including EMS — can act on, but only if it is signed, current, and findable. The next right question is: **if the heart stops tonight, is there a signed order that says what to do, and can the responders find it?**

Advance Care Planning Conversations

THE PLAIN TRUTH

Advance care planning is the process of talking about future medical care before a crisis forces the conversation. It helps a person say what matters most, who should speak for them, what care they would want, and what care they may not want.

This is not only about documents. It is about clarity. A Healthcare Power of Attorney names who can speak. A Living Will or Advance Directive helps explain what the person would want. Medical orders such as DNR, MOST, or POLST may turn certain wishes into medical instructions when appropriate. Advance care planning is the conversation that ties all of it together. The better question is not, "Do we have the forms?" It is: **have we talked honestly about what matters most?**

Why These Conversations Matter

Families often delay because it feels uncomfortable — they do not want to scare Mom, or make Dad think they are giving up, or talk about decline, hospitals, feeding tubes, CPR, hospice, or death. So they wait. Then the crisis comes: a fall, a stroke, a hospitalization, a dementia decline, a doctor asking about code status, a family meeting no one was ready for. That is a hard time to start from zero. Advance care planning reduces guessing. It helps the family understand the person's wishes before emotions are loud and decisions are urgent. It gives the caregiver something solid to hold.

WHAT FAMILIES USUALLY THINK

Families often think, "We know what she wants," "He told me once," "The forms are enough," "Talking about this means we are giving up," or "Everyone will agree." Maybe. But in a crisis, memory gets shaky. One person remembers one conversation, another remembers something different. Someone feels guilty. Someone wants everything done, someone wants comfort, someone says, "She would never want this," and someone else says, "You are giving up on her." Advance care planning does not remove grief. But it can reduce confusion.

What to Talk About

The conversation should cover what matters most; who should make medical decisions and who should be included in updates; what care feels acceptable and what feels too burdensome; where the person would prefer to receive care; whether they would want hospitalization, ICU care, CPR, a ventilator, or

artificial nutrition and hydration; whether they would want hospice or comfort-focused care; what spiritual or faith needs matter; and what the family should remember if decisions get hard. It does not need to happen all at once. It can start small. One honest question is enough to open the door.

When to Have the Conversation

The best time is before the emergency — while the person can still speak for themselves, before major surgery, after a serious diagnosis or a hospitalization, when dementia or memory changes begin, when falls or weakness increase, when chronic illness worsens, before an assisted living or memory care move, and before hospice or palliative care is needed. Advance care planning is not one conversation forever. What someone wanted at 65 may not be what they want at 85, and what they wanted before living with dementia, cancer, heart failure, COPD, kidney disease, or frailty may change after. Ask again.

Who Should Be Involved

Include the people who need to understand the wishes: the person receiving care, the healthcare agent and backup, the spouse, adult children, the main caregiver, a trusted family member or friend, the doctor, and — if involved or desired — the palliative or hospice team and a clergy or spiritual advisor. Not everyone needs to be in the first conversation. Sometimes the first conversation should be quiet and private. But the decision maker should not be left in the dark. The person named to speak should know what they are being asked to carry.

How to Start

Start gently. You do not need a conference room. You need honesty. Try: "I want to make sure we understand what matters most to you if your health changes." Or: "If there ever came a time when you could not speak for yourself, what would you want us to know?" Or: "I know this is not easy to talk about, but I do not want us guessing later." Do not start with machines. Start with values — with what matters. The medical details can come later.

Wishes vs. Orders, and When It Is Hard

The difference between wishes and orders. A conversation is guidance. A legal document records wishes. A medical order gives instructions to medical professionals. They are connected but not the same — a person may say they do not want CPR and write it in an advance directive, but depending on the state and setting, a DNR, MOST, or POLST may still be needed. Ask the doctor: "Do the documents we have match the medical orders needed for this situation?" Do not assume one form does every job.

If the person refuses to talk, that does not mean the family failed. Try again later with smaller questions. Instead of "Do you want CPR?" ask, "What matters most to you if you get sicker?" Instead of "Do you want hospice?" ask, "Would you want care focused on comfort if treatment was no longer

helping?" Instead of "Where do you want to die?" ask, "Where do you feel safest?" Some doors open slowly. Do not kick them down.

If family members disagree, the most important shift is from "What do I want?" to "What would our loved one choose if they could speak clearly today?" That single question lowers the temperature in many family conflicts. It moves the discussion away from guilt and toward honor. Stay anchored to the person's wishes: "Let's come back to what Mom said mattered most." "Dad told us he wanted comfort if recovery was unlikely." "This is not about what we want. It is about what she told us." The families with the least regret are not the ones who made perfect decisions. They are the ones who knew what their loved one wanted. Advance care planning gives the family a compass. It does not calm the storm, but it keeps people from walking in circles.

WHAT TO DO THIS WEEK

1. **Choose one calm time** — not during a crisis or argument if you can avoid it.
2. **Choose the first person to talk with** — start with the person receiving care, if they can participate.
3. **Ask one values question** — "What matters most to you if your health changes?" — and write the answer down.
4. **Identify the decision maker** — who should speak if the person cannot.
5. **Connect the conversation to documents** — which POA, directive, or medical orders exist?
6. **Share the key wishes** so the healthcare agent and main caregiver understand what was said.

■ WORKSHEET — ADVANCE CARE PLANNING CONVERSATION

Use this to guide conversations about future care, values, wishes, decision makers, and documents. It does not replace legal or medical advice — it helps the family talk before a crisis.

Person receiving care _____

Date of conversation _____

Main caregiver / phone _____

Healthcare decision maker / agent / phone _____

Backup decision maker / phone _____

What matters most? (check any priorities mentioned)

- | | |
|--|---|
| ☐ Staying at home | ☐ Avoiding repeated hospital trips |
| ☐ Being comfortable / avoiding pain | ☐ Being with family |
| ☐ Living as long as possible | ☐ Faith / prayer / spiritual support |
| ☐ Being able to communicate | ☐ Maintaining independence |
| ☐ Recognizing family | ☐ Avoiding machines |
| ☐ Avoiding burden on family | |

In the person's own words _____

What are you hoping for? _____

What are you worried about? _____

What would make life still feel meaningful — and what would feel like too much?

Care preferences

CARE QUESTION	WISHES / NOTES
Hospital transfer / ICU	
CPR	
Ventilator / breathing machine	
Feeding tube / artificial hydration	
Dialysis / surgery / antibiotics	
Palliative care / hospice	
Comfort-focused care	

Preferred place of care (if illness worsens)

- | | |
|--|---|
| ☐ Home | ☐ Skilled nursing facility |
| ☐ Assisted living | ☐ Hospice facility |
| ☐ Memory care | ☐ Unsure |
| ☐ Hospital | |

Faith, spiritual & personal wishes

Faith or spiritual support desired? (yes / no / unsure) _____

Clergy / spiritual contact & phone _____

Music, prayer, rituals, readings, or personal comforts _____

Important people to include _____

Decision maker & documents

Primary decision maker / phone _____

Backup decision maker / phone _____

Does the named person know? Yes ☐ No ☐ Needs conversation ☐

Matches the Healthcare POA? Yes ☐ No ☐ Unsure ☐ No document found ☐

Documents that exist (check)

- ☐ Healthcare Power of Attorney
- ☐ Living Will / Advance Directive
- ☐ DNR / MOST / POLST

- ☐ Financial Power of Attorney
- ☐ HIPAA release
- ☐ Important documents folder

■ QUESTIONS FOR THE DOCTOR

1. Do the current documents match the person's medical situation, and are any medical orders needed?
2. Would a DNR, MOST, or POLST be appropriate?
3. What would CPR likely look like for this person, and what would hospitalization likely accomplish?
4. Would palliative care be helpful, and would hospice ever be appropriate?
5. What should the family understand now?

■ CONVERSATION SCRIPTS

To begin: "I want to understand what matters most to you if your health changes, so we are not guessing later."

If the person is nervous: "This does not mean anything is happening today. It just means your wishes matter enough to write them down."

With family: "We need to listen to what Mom or Dad wants, not just what each of us fears."

With the doctor: "Can you help us understand what medical decisions we should talk about now, before there is a crisis?"

ROB'S NOTE

Advance care planning is not a death conversation. It is a love conversation.

It says, "Your voice matters even if one day you cannot use it." It tells the family where to stand when the room gets hard. It gives the healthcare agent something more than guilt to carry.

The goal is not to predict every medical situation. You cannot. The goal is to understand the person — what matters, what scares them, who they trust, what they hope for, what they would want the family to remember.

Start there. The forms matter. But the conversation is the heart of it.

CHAPTER 24 TAKEAWAY

Advance care planning helps the family understand a person's wishes before a crisis. The next right question is: **have we talked honestly about what matters most, who should speak, and what care would match this person's values?**

Medication and Doctor Lists

THE PLAIN TRUTH

When a parent is aging, sick, declining, or moving between care settings, the medication list becomes one of the most important documents in the house. Not the fanciest. Not the most emotional. But one of the most useful.

A clear medication and doctor list can prevent confusion, delays, duplicate prescriptions, missed doses, unsafe combinations, and unnecessary stress during appointments, hospital visits, rehab stays, home health starts, hospice admissions, and facility moves. Families often think they know the medications — then someone asks: What dose? How often? Who prescribed it? Why? When was the last dose? Any allergies? What pharmacy? That is when guessing starts, and guessing is not a care plan. The better question is not, "Do we know most of the medications?" It is: **can we hand the care team one accurate list today?**

Why the Medication List Matters

A medication list helps the care team understand what is actually being taken — not what was prescribed three years ago, not what is sitting in the cabinet, not what one sibling remembers. What is being taken now. It helps with doctor appointments, ER visits, hospital admissions, rehab, home health, hospice, assisted living or memory care move-ins, refills, side-effect concerns, fall-risk reviews, and end-of-life comfort planning. Include prescriptions, over-the-counter medications, vitamins, supplements, creams, inhalers, eye drops, insulin, patches, and as-needed medications. If it goes in, on, under, or near the body for medical reasons, write it down.

WHAT FAMILIES USUALLY THINK

Families often think, "Mom knows what she takes," "The pharmacy has the list," "The doctor has it in the chart," or "We will bring the bag of bottles." But medication lists are often wrong. Doctors change medications, specialists add them, hospitals stop them, rehab restarts them, pharmacies refill old prescriptions, and patients keep taking discontinued pills. Families forget over-the-counter drugs; supplements, eye drops, and inhalers drop out of the conversation. The medication list needs one owner. Someone has to keep it clean.

What Belongs on the List

For each medication, write the name, dose, how often and what time, the reason (if known), the prescribing doctor, the pharmacy, the start date (if known), special instructions, whether it is daily or as-needed, and any side effects or concerns. Also include allergies and medication reactions; vitamins, supplements, and over-the-counter drugs; pain, sleep, and anxiety medications; blood thinners; diabetes medications; inhalers, eye drops, creams, patches, insulin, and oxygen; medical marijuana or CBD if used and legal in the state; and any medication recently stopped. The stopped medications matter too — a discontinued medication sitting in the cabinet has a way of coming back.

Bring the List, Not Just the Bottles

Bottles can help, but bottles alone are not enough. A bottle shows what was prescribed, not what is actually being taken. The dose may have changed, the medication may have been stopped, old bottles may be mixed with new ones, and duplicates may come from different pharmacies. Bring the bottles when useful, but bring the list every time. The list should travel.

When to Update, and the Medication Review

Update the list after every doctor appointment, hospital visit, or rehab discharge; when home health or hospice starts; when a medication is added, stopped, or a dose changes; when the pharmacy changes; after a fall or worsening confusion, dizziness, weakness, sleepiness, or appetite change; after a new diagnosis; and before an assisted living or memory care move. Do not wait for the annual checkup. Medication changes are little hinges that swing big doors.

Ask for a medication review if the person takes many medications, has new confusion, is falling, feels dizzy, sleeps more, has poor appetite, or has multiple prescribers. A review can find duplicates, unnecessary medications, side effects, interactions, wrong doses, medications that raise fall risk or worsen confusion, medications that no longer match the person's goals, and expensive drugs with alternatives. Ask: "Can we review the medication list to make sure everything is still needed, safe, and appropriate?" Do not stop medications on your own. Ask.

The Doctor List, and One Owner

The doctor list is the medication list's twin — one shows what is being taken, the other who is involved. Include the primary care provider, specialists, pharmacy, hospital preference, home health, hospice, palliative team, facility nurse, private duty agency, equipment company, dentist, eye and hearing providers, therapist, and care manager, with each name, role, phone, portal info, last visit, next appointment, and main reason involved. In a crisis, "the heart doctor" is not enough — write the name down.

The family should choose one medication-list person, responsible for keeping the list updated and sharing the newest version. This does not mean one person does all the caregiving — it means one person guards the list. Choose someone organized, reachable, and willing to update details, and share

the list with the main caregiver, healthcare agent, doctor, facility, and anyone taking the parent to appointments. If three people have three different lists, the family does not have a list.

WHAT TO DO THIS WEEK

- Gather all medications** — bottles, pill packs, inhalers, drops, creams, vitamins, supplements, patches, and OTC drugs.
- Write down what is actually being taken** — ask how each medication is really used, not just what the bottle says.
- Add the pharmacy name and phone number.**
- List every doctor and care contact** — specialists, facility, home health, hospice, and pharmacy.
- Ask for a medication review** for safety, duplicates, side effects, and cost.
- Put the list in the documents folder** — paper and digital — and update it whenever something changes.

WORKSHEET — MEDICATION AND DOCTOR LIST

Create one clear list for appointments, emergencies, hospital visits, home health, hospice, assisted living, memory care, or rehab.

Person receiving care _____

Date last updated _____

Medication-list person / phone _____

Primary pharmacy / phone _____

Allergies & medication reactions _____

Current medication list

MEDICATION	DOSE	HOW OFTEN / TIME	REASON	PRESCRIBER

As-needed medications

MEDICATION	DOSE	WHEN USED	MAX ALLOWED
------------	------	-----------	-------------

Over-the-counter, vitamins & supplements

ITEM	DOSE	HOW OFTEN	REASON

Recently stopped medications

MEDICATION	DATE STOPPED	WHO STOPPED IT?	REASON

Medication concerns (check any that apply)

- | | |
|---|--|
| ☐ Missed or extra doses | ☐ Dizziness, falls, or sleepiness |
| ☐ Confusing instructions | ☐ New confusion or poor appetite |
| ☐ Duplicate medications | ☐ Nausea or constipation |
| ☐ Old bottles still in use | ☐ Trouble swallowing pills |
| ☐ Multiple pharmacies or prescribers | ☐ Refusing medications |
| ☐ High cost | |

Doctor & care team list

NAME	ROLE / SPECIALTY	PHONE	NEXT VISIT
Primary doctor			
Specialist			
Specialist			
Pharmacy			
Home health / hospice			
Facility nurse			

Medication safety plan

Who fills the pill box, and how often? _____

Who checks for missed doses? _____

Where are medications stored? _____

Are old medications removed from the home? (yes / no / needs attention)

Who updates the list after changes? _____

■ APPOINTMENT QUESTIONS

1. Is this medication list accurate, and are any medications duplicated or no longer needed?
2. Could any medication increase fall risk, worsen confusion, or cause dizziness or sleepiness?
3. Are there lower-cost alternatives?
4. Are there medications that should not be taken together?
5. Should any medications be taken at a different time?
6. What should we do if a dose is missed?

HOPE'S NOTE — FROM THE PHARMACIST

When a family hands me a current, complete medication list, the whole visit changes — we catch duplicates, interactions, and doses that no longer fit the person. When they hand me a bag of bottles, I am guessing about what is actually being taken.

Bring the list. Update it every time something changes. And use one pharmacy if you can — it is the only way any pharmacist ever sees the whole picture.

ROB'S NOTE

A medication list is not just paperwork. It is protection.

It protects your parent from guessing. It protects the doctor from missing something. It protects the caregiver from panic. It protects the family from standing in the emergency room trying to remember whether the little white pill was for blood pressure, sleep, anxiety, or pain.

Write it down. Update it. Bring it with you.

The list does not need to be pretty. It needs to be right.

CHAPTER 25 TAKEAWAY

A current medication and doctor list helps the family and care team make safer, faster, clearer decisions. The next right question is: **can we hand the care team one accurate list today?**

Emergency Contacts and Crisis Plan

THE PLAIN TRUTH

Every family needs one page that answers the crisis question: **who do we call first?**

When something happens — a fall, a fever, a breathing change, sudden confusion, a late-night decline — people do not think clearly. In those moments, families do not need a search party for phone numbers. They need one page. This chapter helps the family build a simple emergency contact and crisis plan so the right person can call the right number at the right time. The better question is not, "Does someone have the number?" It is: **can the caregiver find the right number in thirty seconds?**

Why This Matters

Most families have contact information scattered everywhere — a doctor's number in one phone, the pharmacy on a pill bottle, the hospice number in a folder, the assisted living nurse's name in a text thread, the power of attorney in a drawer, and an out-of-town sibling who has no idea who to call. That works until something happens. Then the family loses time, and time matters. An emergency contact plan does not solve every problem, but it keeps the family from spinning. It creates a first step.

WHAT FAMILIES USUALLY THINK

Families often think, "We'll just call 911," "The facility will handle it," "My sister knows who to call," or "We can figure it out." Sometimes 911 is the right call. Sometimes the doctor, hospice, facility nurse, home health agency, or on-call provider should be called first — and calling 911 before calling hospice or the facility team can create confusion the family did not want. Sometimes waiting too long creates danger. The family needs a plan. Not a perfect plan. A usable one.

When to Call 911

CALL 911 IMMEDIATELY

For immediate danger or a life-threatening emergency: chest pain, severe trouble breathing, signs of stroke, uncontrolled bleeding, major injury, unconsciousness, a severe allergic reaction, a suspected overdose, a serious fall with injury, a person who cannot be safely moved, immediate danger to self or

others, or fire, violence, or an unsafe environment. **If you are unsure whether something is life-threatening, call 911. Do not let a checklist delay emergency help.**

When to Call the Doctor, Hospice, Home Health, or Facility

The doctor or nurse triage line for concerns that are serious but not clearly life-threatening — fever, new weakness, worsening confusion, medication side effects, new swelling or pain, dehydration, a recent fall without obvious major injury, or a question about whether the ER is needed. Use this script: "Something has changed, and I need guidance on what to do next," then describe the change clearly.

Hospice, if the person is on it, for changes related to comfort, symptoms, decline, or end-of-life concerns — pain, shortness of breath, agitation, nausea, breathing changes, not eating or drinking, or a family sense that death may be near — unless there is immediate danger requiring 911. Say: "We are on hospice and something has changed. I need the nurse or on-call team to guide us." Ask hospice who to call after hours and put that number where everyone can find it.

Home health for questions about the plan of care, wounds, therapy, nursing visits, medication teaching, equipment, or safety concerns being monitored by the team. Home health is not an emergency service — if there is immediate danger, call 911.

The facility, if the person lives in assisted living, memory care, rehab, or a nursing home. Ask in advance who to call during the day and after hours, who the nurse and administrator are, who calls the doctor, what changes require family notification, and what changes require 911. Do not wait until the facility calls at midnight — get the contact plan in writing.

The Family Call Chain

A call chain prevents ten people from calling ten people with half-information. Choose one family point person who updates the rest of the family; if that person is unavailable, a backup takes over. The plan should answer: Who gets called first? Who calls the doctor or agency? Who updates family? Who goes to the hospital? Who stays with the spouse or other parent? Who brings the document folder? Who handles medications, pets, home, or transportation? In a crisis, roles beat opinions.

What to Keep Ready, and What to Say

The emergency page should include the person's full name, date of birth, and address; main diagnosis and allergies; where the medication list is; doctor names and numbers; pharmacy; insurance information; the healthcare agent; family, facility, home health, and hospice contacts; preferred hospital; DNR/MOST/POLST status if applicable; funeral home if chosen; and the document folder location. Print it, store it digitally if possible, and put it where the caregiver can grab it.

A clear call gets better help. When you call, give the name of the person receiving care, their age, location, main diagnosis, what changed and when, current symptoms, any recent medication change, and what you need. For example: "My mother, Mary Smith, is 84 and lives at home. She has dementia and heart failure. Since this morning she is much weaker, more confused, and not eating. She fell yesterday but did not go to the hospital. I need guidance on whether she should be seen today." Simple. Clear. Useful.

If the Caregiver Breaks Down

A caregiver crisis is still a crisis. Call for help if the caregiver has not slept, feels unsafe, cannot lift or transfer the person, cannot manage medications, feels emotionally overwhelmed, is afraid they may yell or collapse, cannot leave the person alone, has their own medical emergency, or is no longer able to provide safe care. Families sometimes wait until the caregiver is completely broken. Do not wait. Use this script: "**The caregiver cannot safely continue at this level. We need help making a safer plan today.**" That is not weakness. That is honesty.

WHAT TO DO THIS WEEK

1. **Choose the main family point person** and a backup.
2. **Write down the most important medical contacts** — doctor, specialists, pharmacy, facility, home health, hospice, and preferred hospital.
3. **Write down when to call 911** so the caregiver knows what counts as immediate danger.
4. **Put the contact page somewhere visible** — refrigerator, medication area, caregiver binder, or hospital bag.
5. **Build the family call chain** — who calls whom, and who does what.
6. **Share the plan** with the caregiver, point person, healthcare agent, backup caregiver, and facility.

■ WORKSHEET — EMERGENCY CONTACTS & CRISIS PLAN

Create one clear emergency page. The goal: the right person can find the right number fast.

Person receiving care / date of birth _____

Address _____

Main diagnosis / conditions _____

Allergies _____

Medication list location _____

Preferred hospital _____

Important documents folder location _____

Family point people

ROLE	NAME	PHONE
Main family point person		
Backup point person		
Healthcare agent		
Main caregiver		
Backup caregiver		

Medical contacts

CONTACT	NAME / ORGANIZATION	PHONE	AFTER-HOURS
Primary doctor			
Specialist			
Pharmacy			
Home health			
Hospice			
Facility nurse			

Call 911 for (check to confirm the caregiver knows)

- | | |
|---|---|
| ☐ Chest pain | ☐ Uncontrolled bleeding |
| ☐ Severe trouble breathing | ☐ Serious fall or major injury |
| ☐ Stroke signs — sudden face drooping, arm weakness, or speech trouble (note the time) | ☐ Severe allergic reaction |
| ☐ Unconsciousness | ☐ Fire or unsafe environment |
| | ☐ Immediate danger to self or others |

Hospice / home health

On hospice? Yes ☐ No ☐ Receiving home health? Yes ☐ No ☐

Hospice agency / main & after-hours number _____

Hospice nurse _____

Home health agency / number _____

Instructions from the team _____

Documents & medical orders (check what applies)

- | | |
|---|--|
| ☐ Healthcare POA | ☐ Living Will / Advance Directive |
|---|--|

- ☐ DNR / MOST / POLST
- ☐ Medication & allergy list

- ☐ Insurance / Medicare / Medicaid / VA cards
- ☐ Long-term care insurance

Family call chain

ORDER	WHO GETS CALLED?	PHONE	WHO CALLS THEM?
1			
2			
3			

Before you call — what changed?

Awake & responsive? Yes ☐ No ☐ Unsure ☐ Fall? Y ☐ N ☐ Breathing trouble? Y ☐ N ☐

Chest pain? Y ☐ N ☐ New confusion? Y ☐ N ☐ Fever? Y ☐ N ☐ New pain? Y ☐ N ☐
☐ Medication change? Y ☐ N ☐

■ CRISIS CALL SCRIPT

"My name is _____. I am calling about _____, who is _____ years old and is located at _____. The main diagnosis is _____. Something changed: _____. It started around _____. Current symptoms are _____. The medication list is at _____. I need guidance on what to do next."

ROB'S NOTE

A crisis plan will never feel dramatic until the crisis arrives. Then it becomes gold.

The number on the fridge. The hospice line by the phone. The doctor's name written clearly. The family point person already chosen. The medication list in the folder.

That is what helps a family move — not because everything is easy, but because the first step is clear.

When people are scared, they do not need more noise. They need a number, a name, and the next right call.

CHAPTER 26 TAKEAWAY

An emergency contact and crisis plan gives the family one clear place to find who to call and what to say when something changes. The next right question is: **can the caregiver find the right number in thirty seconds?**

The Last Days Plan

Part 5 — Hospice and the Last Days. This is the most tender part of the toolkit. These chapters walk with a family through the final season — what the last days may look like, what comfort means, what to expect and what to do, and what happens after death. The goal is not to control death or predict its timing. It is to help the family show up — calmer, clearer, and more present — for the person they love. If someone is in immediate danger and emergency treatment is wanted, call 911.

“You matter because you are you, and you matter to the end of your life.”

— DAME CICELY SAUNDERS, FOUNDER OF THE MODERN HOSPICE MOVEMENT

THE PLAIN TRUTH

The last days of life are hard enough without the family trying to build a plan in the middle of them.

A last days plan helps the family know what to expect, who to call, what comfort means, where documents are, who should be present, and what the person would want if time becomes short. This is not about predicting the exact moment. No one can do that. It is about preparing the family so the room can become calmer, softer, and more focused on the person receiving care. The better question is not, “How much time is left?” It is: **what can we do now to make the last days more peaceful, supported, and clear?**

Why This Plan Matters

Families often wait until the final days to ask the hardest questions: Who should be called? Should we go to the hospital? Who has the hospice number? What medications are for comfort? Where is the DNR? Should we keep offering food? What if breathing changes? What if they stop waking up? Who needs to say goodbye? What happens when death occurs? Without a plan, fear fills the room. With a plan, the family still grieves, but they are not guessing every minute. A last days plan gives everyone something steady to hold.

WHAT FAMILIES USUALLY THINK

Families often think, “We will know what to do,” “The nurse will be there the whole time,” “We should call 911 if breathing changes,” “If they stop eating, we are starving them,” or “If we give comfort

medicine, we are causing death." These thoughts come from fear and love. But fear and love need guidance. The last days plan helps turn panic into care.

What the Last Days May Look Like

Every person is different, but families may notice more sleeping; less interest in food and fluids; more weakness; less talking; more confusion; changes in breathing; cool hands or feet; changes in skin color; restlessness; less urine; difficulty swallowing; pulling away from activity; moments of alertness followed by sleep; seeing or talking to people who are not present; less ability to take pills; and more need for mouth care and repositioning. These changes can be frightening if the family does not expect them. Ask the hospice nurse, palliative care team, doctor, or facility nurse what signs to watch for and what to do. Do not rely on internet panic. Use the care team.

Comfort Becomes the Priority

In the last days, comfort may become the main goal — pain relief, breathing comfort, anxiety and restlessness support, mouth care, gentle repositioning, skin care, a quiet room, dim lights, familiar music, prayer, family presence, welcome touch, fewer interruptions, avoiding unwanted transfers, and honoring the person's wishes. Comfort care is not doing nothing. It is doing the right things when the body is changing. It is focused care.

Food, Fluids, and Breathing Changes

Food and fluids. Families often struggle when a loved one stops eating or drinking, because food and water are love. But near the end of life the body changes — appetite decreases, swallowing may become harder, and food and fluids may no longer be helpful to the person. Ask the care team whether it is safe to offer food or fluids, what swallowing trouble looks like, whether to offer small sips or mouth swabs, and what comfort feeding means. Do not force food or fluids. Offer comfort. Mouth care often becomes more helpful than pushing intake.

Breathing changes. Breathing may become slower, faster, irregular, noisy, shallow, paused, or labored-looking. Call the care team when breathing changes and ask what is expected, what comfort medications may help, and when the nurse should come. If the person is on hospice, call hospice; if they are not on hospice and there is immediate danger or the family wants emergency treatment, call 911. Know ahead of time who to call first, whether you are trying to avoid hospital transfer, whether there is a DNR/MOST/POLST, and what comfort looks like. A breathing change should not be the first time the family asks these questions.

Comfort Medications

Families may be afraid of comfort medications, worrying that giving them means causing death. Discuss this fear directly with the nurse or doctor. Ask what each medication is for, when and how much to give, how often, what to watch for, what to do if the person cannot swallow, who to call if it does not work, and how to know the person is comfortable. **Never guess with comfort medications.** Let the care team teach the family, and write down the instructions.

The Room, the People, and What to Say

Who should be there. The plan should include who to call if time becomes short — spouse, children, siblings, close friends, clergy or a spiritual advisor, the hospice chaplain, and anyone who needs a chance to say goodbye. Not everyone needs to be in the room at once. Sometimes the most loving room is quiet; sometimes it is full. Ask what the person would want.

What the room should feel like. A room can be cared for too. Consider music or quiet, prayer, dim lights, the TV off, familiar photos or blankets, limited or open visiting, and soft voices. Remember the person may still hear or sense presence. Speak with respect. Tell stories. Say what needs to be said. Do not argue in the room — the room is not the place for family politics.

What to say. There is no perfect script. Simple words are enough: "I love you." "Thank you." "I forgive you." "Please forgive me." "We are here." "You are not alone." "We will take care of each other." "You can rest." Not every family can say every sentence. That is okay. Say what is true. Say what brings peace.

If Family Disagrees, and When Death Occurs

If family members disagree in the last days — one wants the hospital, another comfort at home; one fears medication, another fears suffering — return to the person's wishes. Ask what they said they wanted, what documents exist, who the healthcare decision maker is, and what the hospice nurse recommends. Use this script: "This is hard for all of us. Let's come back to what Mom wanted and what keeps her most comfortable now." The goal is not to win the argument. The goal is to protect the person.

When death occurs, the family should already know what to do. Ask ahead: Who do we call first? If on hospice, do we call hospice? If in a facility, who does the facility call? Who pronounces death? Do we call 911? Is a funeral home selected, and who calls them? Who notifies family? What happens to medications and equipment? Who stays with the body until the funeral home arrives? What spiritual or cultural wishes should be honored? If the person is on hospice, the hospice team should explain the process clearly. Write it down before the moment comes.

WHAT TO DO THIS WEEK

1. **Ask what signs to expect** from the hospice nurse, palliative team, doctor, or facility nurse.
2. **Confirm who to call first** — hospice, facility nurse, doctor, or 911, depending on the situation.
3. **Review documents** — POA, advance directive, DNR/MOST/POLST.

4. **Choose the family point person** to update family and coordinate calls.
5. **Decide who should be notified** before the final hours.
6. **Ask about comfort medications and the death process** — have the nurse or doctor explain both.

■ WORKSHEET — LAST DAYS PLAN

Use this when a serious illness is progressing or hospice is involved. It does not replace medical advice — call the care team with any concern, and 911 for immediate danger if emergency treatment is wanted or needed.

Person receiving care / current location _____

Main caregiver / phone _____

Family point person / phone _____

Healthcare decision maker / phone _____

Current care team

ROLE	NAME / AGENCY	PHONE	AFTER-HRS
Primary doctor			
Hospice agency / nurse			
Facility nurse			
Pharmacy			
Funeral home			
Clergy / spiritual			

Who do we call first? (check the plan)

- | | |
|--|---|
| ☐ 911 for immediate danger or emergency treatment | ☐ Facility nurse |
| ☐ Hospice | ☐ Primary doctor |
| | ☐ Palliative care team |

First call number _____

Backup call number _____

Documents confirmed (check what exists, note location)

- | | |
|--|---|
| ☐ Healthcare POA | ☐ Medication list |
| ☐ Living Will / Advance Directive | ☐ Insurance cards |
| ☐ DNR / MOST / POLST | ☐ Funeral home information |

Comfort priorities (check what matters most now)

- | | |
|--|--|
| ☐ Pain relief | ☐ Prayer / spiritual support |
| ☐ Breathing comfort | ☐ Music |
| ☐ Anxiety / restlessness relief | ☐ Familiar blanket or photos |
| ☐ Quiet room | ☐ Avoiding hospital transfer if possible |
| ☐ Family nearby | ☐ Staying in current location if possible |

Symptoms to watch (ask the care team what to do)

SYMPTOM	WHAT WE WERE TOLD TO DO	WHO TO CALL
Pain		
Shortness of breath		
Anxiety / restlessness		
Breathing changes		
Trouble swallowing		
Not eating / drinking		

Comfort medications (use only as directed by the care team)

MEDICATION	WHAT IT IS FOR	DOSE / INSTRUCTIONS	WHEN TO CALL

Who taught the family how to use these, and when? _____

People to notify if time becomes short

NAME	RELATIONSHIP	PHONE	WHEN?

What to say (check words the family may want to share)

- | | |
|---|---|
| ☐ I love you. | ☐ We are here. |
| ☐ Thank you. | ☐ You are not alone. |
| ☐ I forgive you. | ☐ We will take care of each other. |
| ☐ Please forgive me. | ☐ You can rest. |

If death occurs (ask the care team ahead of time)

Who do we call first? / phone _____

Who pronounces death? _____

Funeral home & who calls them _____

Who updates family? _____

What happens with medications & equipment? _____

Special faith, cultural, or family wishes _____

Family role assignments

ROLE	PERSON RESPONSIBLE	PHONE
Main bedside contact		
Family updates		
Calls hospice / nurse		
Handles documents		
Calls funeral home		
Supports spouse / caregiver		

ROB'S NOTE

The last days are sacred ground. They are also practical.

Someone needs the number. Someone needs the medication instructions. Someone needs to know who to call. Someone needs to know where the documents are. Someone needs to tell the family.

That does not make the moment less holy. It makes it more protected.

The goal is not to control death. The goal is to care well until it comes. Quietly. Clearly. Together.

CHAPTER 27 TAKEAWAY

A last days plan helps the family prepare for comfort, communication, calls, documents, visitors, and what to do when death occurs. The next right question is: **what can we put in place now so the final days are less chaotic and more peaceful?**

What to Expect in the Last Days

THE PLAIN TRUTH

The last days of life can feel frightening when the family does not know what they are seeing. Breathing changes. Eating stops. Sleeping increases. Words become fewer. Hands and feet may cool. The person may seem between here and somewhere else.

Families may wonder: Is this normal? Are they suffering? Should we call 911? Are we supposed to keep feeding them? This chapter does not predict exactly what will happen — every person is different — but it gives families a plain-language guide to common changes, what they may mean, what to ask the care team, and how to respond with calm, comfort, and dignity. The better question is not, "Is this the exact sign?" It is: **what is changing, who should we call, and what comfort support is needed now?**

Why Families Need to Know

The body often changes in the last days, and these changes can look scary if no one explains them. A family may panic when food intake drops, fear breathing changes, worry that comfort medication is too strong, or think the person is suffering when the body is naturally slowing. Knowing what to expect does not remove grief, but it can reduce panic. It helps the family stay present, call the right person, and focus on comfort instead of fear.

WHAT FAMILIES USUALLY THINK

Families often think, "If they stop eating, they will starve," "If breathing changes, we should call 911," "If they cannot talk, they cannot hear us," "If we give comfort medicine, we are speeding things up," or "If they rally, they are getting better." Some of these thoughts come from love, some from fear, most from not knowing. The care team should explain what changes are expected and what needs immediate attention. Families should never be left guessing.

Sleeping, Eating, and Drinking Less

More sleeping. The person may sleep much more, be awake only briefly, and drift in and out. This is hard for family members who want one more long talk, but sleepiness is often part of the body slowing down. Sit nearby, speak softly, avoid forcing alertness, keep the room calm, tell them who is present, and say what needs to be said. Do not assume they cannot hear.

Eating less. The person may refuse meals, take only bites, or cough. Food is love, but near the end of life the body may not need or tolerate food the same way, and forcing it can cause distress, choking, or aspiration. Ask the care team whether it is safe to offer food, which foods are safest, and what comfort feeding means. Offer, do not force; use small amounts; stop if coughing or distress occurs; keep the mouth clean and moist. The family is not starving the person. The illness is changing the body.

Drinking less. The person may stop asking for fluids, have trouble swallowing, or cough with liquids. Often mouth care becomes more helpful than forcing fluids. Ask whether small sips or thickened liquids are safe and how often to do mouth care. Offer small sips only if safe, use mouth swabs if recommended, apply lip balm, keep the mouth moist, and do not force fluids. Comfort is the guide.

Breathing Changes

Breathing changes are often the most frightening part for families. Breathing may become slower, faster, uneven, shallow, noisy, paused, wet-sounding, or labored-looking. (Families sometimes hear noisy, wet breathing called "the death rattle" — unsettling to hear, but usually not a sign of suffering.) Call the care team when breathing changes — if the person is on hospice, call hospice. Ask whether it is expected, whether the person appears uncomfortable, whether to reposition, whether comfort medicine or oxygen should be used, and what signs mean urgent help is needed. Stay calm, keep the room calm, reposition gently if instructed, use comfort medications only as directed, avoid crowding, and speak softly. Noisy breathing does not always mean the person is suffering — but still call and ask for guidance.

Skin, Circulation, and Urine

Cool hands and feet. Hands, feet, arms, or legs may become cool, and skin may look pale, gray, bluish, or mottled as circulation changes. Use light blankets if comfortable, do not use heating pads unless instructed, avoid heavy blankets that cause discomfort, and tell the nurse what you see. The goal is comfort, not warming the body back to normal — the body may be doing what it does near the end.

Less urine. Urine may decrease and darken as intake drops and the body slows. Call the nurse if there is no urine for a long time, signs of discomfort, bladder fullness, fever, pain, agitation, or catheter concerns. Track changes and keep the person dry and clean. Do not guess. Ask.

Restlessness, Visions, and Pain

Restlessness or agitation. Some people become restless, pull at blankets, try to get up, call out, or seem anxious. Ask the care team whether it could be pain, anxiety, urinary retention, constipation, or medication, and what comfort step to use. Stay calm, reduce noise, keep the person safe, do not argue with what they are seeing, offer reassurance, and use medications only as directed. Restlessness is not a family failure. It is a symptom to report and manage.

Confusion or visions. Some people talk to loved ones who have died, reach toward something, or speak about going home. Often correction is not needed. Listen, reassure, avoid arguing, do not shame, and let

the moment be peaceful. Sometimes the kindest response is: "You are safe. We are here with you."

Pain. Do not assume pain is unavoidable. Signs may include grimacing, moaning, restlessness, guarding, a tense body, faster breathing, or a furrowed brow. Ask how to know if they are in pain, what medication is ordered, when to give it, what to do if they cannot swallow, how quickly it should help, and when to call. Pain management is a central goal of comfort care. The family should not have to improvise.

The Rally, and Hearing

The rally. Sometimes a person has a period of unexpected alertness — they wake, talk, ask for food, or recognize people, and the family feels hopeful. The rally can be meaningful and beautiful, but it does not always mean recovery. Receive the moment, say what matters, do not overload the person, let them rest afterward, and avoid making sudden major decisions based only on one good hour. A rally can be a gift. Let it be one.

Hearing may remain. Even when a person cannot respond, they may still hear or sense presence. Speak as if they can hear: "We are here." "You are safe." "We love you." "Thank you." "You can rest." Avoid arguing in the room, and avoid talking over the person as if they are already gone. The room should protect their dignity.

When to Call the Care Team

Call when breathing changes, pain seems uncontrolled, restlessness increases, the person cannot swallow medications, the family is unsure what to do, there is a fall or bleeding, there is fever or distress, skin changes are concerning, the caregiver is overwhelmed, death seems near, or death has occurred. If on hospice, call hospice. If not on hospice and emergency treatment is wanted or needed, call 911. If unsure, call for guidance. Families should not sit alone with unanswered fear.

WHAT TO DO THIS WEEK

1. **Ask what changes to expect** from the nurse, doctor, hospice, or palliative team.
2. **Ask what symptoms require a call** — write down when and who to call.
3. **Review comfort medications** — know what each is for and how to use it safely.
4. **Prepare the room** with comfort items, music, blankets, photos, or prayer items.
5. **Prepare the family** — tell key members what changes may happen so they are not shocked.
6. **Write down the death process** — who to call, who pronounces, who calls the funeral home.

■ FIELD GUIDE — LAST DAYS SIGNS & COMFORT

A companion to the **Chapter 27 Last Days Plan** — keep the two together. Care team numbers, comfort medications, who to call first, what to say, and the after-death steps all live on that plan. This page is for what

you are seeing and what helps. It does not replace medical advice — call the care team with concerns, and 911 for immediate danger if emergency treatment is wanted or needed.

Common changes to watch (check what is happening)

- | | |
|--|--|
| ☐ Sleeping more / less talking | ☐ Cool hands or feet / skin color changes |
| ☐ Less eating / less drinking | ☐ Less or darker urine |
| ☐ Trouble swallowing | ☐ Pain signs |
| ☐ More weakness or confusion | ☐ Seeing or talking to people not present |
| ☐ Restlessness or agitation | ☐ Short rally or burst of alertness |
| ☐ Breathing changes / noisy breathing | |

Eating, drinking & mouth care

Food instructions from the care team _____

Fluid instructions _____

Swallowing concerns? (yes / no / unsure) _____

Mouth care instructions _____

Supplies needed: Mouth swabs ☐ Lip balm ☐ Small spoon ☐ Thickened liquids (if ordered) ☐
Towels ☐

Breathing, pain & restlessness plan

QUESTION	WHAT THE CARE TEAM TOLD US
What breathing changes may happen, and what means discomfort?	
What position or medication may help breathing?	
How do we know if the person is in pain, and what is ordered?	
What might cause restlessness, and what should we try first?	
When should we call?	

Room comfort (check what would help)

- | | |
|---|---|
| ☐ Quiet | ☐ Familiar scent |
| ☐ Soft music | ☐ TV off |
| ☐ Prayer / scripture / readings | ☐ Limited visitors |
| ☐ Dim lights | ☐ Open visiting |
| ☐ Favorite blanket / family photos | |

If death seems near

What signs suggest death may be close? (ask the care team) _____

Who to call, who pronounces, the funeral home, and what happens with medications and equipment: see your Chapter 27 plan.

■ IF THE FAMILY IS UNSURE — SCRIPT

"We are seeing changes and we are not sure what they mean. Can you help us understand what is expected, what may be discomfort, and what we should do next?"

ROB'S NOTE

The last days can look different than families expect. That does not mean the family is doing something wrong. The body is changing, and the work changes too.

Less fixing. More comforting. Less forcing. More listening. Less panic. More presence.

Call the care team. Ask what you are seeing. Write down the instructions. Then come back to the person in the bed. Hold a hand. Speak gently. Protect the room.

The family does not have to understand every sign perfectly. They just need to know who to call, what comfort looks like, and that their presence still matters.

CHAPTER 28 TAKEAWAY

The last days may bring changes in sleep, eating, drinking, breathing, skin, awareness, and comfort needs. The next right question is: **what are we seeing, who should we call, and what comfort support is needed now?**

After Death

THE PLAIN TRUTH

When death comes, the family does not need to rush. This is one of the most important things to know: **in most situations, there is time.**

Unless the situation requires otherwise, the family can pause. They can sit. They can breathe. They can say goodbye. When someone is on hospice, or a death is expected and a plan is in place, there is usually no need to call 911 or hurry anyone out of the room. This chapter walks the family through what to do and what not to do in the first hours after a death — the calls, the paperwork, the next steps — so that the sacred part is protected and the practical part does not fall to a family that is too stunned to think. The better question is not, "What do we do first?" It is: **who do we call, and what can wait until we are ready?**

Why This Matters

The moments after a death are a strange mix of the holy and the practical. The family is grieving, and yet phone calls have to be made, a funeral home has to come, and decisions arrive that no one wants to make. Families who have a plan move through these hours more gently. Families without one often panic, call 911 by reflex, and find themselves swept into a chaotic scene the person never wanted. Knowing the steps ahead of time is a gift — it lets the family stay in the room a little longer, and it keeps the first hours from becoming a second trauma.

WHAT FAMILIES USUALLY THINK

Families often think they must call 911 immediately, that the body must be moved right away, that they cannot touch or sit with their loved one, or that everything must be handled in the first hour. In an expected death with hospice or a plan in place, none of that is usually true. Calling 911 for an expected death can bring paramedics and sometimes police, and can turn a peaceful bedside into an emergency scene. If there is any doubt about what to do, the family should know in advance who to call — and it is usually the hospice or the doctor, not 911.

If the Person Was on Hospice

If the person was on hospice, **call hospice first — not 911.** The hospice team will guide the family through the next steps, send a nurse to confirm the death — in many states the hospice nurse can

pronounce it, or will arrange for it — and help notify the physician and the funeral home. The nurse can also dispose of medications properly and arrange for equipment. This is one of the quiet gifts of hospice: the family does not have to manage the after-death process alone. Keep the hospice after-hours number where it can be found, and when you call, simply say, "We believe our loved one has died." The team takes it from there.

If the Person Was Not on Hospice

If the person was not on hospice, what to do depends on the setting and whether the death was expected. In a facility, the staff will usually know the process — they will notify the nurse and physician and help contact the funeral home. At home with an expected death and a doctor who will sign the death certificate, the family may be able to call that doctor or an on-call line. If the death was unexpected, or there is no plan and no clear person to call, calling 911 may be necessary. Understand that this may bring emergency responders — and, in some areas, law enforcement — and may lead to resuscitation attempts unless a valid DNR or POLST is present and visible. This is exactly why the documents in Part 4 matter, and why they should be easy to find.

Taking Your Time in the Room

Once the immediate calls are made, or while you are deciding, it is okay to stay. Many families find comfort in sitting with their loved one, holding a hand, praying, playing music, reading scripture, or simply being present. Some families like to bathe or dress the person, or to gather others so everyone can say goodbye. There is usually no rush for the funeral home to arrive. Ask the hospice nurse or funeral home how much time you have — it is often more than families expect. This is a moment you do not get back. If it helps you to linger, linger.

When the Funeral Home Comes

When the family is ready, the funeral home is called to take the person into their care. If a funeral home was chosen ahead of time, this call is simple. If not, the hospice or facility can help, or the family can choose one now. When the funeral home arrives, family members may stay or step away — there is no right choice, only what each person can bear. Some want to accompany their loved one to the door; some cannot watch. Both are acts of love. The funeral home will explain the next steps, including when to come in to make arrangements and what documents they will need.

The First Days of Practical Tasks

In the days after, practical tasks arrive — often more than the family expects. These may include obtaining copies of the death certificate (get several; many institutions require an original), meeting with the funeral home, notifying Social Security, contacting pension and insurance companies, notifying banks and closing or changing accounts, canceling appointments and services, and beginning to sort belongings. None of this has to happen at once. Choose one point person to lead, and let the rest wait.

Grief and paperwork do not keep the same schedule, and the paperwork can almost always wait a little longer than it feels like it can.

WHAT TO DO — IN THE FIRST HOURS

1. **Pause.** Unless the situation requires otherwise, there is time. Breathe before you dial.
2. **Call the right first number** — hospice if on hospice, the facility staff in a facility, the doctor for an expected home death, or 911 only if there is no plan or the death was unexpected.
3. **Have the DNR or POLST visible** if responders may come, so the person's wishes are honored.
4. **Stay as long as you need.** Sit, hold a hand, pray, gather family, say goodbye.
5. **Call the funeral home when ready** — or let hospice or the facility help you choose one.
6. **Name one point person** for the calls and paperwork that follow, and let everything else wait.

■ WORKSHEET — AFTER-DEATH PLAN

Fill in as much as you can ahead of time. This does not replace guidance from hospice, the facility, or the funeral home. In an unexpected death with no plan, call 911.

Person receiving care _____

Current location _____

Family point person / phone _____

Who to call first

First call: Hospice ☐ Facility staff ☐ Doctor (expected home death) ☐ 911 (unexpected / no plan) ☐

CONTACT	NAME / AGENCY	PHONE (INCL. AFTER-HOURS)
Hospice		
Doctor who will sign		
Facility staff		
Funeral home		
Clergy / spiritual support		

Documents ready (check what is visible / located)

- | | |
|--|--|
| ☐ DNR / POLST visible for responders | ☐ Funeral or burial wishes / prepaid plan |
| ☐ Healthcare POA & advance directive | ☐ Important documents folder |
| ☐ Insurance & Social Security information | |

Time in the room — what would bring comfort? (check any)

- ☐ Sitting together
- ☐ Holding a hand
- ☐ Prayer / scripture
- ☐ Music
- ☐ Gathering family to say goodbye
- ☐ Bathing or dressing the person
- ☐ A quiet, private room

Faith, cultural & personal wishes at the time of death

Clergy or spiritual contact to call _____

Rituals, prayers, or readings to honor _____

Who should be present _____

Anything to avoid _____

People to notify (after the first hours)

NAME	RELATIONSHIP	PHONE	WHO CALLS?

■ First Days — Practical Checklist (none of this is urgent)

- ☐ Order several copies of the death certificate
- ☐ Meet with the funeral home to make arrangements
- ☐ Notify Social Security
- ☐ Contact pension, VA, and life insurance
- ☐ Notify banks; change or close accounts
- ☐ Cancel appointments, subscriptions, and services
- ☐ Notify employer or organizations, if applicable
- ☐ Return or arrange pickup of medical equipment
- ☐ Dispose of medications safely (hospice can help)
- ☐ Begin sorting belongings when ready

Point person leading these tasks / phone _____

Next step, and when _____

ROB'S NOTE

The first thing I tell families after a death is this: you do not have to hurry.

The world will tell you to move fast — make the calls, sign the forms, clear the room. But in most cases, the person you love is not going anywhere in the next hour, and neither are you. You are allowed to stay. You are allowed to sit in the quiet. You are allowed to say the last things.

I have watched families panic and call 911 out of reflex, and then stand in a hallway while strangers work over a body that was ready to rest. It does not have to be that way. That is what the plan is for.

When my father died, I did not need a checklist to tell me to hold his hand. But I was grateful, later, that someone had told me the paperwork could wait. Make the one call that matters. Then go back and be a son, a daughter, a spouse — not a coordinator. The rest will keep.

CHAPTER 29 TAKEAWAY

After a death, there is usually more time than families think, and the right first call is rarely 911. The next right question is: **who do we call first, and what can wait until we are ready?**

About the Author

Rob Brizzi is a Certified Dementia Practitioner who has spent his career in home health, hospice, senior living, and other healthcare settings — years of sitting with families in hospital hallways, facility lobbies, and living rooms as they faced the decisions in this book. He is the founder of The Cardinal's Promise, an education effort helping families navigate aging, dementia, caregiving, serious illness, and loss, and the author of the forthcoming memoir *The Cardinal's Promise*. He wrote this toolkit after the death of his father, Lou Brizzi — the man who taught him that showing up is most of the job.

Hope Brizzi, PharmD, served as project consultant. A Doctor of Pharmacy, her clinical eye runs through this book — especially its chapters on medications, prescription costs, and comfort care. She is also the reason it exists: she is the one who told Rob to turn his journals into a book.

Learn more, find caregiver tools, and reach the author at thecardinalspromise.com.

Your State Guide

Almost every office in this book has a version in your state — same job, different name. This page shows you how to find each one. Programs and websites change; when in doubt, start with the Eldercare Locator (1-800-677-1116), whose entire job is connecting you to your local agencies.

WHAT THIS BOOK CALLS IT	HOW TO FIND YOUR STATE'S VERSION
State Medicaid office	Medicaid.gov → "Contact your state," or search "[your state] Medicaid long-term care." Applications run through your county or state social services office — names vary ("Social Services," "Human Services," "Job and Family Services") — or your state's online portal.
SHIP — free, unbiased Medicare counseling	shiphelp.org or 1-877-839-2675. Every state has one; many use their own name (SHIIP, SHIBA, HICAP, and others).
Area Agency on Aging	Eldercare Locator, 1-800-677-1116 or eldercare.acl.gov — it routes you by ZIP code.
Adult Protective Services	Find yours through the National Adult Protective Services Association directory (napsa-now.org) or the Eldercare Locator — every state has an APS program, and many run a statewide hotline.
Portable medical order form (Chapter 23)	Ask the doctor which form your state uses — POLST in many states, MOST in others (polst.org lists every state's program and form).
Advance directive forms (Chapters 20–21)	In most states the attorney general, health department, or bar association publishes free statutory forms; an elder law attorney can confirm what your state requires (witnesses, notarization).
Veterans services (Chapter 18)	Your county Veterans Service Officer (search "[your county] veterans service officer") or VA.gov; every state also has a Department of Veterans Affairs.
Assisted living / facility inspection records	Each state's licensing agency publishes inspection reports — search "[your state] assisted living inspection reports" — and Medicare's Care Compare (medicare.gov/care-compare) rates nursing homes and home health agencies nationally.

State-specific companion guides in the Cardinal's Promise Series — with every program, office, and form for your state named and explained — are available at thecardinalspromise.com.

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Numbers That Matter

Tear this page out, photograph it, or tape a copy to the refrigerator. Phone numbers change — confirm before relying on them, and write your local numbers in the blanks.

WHEN	WHO	NUMBER
Immediate danger or life-threatening emergency	Emergency services	911
Mental health crisis — yours or theirs, any hour	Suicide & Crisis Lifeline	988 (call or text)
Finding local aging services anywhere in the U.S.	Eldercare Locator	1-800-677-1116
Dementia questions or a dementia crisis, 24/7	Alzheimer's Association Helpline	1-800-272-3900
Medicare questions	Medicare	1-800-MEDICARE (1-800-633-4227)
Free, unbiased Medicare counseling	State Health Insurance Assistance Program (SHIP)	1-877-839-2675 · shiphelp.org
Caring for a Veteran	VA Caregiver Support Line	1-855-260-3274
Suspected abuse, neglect, or financial exploitation	Adult Protective Services (statewide hotline in most states)	napsa-now.org or Eldercare Locator 1-800-677-1116

YOUR LOCAL NUMBERS	WRITE THEM HERE
Primary doctor	
Pharmacy	
County / state social services office (Medicaid, APS)	
Area Agency on Aging	
Hospice / home health agency	
Facility nurse / after-hours line	
Family point person	

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Rob Brizzi, Certified Dementia Practitioner

with Hope Brizzi, PharmD, Project Consultant

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